



IT'S ~~ALL~~ ^{Not} YOUR FAULT

— ♦ —
Helping Caregivers with Aging Loved Ones

FILL-IN WORKBOOK · COLOR-CODED

— ♦ —
ROB BRIZZI

For Hope Brizzi, PharmD

*Author of **The Cardinal's Promise***

CARDINAL'S PROMISE SERIES

It's Not Your Fault

Helping Caregivers with Aging Loved Ones

*Caregivers aren't taught how to manage the care of their aging loved ones.
This will be your guide. I created this based on working in healthcare for over
20 years.*

RLB, Certified Dementia Practitioner

Free worksheets, state guides & more at thecardinalspromise.com

“Bear ye one another’s burdens.”

— GALATIANS 6:2

How to Read the Colors

This guide is color-coded so families can find the right level of urgency quickly.

RED — Act Today

Crisis, urgent safety, hospital discharge, hospice, last-days decisions, or caregiver collapse.

PURPLE — Watch Closely

Something has changed. Get support soon, ask better questions, and do not ignore the pattern.

BLUE — Plan Ahead

Documents, money, benefits, medication lists, and conversations best handled before a crisis.

IN A CRISIS RIGHT NOW?

Do not read this book. Turn to page **14**, find the line that sounds like your family, and go to the chapter it names. Every phone number you might need is on the last page.

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FROM THE AUTHOR

Why I Wrote This Book

After my father passed away, I began to journal. That is when my wife, Hope Brizzi, encouraged me to write a book — *The Cardinal's Promise*. It is the raw version of my story.

During that time, I realized there was still a great need: a guide caregivers could use to know what the next right step should be. Nobody trains you for this.

So I wrote it down. The book in your hands is that guide.

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EMERGENCY

If someone is in immediate danger — chest pain, trouble breathing, signs of stroke, a serious injury, uncontrolled bleeding, or a person who cannot be safely moved — **call 911**. This guide helps you plan. It does not replace emergency care.

Dedication

For **Hope Brizzi, PharmD** — my wife, who took a chance on me.

Thank you for your love, your wisdom, and your unwavering support — and for telling me to turn my journals into this book. I love you.

In honor of **Lou Brizzi** — my father, who inspired it. Thank you for choosing me, for believing in me, and for showing me what love looks like.

How to Use This Guide

This book is built to be useful on your worst day, not read straight through.

If you are caring for a husband, wife, partner, sibling, or friend — this book is yours too. It says "parent" because that is the road I walked; wherever it does, read "the person you love."

Start wherever the pressure is. If you're in a crisis right now, go straight to Start Here — the triage pages right after the Contents — and to whichever chapter matches what you're facing. If you've got a little more breathing room, read Part 1 first to get your bearings, then work into the care-options, financial, documents, and hospice sections as you need them.

Every chapter follows the same pattern, on purpose: a plain-truth explanation, what families usually think, the teaching, and then:

- **What to Do This Week** — concrete steps you can work through and check off.
- **The Tools** — the worksheets, checklists, and fill-in fields for that chapter.
- **Rob's Note and the Chapter Takeaway** — the heart of it, and the single next question to carry forward.

Write in it. The worksheets in each chapter belong to you. Copy them, fill them in, take them to appointments, hand them to a sibling. A guide with notes scribbled in the margins is a guide doing its job. Need a clean copy? Every worksheet in this book can be downloaded and printed for free at **thecardinalspromise.com** — make a fresh set for each parent, each appointment, each sibling.

It gets specific where that matters. Medicaid, aging services, and a lot of benefits are run at the state and county level, which is exactly where generic national advice leaves families stranded. When a state program, office, or rule applies, this book points you to the right door to knock on — your state Medicaid office, your State Health Insurance Assistance Program (SHIP), your Area Agency on Aging, and your county Department of Social Services. **Your State Guide** at the back of this book shows you how to find your state's version of every one of them.

And it doesn't replace the professionals. What it does is help you ask better questions and show up prepared. The real decisions about money, law, health, and benefits still belong with qualified experts in your state. Use this to walk into those rooms already knowing what to ask.

What Are You Facing?

You do not have to read this book from the beginning before you use it. If your family is calm and planning ahead, start at Part 1 and move through the guide in order. If something is happening right now, this page is the map: find the situation closest to yours, go there, and come back for the rest later. And if your situation is bigger than one chapter — a stroke, a diagnosis, a parent who cannot manage alone — the **Find Your Situation** pathways right after these pages sequence the first moves for you.

If This Is Happening, Start Here

IF THIS IS HAPPENING	GO TO
Mom fell, seems weaker, or is suddenly less steady	Chapter 2: Falls, Weakness, and Decline
Dad is forgetting medications, bills, appointments, or directions	Chapter 3: Memory Loss and Dementia
Your parent is repeating questions or acting confused in new ways	Chapter 3: Memory Loss and Dementia
The caregiver is exhausted, angry, resentful, or overwhelmed	Chapter 4: Caregiver Burnout
One person is doing everything and the family is not helping	Chapter 4 and Chapter 5: The Family Meeting
The family is arguing, avoiding decisions, or blaming each other	Chapter 5: The Family Meeting
Your parent is coming home from the hospital and you feel unprepared	Chapter 6: Home Health — or Chapter 10 if a rehab facility comes first
Your parent needs help bathing, dressing, cooking, or staying safe	Chapter 7: Private Duty Care
Living alone does not seem safe anymore	Chapter 8: Assisted Living
Doctors are talking about comfort, symptoms, or quality of life	Chapter 11: Palliative Care
You are wondering if it may be time for hospice	Chapter 12: Hospice

Death may be getting close	Part 5: Hospice and the Last Days
You are wondering whether Medicare pays for assisted living	Chapter 8: Assisted Living and Chapter 13: Medicare Basics
You are trying to understand home health versus hospice	Chapter 6: Home Health and Chapter 12: Hospice
You need a sitter or daily help and are not sure that is "home health"	Chapter 7: Private Duty Care (Chapter 6 explains the difference)
Prescriptions are getting too expensive	Chapter 17: Prescription Assistance
You are confused about Medicare, Medicaid, insurance, or who pays	Part 3: Paying for Care
You do not know where the legal, medical, or financial papers are	Part 4: The Documents That Matter

EMERGENCY

If you are in immediate danger — chest pain, trouble breathing, signs of stroke, a serious injury, uncontrolled bleeding, or a person who cannot be safely moved — **call 911**. This guide can help you organize decisions. It does not replace emergency care.

If You Are Not Sure Where to Start

Start with these three questions.

- 1. Is anyone unsafe today?** Unsafe can mean falling, wandering, leaving the stove on, missing critical medication, driving dangerously, or being alone when they cannot manage safely. If yes, focus on safety first — see Chapters 2, 3, and 4.
- 2. Has something changed recently?** A change may be physical, mental, emotional, financial, or behavioral: more falls, new confusion, weight loss, missed medications, unpaid bills, less bathing, more sleeping, new agitation, caregiver exhaustion. If yes, write down what changed — see Chapter 1.
- 3. Who needs to be involved?** Most caregiving crises get worse when one person carries everything alone. Ask who is the main caregiver, who has medical and financial information, who can attend appointments, and who needs to be part of decisions — see Chapter 5.

In the First 24 Hours — Do These

SEVEN ACTIONS, NOT SEVEN CHAPTERS

1. **Handle immediate safety** — the person, the stove, the car keys, the stairs.
2. **Photograph the insurance and Medicare cards**, front and back.
3. **Gather the medication list** — or the bottles, in one bag (Chapter 25).
4. **If a hospital is involved, ask:** "Is my parent admitted as an inpatient, or under observation?" Write down the answer and who gave it — Chapter 13 explains why this one question matters so much.
5. **Find out who is legally authorized to make decisions**, and where the power of attorney papers are (Chapters 19–20).
6. **Start a notes page** — what changed, when it started, who told you what.
7. **Then go to the chapter this page pointed you to.** That is your next right step.

In the next 24–72 hours: read the chapter that matches your situation, hold a short family meeting (Chapter 5), check the caregiver's own limits honestly (Chapter 4), and start the Important Documents Folder (Chapter 19) and the Emergency Contacts and Crisis Plan (Chapter 26).

Finding immediate help: your Area Agency on Aging, your county or state social services office, your state Medicaid office, and your State Health Insurance Assistance Program (SHIP) for Medicare counseling. Your State Guide at the back of this book shows how to find each one, and Numbers That Matter — the last page — puts every phone number in one place.

THE CARDINAL RULE

When everything feels urgent, do not try to solve the whole future. Write down: What changed? What is unsafe today? Who needs to know? What is the next call? The next appointment? The next document to find? That is enough for now.

One more thing. You don't have to solve all of this today. Find the most urgent issue, do one thing about it, and then take the next right step. That's it. That's the whole method.

Choose Your Path

Answer each question and follow the arrow — like a story where you pick the page. Start at 1. You will land on your pathway or chapter within a minute.

1 • IS ANYONE IN IMMEDIATE DANGER RIGHT NOW?

Yes → Call **911** now. This book will be here when you get back.

No → Go to 2.

2 • IS YOUR PARENT IN A HOSPITAL RIGHT NOW?

Yes → Ask today: "Inpatient, or under observation?" — then go to **Pathway 1** (next pages) and Chapters 10 and 13.

No → Go to 3.

3 • HAS A DOCTOR MENTIONED HOSPICE, COMFORT CARE, OR THAT TIME MAY BE SHORT?

Yes → **Chapters 11–12** (Palliative Care and Hospice) and **Part 5** (The Last Days). The Hospice chapter tells you what to ask first — and earlier is almost always better than later.

No → Go to 4.

4 • DID SOMETHING CHANGE SUDDENLY — OVER DAYS, NOT MONTHS?

Yes → Call the doctor today. Sudden confusion or weakness is often reversible — infection, dehydration, medications. Chapters 1–3.

No → Go to 5.

5 • IS MEMORY THE MAIN WORRY — OR WAS DEMENTIA JUST DIAGNOSED?

Yes → **Pathway 3** and Chapter 3. If wandering has started, call the Alzheimer's Helpline (1-800-272-3900) today.

No → Go to 6.

6 • CAN YOUR PARENT STILL MANAGE DAILY LIFE ALONE, SAFELY?

Yes, and nothing has really changed → Go to 7.

Yes — but something is slowly slipping: small changes, a messier house, a quieter

parent → **Chapter 1.** Start the change log and tell the doctor what you are seeing. Worry with nothing provable is a real stage, and it has a chapter.

No → **Pathway 2**, then match the help to the gaps: Chapters 6–9.

7 · IS THE WORRY MONEY, COVERAGE, OR PROTECTING ASSETS?

Yes → **Pathways 4 and 5** and Part 3 (Paying for Care).

No → Go to 8.

8 · IS THE CAREGIVER AT THE EDGE — EXHAUSTED, ANGRY, NOT SLEEPING?

Yes → Chapter 4, today. The self-check takes five minutes. If you are having thoughts of harming yourself, call or text **988** any hour.

No → Go to 9.

9 · NOTHING IS ON FIRE?

Then you have the gift of time → start the Important Documents Folder (Chapter 19), get the Powers of Attorney and advance directive signed (Chapters 20–22), and read For the Future Planners. Future-you says thank you.

Find Your Situation

Wherever you are, this book can meet you there. Some problems are too big for one chapter. They need steps from a few chapters, in the right order, and often fast. These five pathways cover the situations families face most. Each one is two pages: the first steps to take, the mistake that costs families the most, and which chapters to turn to next. If your situation is here, start here. If it is not, the "What Are You Facing?" pages just before this will point you the right way — and the Choose Your Path questions on the page before walk you here, step by step.



Start where you are. The next page begins your path.

YOUR SITUATION

"Mom had a stroke — she's in the hospital and they say she needs a skilled nursing facility."

THE ONE THING THAT PROTECTS EVERYTHING ELSE

Before anything else, ask the hospital — today, and get it in writing: **"Is my mother admitted as an inpatient, or under observation?"** Medicare generally covers the SNF stay only after a **3-day inpatient** hospital stay — observation days and the discharge day do not count. If she is on observation status, ask the doctor whether inpatient admission is appropriate. (Many Medicare Advantage plans — and some Medicare programs — waive this rule; call the plan and ask.) → *Chapters 10 and 13, pp. 101, 121*

Your First Five Moves

1. **Confirm inpatient status** (above) and write down who told you, and when.
2. **Meet the discharge planner** — ask for a list of SNFs with open beds, then check each one on [medicare.gov/care-compare](https://www.medicare.gov/care-compare) before you agree. You may say: "We would like to understand our choices." → *Chapter 10, p. 101*
3. **Get the medication list** and the names of her hospital doctors before the transfer — hand-offs are where medication errors happen. → *Chapter 25, p. 192*
4. **Know the money clock:** with a qualifying stay, Medicare generally pays days 1–20 in full, days 21–100 with a daily coinsurance (\$217 in 2026 — confirm at [medicare.gov](https://www.medicare.gov)), and nothing after 100. Rehab rarely runs that long — but know the numbers before the bills arrive. → *Chapters 13–15, pp. 121–133*
5. **Attend the SNF care conference** (ask for one in week one). Ask: "What are the therapy goals, and what does discharge planning look like from here?"

THE TRAPDOOR TO WATCH

If the SNF says therapy will end because she has "plateaued" or is "not making progress" — **that is not the rule.** Medicare covers skilled care to maintain function too (the *Jimmo* settlement). You can appeal the cut-off notice through your Medicare QIO — the fast appeal costs nothing to file, and the clock is short: call the same day. → *Chapter 10, p. 101.*

While She Recovers — Two Quiet Tasks

- ☐ Learn the stroke warning signs for next time — **BE-FAST**: balance, eyes, face drooping, arm weakness, speech trouble → note the time, call 911. → *Chapters 1 and 26, pp. 33, 198*
- ☐ If her paperwork is not done and she can still make decisions, this is the week: Healthcare Power of Attorney and advance directive. → *Chapters 20–21, pp. 161–167*

PATHWAY TAKEAWAY

Confirm **inpatient** status today, pick the SNF from real inspection data, and never accept "plateaued" as the end of therapy without an appeal.

YOUR SITUATION

"Dad has been declining — he can't take care of himself anymore."

FIRST, RULE OUT THE REVERSIBLE

If the change is **sudden** — days, not months — treat it as medical until proven otherwise: infections, dehydration, and medications cause confusion and weakness that can be reversed. Call his doctor today and describe what changed, with dates. → *Chapters 1–3, pp. 33–47*

Your First Five Moves

1. **Write the change log:** what he can no longer do safely — bathing, meals, medications, stairs, driving — with examples and dates. Facts move doctors. → *Chapter 1, p. 33*
2. **Book a full evaluation** with his doctor. Bring the log and ask: "Is any of this reversible, and does he meet criteria for a home health evaluation?" → *Chapter 6, p. 75*
3. **Match help to the real gaps** — not the brochure: daily hands-on help is private duty or family (Ch. 7); therapy and nursing at home is home health (Ch. 6); rising supervision needs point to assisted living or memory care (Ch. 8–9).
4. **Get the documents signed while he can:** Healthcare and Financial Power of Attorney and an advance directive require capacity — waiting can cost the family a guardianship case later. → *Chapters 20–22, pp. 161–174*
5. **Call the family meeting** before the crisis picks the date for you — who does what, who pays what, who decides what. → *Chapter 5, p. 64*

THE TRAPDOOR TO WATCH

Medicare does not pay for long-term custodial care — not at home, not in assisted living, not in a nursing home. If his money may run short, start the Medicaid conversation early: the **five-year look-back** (in most states) can penalize money moved late, and an elder-law attorney before any transfer is cheaper than one after. → *Chapters 14–15, pp. 127–133*

And One Thing for You

- ☐ If you are the one absorbing the decline, read the burnout chapter before you think you need it — the scored self-check takes five minutes, and asking for help is planning, not failure. → *Chapter 4, p. 55*

PATHWAY TAKEAWAY

First, check for problems that can be fixed. Then get help that fits what he really needs, sign the legal papers while he still can, and start the money talk now — before a crisis starts it for you.

YOUR SITUATION

"Mom was just diagnosed with dementia — and Dad can't watch her every minute."

FIRST, STEADY YOURSELF

A dementia diagnosis changes the timeline, not the person — today she is still who she was yesterday. Two things matter more this month than everything else combined: **use the capacity window** (documents she can still legally sign) and **build the safety net before the first crisis**, not after. → *Chapters 3, 20–22, p. 47*

Your First Five Moves

1. **Ask the doctor what kind and what stage** — and whether anything reversible is mixed in (medications, thyroid, B12, depression can mimic or worsen dementia). → *Chapter 3, p. 47*
2. **Documents this month, while she can sign:** Healthcare and Financial Power of Attorney and an advance directive. Waiting risks a guardianship case later. → *Chapters 20–22, pp. 161–174*
3. **Call the Alzheimer's Association 24/7 Helpline — 1-800-272-3900** — for local programs, caregiver classes, and support groups; ask about a wandering-response or locator program now, before you need one. → *Chapter 3, p. 47*
4. **Make the house honest:** medication lockbox or organizer, stove and driving plan, door alerts if wandering starts, and a current photo for emergencies. → *Chapters 3, 26, pp. 47, 198*
5. **Decide how Dad gets breaks** — respite is not a luxury; a plan that burns out the well spouse fails both of them. → *Chapter 4, p. 55*

THE TRAPDOOR TO WATCH

Sudden confusion is not "the dementia getting worse." A fast change — hours or days — is delirium until proven otherwise: infection, dehydration, or a new medication. It is often reversible. Call the doctor the day it happens, not after the weekend. → *Chapter 3, p. 47*

When Home Stops Being Enough

- ☐ Watch for the tipping points: wandering Dad cannot catch, nighttime agitation nobody sleeps through, aggression, or care needs beyond two hands. Touring memory care *before* the crisis keeps the choice yours. → *Chapter 9, p. 95*

- On tours, ask the question most families miss: *"How do you handle agitation, fear, aggression, or refusal of bathing and medication — what non-drug approaches do you try first, and what is your approach to antipsychotic medications?"* → Chapter 9, p. 95

PATHWAY TAKEAWAY

Sign the legal papers while he still can, keep the crisis Helpline handy, and put safety steps in place before he wanders the first time. And build in real breaks — this is a long road, and no one can walk it alone.

YOUR SITUATION

"I lost Mom years ago. Now Dad is declining — how do I protect his assets and make sure he gets care?"

THE HONEST ANSWER FIRST

"Protect all the assets" and "qualify for help paying for care" pull against each other, and the law knows it: Medicaid looks back **five years in most states** at money and property moved for less than value, and late transfers can create penalty months when he needs care most. The single highest-value move on this page: **see an elder-law attorney before anyone moves a dollar** — the consult costs a fraction of one look-back mistake. → *Chapters 14–15, pp. 127–133*

Your First Five Moves

1. **Documents while he has capacity:** durable Financial Power of Attorney, Healthcare Power of Attorney, advance directive. Without them, you may need a guardianship case just to help him. → *Chapters 20–22, pp. 161–174*
2. **Build the inventory:** accounts, deed, income, insurance policies — including any **long-term care insurance** he may have forgotten he owns. One folder. → *Chapters 16, 19, pp. 139, 157*
3. **Elder-law consult** (look for CELA certification or a NAELA member): what is exempt, what spend-down would look like, whether any planning still makes sense at his timeline — *before* any transfer, "adding names" to accounts, or signing the house over.
4. **Match care to today's gaps** — home health evaluation, private duty hours, adult day, or a residential move. → *Chapters 6–9, pp. 75–95*
5. **If he ever served:** ask about VA pension with Aid and Attendance — and note the VA has its own **36-month look-back**. → *Chapter 18, p. 150*

THE TRAPDOOR TO WATCH

The neighbor's advice — **"just put the house in your name"** — is how families create Medicaid penalty periods and surprise tax bills at the same time. Widowed parents managing money alone are also prime targets for exploitation: set up account alerts, and treat any new "friend" with financial interest as a red flag. → *Chapter 26, p. 198*

PATHWAY TAKEAWAY

Get the documents done first, then take stock of his money and property, and see an elder-law attorney before moving any assets. Match the help to what he needs now — not to the crisis you're afraid of later.

YOUR SITUATION

"Mom is deciding between Original Medicare and Medicare Advantage."

THE TRADE IN ONE PARAGRAPH

Original Medicare (usually with a Medigap supplement and a Part D drug plan) costs more per month, but nearly any doctor or hospital that takes Medicare, with few prior authorizations. **Medicare Advantage** replaces that with one private plan — lower premium, often dental/vision extras — in exchange for networks, prior authorization, and copays that stack up in a bad year. Neither is "wrong." One question decides most cases: *which would she rather have in the year she gets really sick?* → Chapter 13, p. 121

The Five Questions to Answer Before Choosing

1. Are **her doctors and preferred hospital** in the Advantage plan's network — and what happens if they leave it?
2. Are **her medications** on the plan's formulary, and at what tier? (Run them through the Medicare Plan Finder at [medicare.gov](https://www.medicare.gov).)
3. Can she **afford the Medigap premium now**? Predictable premiums versus unpredictable copays is the real math.
4. Does she **travel or split the year** between states? Networks travel poorly; Original Medicare goes everywhere.
5. How would she feel about **prior authorization** for rehab or home health during a serious illness?

THE TRAPDOOR TO WATCH

The choice **can get you stuck** more than you might think. Leaving Advantage later may mean buying Medigap *with medical underwriting* — in most states the insurer can charge more or say no because of her health, exactly when she most wants back in. Ask SHIP about guaranteed-issue windows and trial rights *before* choosing, not after.

Free, Unbiased Help

- ☐ **SHIP** — free, unbiased Medicare counseling: **1-877-839-2675** · shiphelp.org (they will sit with her and run her exact doctors and drugs)

- **Open Enrollment** runs October 15 – December 7 each year; plans change their networks and formularies annually — recheck even if she is happy

PATHWAY TAKEAWAY

Check that her own doctors and medicines are covered by the plan. Look at what a bad year would really cost, not just the monthly premium. And learn how the Medigap health questions work before she picks a plan — not after.

For the Future Planners

If nothing is urgent yet, this is where you start.

A calm stretch may be the best time to use this book.

Most families wait until the fall, the hospital stay, the diagnosis, the discharge meeting, or the late-night phone call. By then, every decision feels heavier because it has to be made fast. Planning ahead does not mean you are expecting the worst. It means you are protecting the people you love from having to guess later.

Start with the blue chapters: Medicare Basics, Medicaid Basics, Private Pay, Long-Term Care Insurance, Prescription Assistance, VA Benefits, The Important Documents Folder, Healthcare Power of Attorney, Living Will / Advance Directive, Financial Power of Attorney, and Medication and Doctor Lists. Then build the Emergency Contacts and Crisis Plan (Chapter 26) — it wears red because of *when* you will need it, but the best time to build it is now, while nothing is burning.

Your goal is simple: know where the documents are, understand who can make decisions, talk about money before panic enters the room, and write down the medical information no one wants to hunt for during an emergency.

A future planner does not need to solve everything. Start with three questions: Who can speak for my parent if they cannot speak for themselves? Where are the documents, passwords, insurance cards, and medication lists? What would my parent want if life changed quickly?

That is not fear. Not having a plan is what costs families — sometimes thousands, sometimes millions.

■ **Worksheet — Future Planner Snapshot** — fill it in from the printable companion workbook · TheCardinalsPromise.com/resources

You Are Not Alone

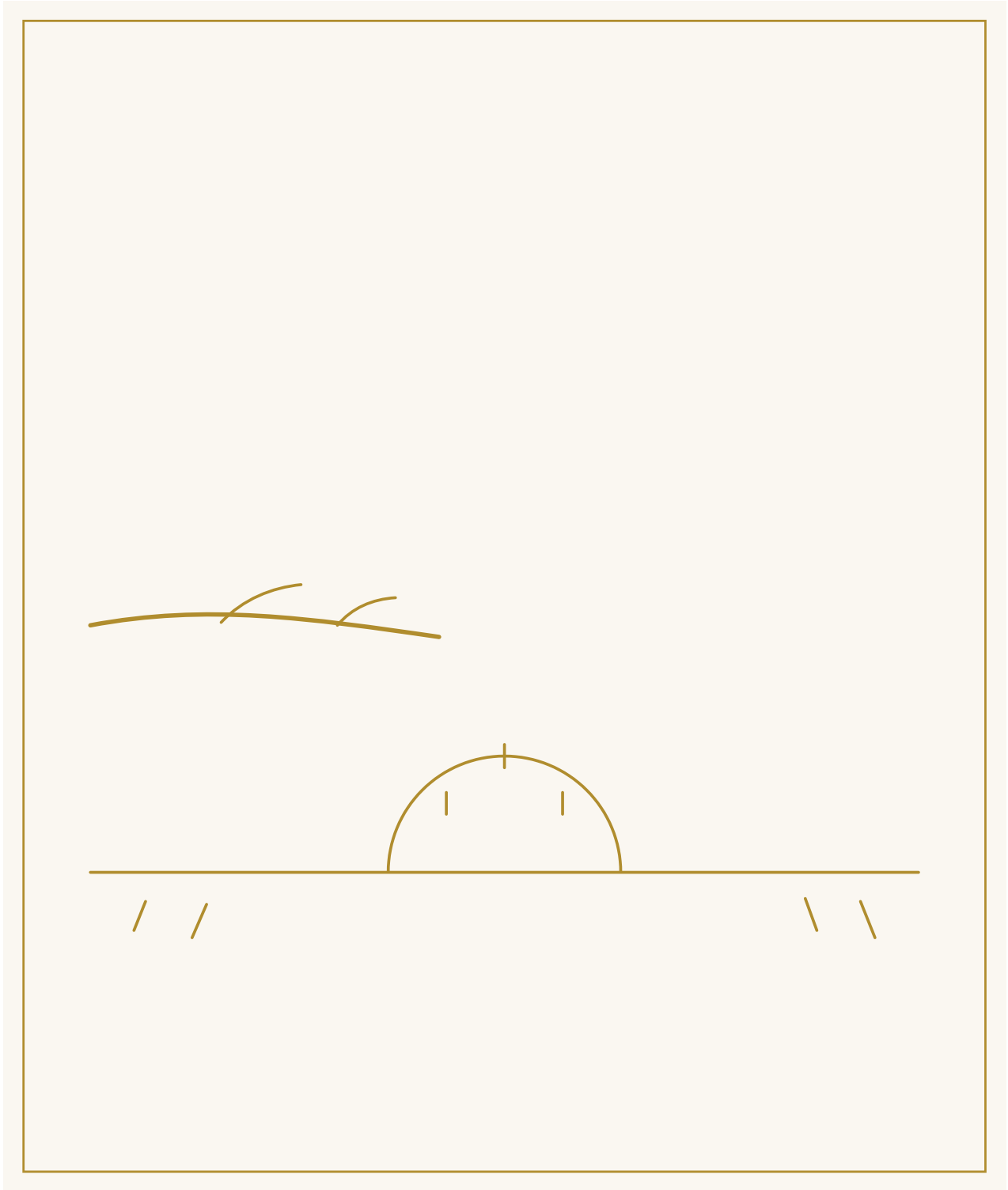
Before you learn anything in this book, put this down first: you are not alone, and you are not failing.

If you are reading this, someone you love needs more than they used to. Maybe it happened overnight — a fall, a phone call, a hospital hallway. Maybe it crept in so slowly you only see it now, looking back. Either way, you did not sign up for this, nobody trained you for it, and you are somehow supposed to know what to do. Of course you feel overwhelmed. Anyone standing where you are standing would.

So take one breath before the work begins. You do not have to fix everything today. You do not have to have the answer to a question you were handed an hour ago. You only have to take the next right step — and this book exists to show you what that step is, one page at a time.

You will feel tired. You will feel guilt you did not earn. You will wonder if you are doing it right. That doubt is not a sign you are the wrong person for this. It is a sign you love them. The ones who never wonder are rarely the ones sitting where you are sitting.

There are millions of us — the sons and daughters and spouses quietly carrying someone through the hardest season of their life. You have joined us. We are glad you are here, and we are sorry you needed to come. Whatever brought you to this page, you do not have to walk it alone. Turn the page when you are ready.



Every journey begins with one honest look.

PART 1 · SOMETHING HAS CHANGED

CHAPTER 1

Something Has Changed

Part 1 — Something Has Changed. This part is for the moment when your gut knows something is different, but you do not yet know what it means. A parent may seem weaker. More forgetful. Less steady. More withdrawn. Or maybe nothing dramatic has happened yet, but the house feels different — the daily rhythm is off. Families often wait for a major event before they act — a fall, a hospitalization, a stove left on, a doctor saying, "I'm concerned." But many caregiving journeys begin before the crisis, with small signs that are easy to explain away. Sometimes those explanations are true. Sometimes they are the family's way of buying time. You do not need to panic. You do need to pay attention.

"The best time to plant a tree was twenty years ago. The second best time is now."

— PROVERB

THIS CHAPTER IN 30 SECONDS

- **The rule:** patterns deserve attention — and a sudden change should always be taken seriously. The better first question is not "Is this normal aging?" — it is: what changed, when, and is it affecting safety?
- **The myth:** families think they need to be certain before they act. Caregiving does not require perfect certainty. It requires honest attention.
- **First move:** write down the change with facts and dates ("got lost driving to church on March 3") — vague worry is easy for a busy doctor to dismiss; concrete observations get taken seriously.
- **The script:** raise it with your parent first, and gently — *"I've noticed the stairs seem harder lately — can we figure out something together?"*
- **Watch for:** sudden change — confusion is sometimes caused by infection, dehydration, medication changes, pain, poor sleep, or low oxygen, problems that may be treatable. If the change is sudden, call the doctor; if the person may be in immediate danger, call 911.

WHAT FAMILIES USUALLY THINK

Families often think they need to be certain before they act. So they wait — until they can prove something is wrong, until the parent agrees, until the doctor brings it up. They wait because naming the change makes it real.

But caregiving does not require perfect certainty. It requires honest attention. You are not accusing your parent of failing. You are noticing that life may require more support than it used to. That difference matters.

What You May Be Seeing

Physical changes.

- ☐ More falls or near falls
- ☐ Trouble standing from a chair
- ☐ New weakness
- ☐ Weight loss
- ☐ Sleeping more
- ☐ Less interest in food
- ☐ Poor hygiene
- ☐ Trouble walking
- ☐ New pain
- ☐ Shortness of breath
- ☐ Swelling
- ☐ Frequent infections
- ☐ New incontinence

A sudden change should always be taken seriously.

Memory and thinking changes.

- ☐ Repeating the same question
- ☐ Getting lost in familiar places
- ☐ Missing appointments
- ☐ Forgetting medications
- ☐ Confusing dates
- ☐ Trouble managing bills
- ☐ Poor judgment
- ☐ Falling for scams
- ☐ New suspicion

Memory changes that affect safety, finances, medication, or daily living should not be ignored.

Home safety changes. Spoiled food, burned pans, cluttered walkways, unpaid bills, mail piling up, scattered medication bottles, poor lighting, loose rugs, or trouble getting in and

out of the shower. The home often tells the truth before the person does. Look gently. Do not shame. Just notice.

Emotional and behavioral changes. Withdrawal, irritability, anxiety, depression, new anger, suspicion, loss of interest in hobbies, refusing help, or increased confusion in the evening. Behavior is information. It may be grief, pain, depression, infection, dementia, medication side effects, or loneliness. The point is not to label it immediately — it is to write it down and ask better questions.

When to Call the Doctor

Call the doctor when a change is new, worsening, or affecting daily life. Call especially for sudden confusion, a fall, new weakness or trouble walking, new shortness of breath, chest pain, signs of infection, medication mistakes, weight loss, dehydration, new incontinence, or sudden mood changes.

A sudden change in an older adult can have a medical cause. Do not assume it is "just aging." Confusion is sometimes caused by infection, dehydration, medication changes, pain, poor sleep, or low oxygen — problems that may be treatable. If the change is sudden, call. If the person may be in immediate danger, call 911.

What Is Urgent and What Can Wait

CALL 911 FOR

Chest pain, trouble breathing, signs of stroke, serious injury from a fall, uncontrolled bleeding, loss of consciousness, sudden severe confusion, or a person who cannot be safely moved. **Signs of stroke are sudden:** face drooping, arm weakness, slurred or garbled speech, sudden confusion, sudden vision loss, or trouble walking. Note the time it started and call 911 immediately — stroke treatment is time-limited.

Call the doctor soon for: a fall without obvious serious injury, new confusion or weakness, medication errors, weight loss, less eating or drinking, increased pain, new swelling, or a caregiver who can no longer manage safely. (One exception: a head strike in someone on a blood thinner is an emergency-department visit the same day — see the red box in Chapter 2.)

Schedule a planning conversation for: gradual decline, increasing need for help, difficulty managing the home, unclear documents, no emergency plan, or concerns about driving or living alone. Planning early is not overreacting. It is how families avoid being forced into decisions during a crisis.

How to Raise It

Raise it with your parent first, and gently. Lead with care, not a verdict. "I've noticed the stairs seem harder lately — can we figure out something together?" lands far better than "You can't manage anymore." People resist being managed and respond to being respected — preserve their dignity and their voice in the decision wherever you safely can. Then bring it to the physician. Before the visit, write down specific examples: "got lost driving to church on March 3." Vague worry is easy for a busy doctor to dismiss; concrete observations get taken seriously.

WHAT TO DO THIS WEEK

1. **Write down the change.** Use facts and dates. Do not rely on memory during stress.
2. **Check immediate safety.** Bathroom, medications, food, stove, driving, ability to call for help, and the caregiver's safety too.
3. **Call the doctor** if the change is new or concerning. Use your notes. Be specific.
4. **Tell the key family members.** Create awareness, not panic.
5. **Choose the next right section of this guide.** Physical change → Chapter 2, p. 39.
Memory → Chapter 3, p. 47. Caregiver overwhelmed → Chapter 4, p. 55. Decisions needed → Chapter 5, p. 64.

■ MINI-TOOL — THE "SOMETHING HAS CHANGED" NOTE

Use this when you are worried but not sure what to do next.

What changed? _____

When did it start? _____

Is it getting better, worse, or the same? _____

Is anyone unsafe today? _____

What have we already tried? _____

Who needs to know? _____

What is the next call? _____

What is the next appointment? _____

What information do we need to find? _____

■ QUESTIONS TO ASK THE DOCTOR

1. Could this change be caused by infection, dehydration, medication, pain, or another treatable issue?
2. Are any medications possibly causing confusion, dizziness, weakness, or falls?
3. Should we check bloodwork, urine, oxygen, or blood pressure?
4. Is it safe for my parent to live alone right now?
5. Should my parent be driving?
6. Would home health, therapy, or nursing support help?
7. Should we consider a memory evaluation?
8. What changes should make us call you again — and what should send us to the hospital?

ROB'S NOTE

Families usually know more than they think.

They may not know the diagnosis. They may not know the benefit rules. But they know when something feels different.

I have watched families talk themselves out of that feeling because they did not want to upset anyone. I understand that. No one wants to be the person who says, "Mom is not safe anymore."

But noticing a change is not betrayal. It is care. You are not taking over. You are paying attention. And sometimes, paying attention is the first loving thing you can do. It can turn panic into a plan.

CHAPTER 1 TAKEAWAY

You do not need to diagnose the problem today. The next right question is: **what changed, and is anyone unsafe today?**

CARDINAL MOMENT



A daughter once told me her mother was "completely fine." Then, almost like it was no big deal, she added that her mom had started keeping the mail in the refrigerator. She said it in a calm, everyday way. For months, she had found a reason for each small thing. On its own, each one was easy to explain away.

But when she listed them out loud, all in a row, she finally heard it. She cried. Nothing new had happened. She had just let herself see what she already knew. Then she wiped her face and said, "Okay. What do I do first?"

That question is where everything starts. Noticing is not being disloyal. It is a loving thing to do for someone who can no longer take care of everything alone.

A composite drawn from years at the bedside. Names and details changed.

Falls, Weakness, and Decline

THIS CHAPTER IN 30 SECONDS

- **The rule:** a fall is not just a fall. The better question is not whether anything broke — it is: why did the fall happen, and what needs to change so it does not happen again?
- **The myth:** "She just tripped." Caution is not a care plan. The family's job is not to blame — it is to investigate.
- **Red line:** a head strike on a blood thinner — even daily aspirin — means the emergency department the same day, even if they seem fine. Bleeding inside the head can show up hours or days later.
- **The script:** not "Can you order home health?" — say *"Can we schedule an evaluation to discuss this fall or decline and determine whether home health is appropriate?"*
- **Watch for:** near falls — the smoke alarm before the fire. Write down every fall and near fall and bring the log to appointments.

WHAT FAMILIES USUALLY THINK

Families often say, "She just tripped," "It was one time," or "We moved the rug, so it should be fine now." Sometimes that is true. But caution is not a care plan. Most older adults are not falling because they are careless. They are falling because something has changed — the body, the home, the medications, the disease, or the level of support needed. The family's job is not to blame. It is to investigate.

Why Falls Matter

A fall can change everything quickly — fractures, head injury, hospitalization, rehab, loss of confidence, less activity, more weakness, more dependence. Even without a major injury, many older adults become afraid after a fall. They move less, their legs weaken, and the risk of the next fall rises. That loop can become a trap: fall, fear, less movement, more weakness, another fall. This is why a "minor" fall should not be ignored. It may be the first sign of a major shift.

The Fall That Changes Everything

Families often picture a fall as a single bad moment — someone goes down, someone gets patched up, life returns to normal. Sometimes that is exactly how it goes. But for many older adults, one fall is the hinge the rest of life turns on. A broken hip leads to surgery. Surgery leads to a hospital stay. The hospital stay leads to days in bed, and days in bed cost strength that a frail body may never fully rebuild. The person who walked in on their own can come home needing a walker, a wheelchair, or a level of help they did not need the week before.

This is the part no one warns families about: it is often not the fall itself that steals independence — it is everything the fall sets in motion. Doctors call the loss of strength from lying still *deconditioning*, and in an older adult it can happen fast. That is why the goal is never just to treat the injury. It is to protect the strength, the confidence, and the independence that the fall put at risk — starting the same week, with movement, therapy, and a plan, not bed rest and waiting. Treat the first serious fall as a turning point, because for many families, that is exactly what it turns out to be.

After a Fall: First Questions

Start with safety. Are they hurt? Did they hit their head? Are they bleeding? Can they move normally? Are they confused? Do they have new pain? Are they on blood thinners? Do they remember what happened? Has this happened before?

ACT NOW

If there is serious injury, a possible head injury, sudden confusion, chest pain, trouble breathing, one-sided weakness, severe pain, or they cannot be safely moved, **call 911**. Do not drag or force someone up if you are not sure they are safe to move — a second injury can happen when a frightened caregiver tries to lift someone alone.

While they are on the floor: check for injury before any move, keep them warm and calm, and if they are uninjured but cannot get up, call 911 or the non-emergency line and ask for a **lift assist** rather than lifting alone. And know the term "long lie": an older adult who has been down for an hour or more needs medical evaluation even without visible injury.

When to Call the Doctor After a Fall

Call after a fall if:

- ☐ It is new or repeated
- ☐ They hit their head
- ☐ They are on blood thinners

- ☐ They have new pain or weakness
- ☐ They are dizzy or confused
- ☐ They do not remember falling
- ☐ They are walking differently
- ☐ They are afraid to walk
- ☐ They are eating or drinking less
- ☐ They recently changed medication
- ☐ You are not sure why the fall happened

Do not assume a fall is normal aging. A doctor may review medications, check blood pressure and hydration, screen for infection, or order therapy. The goal is to prevent the next fall, not just to write this one down.

HEAD INJURY + BLOOD THINNERS

If your parent takes a blood thinner — anticoagulants like warfarin, Eliquis, or Xarelto, antiplatelet drugs like Plavix, or even daily aspirin — and hits their head, do not wait for symptoms. **Go to the emergency department the same day, even if they seem fine.**

Bleeding inside the head can show up hours or days later. After any head strike, watch for worsening headache, vomiting, new confusion, or unusual sleepiness over the next 24–72 hours.

Weakness Is Information

Weakness may look like trouble standing from a chair, holding walls or furniture while walking, shuffling steps, needing more help with stairs, difficulty bathing, more fatigue after small tasks, or refusing a needed cane or walker. Weakness affects safety, toileting, bathing, nutrition, and the caregiver's workload. If your parent is getting weaker, write it down: when did it start, is it sudden or gradual, did it begin after a hospital stay or a medication change, is there pain, shortness of breath, or dizziness? Weakness has a story. The family needs to help tell it clearly.

Decline After the Hospital

Many families are shocked by how much an older adult can decline after a hospital stay. This does not always mean the decline is permanent, but it does mean the discharge plan matters. Before discharge, ask: Can they walk safely? Can they get to the bathroom? Do they need a walker, wheelchair, shower chair, or bedside commode? Are they safe alone? Do they need home health or rehab? Who is managing medications? What changed during

the stay, and who do we call if things get worse? Hospital discharge is not the finish line. It is a handoff — and bad handoffs create crises.

When Home Health May Help

After a fall, new weakness, illness, or a hospital stay, families should ask whether home health may be appropriate. Home health is medical care in the home — skilled nursing, physical, occupational, or speech therapy, medical social work, and limited aide support tied to the care plan. It is not the same as private duty care and does not usually provide long blocks of supervision, housekeeping, or ongoing sitting services.

IMPORTANT

Home health is not something a family can simply request over the phone and receive automatically. For Medicare-covered home health, a clinician must determine that the person meets the requirements — including a skilled need, homebound status, and the required face-to-face documentation. So the better request is not "Can you order home health?" It is: **"Can we schedule an evaluation to discuss this fall or decline and determine whether home health is appropriate?"**

Home Health vs. Private Duty Care

These sound similar but are not the same.

HOME HEALTH	PRIVATE DUTY CARE
Usually short-term and medical	Usually non-medical daily support
Ordered by a clinician; criteria apply	Arranged by the family; often paid privately
Nursing, PT, OT, speech, social work, limited aide	Bathing, dressing, meals, housekeeping, companionship
Not there all day	Can cover longer blocks, including overnight
Helps treat, teach, assess, and strengthen	Helps with daily support and supervision

A family may need both. The question is not which sounds better. It is: **what does my parent actually need at home?**

Home Safety After a Fall

Families often say, "She's lived in this house for forty years — it's perfectly safe." The familiarity is real, but so are the new risks. A home that was fine for a healthy adult can be

full of hazards for someone with reduced balance, vision, or cognition. And the changes that prevent falls usually extend independence rather than restrict it — they let a parent stay home longer and more safely.

A safer home does not have to be fancy. Start with the obvious risks:

- ☐ Loose rugs
- ☐ Clutter
- ☐ Poor lighting
- ☐ Stairs without railings
- ☐ Slippery bathroom floors
- ☐ No grab bars
- ☐ A low toilet seat
- ☐ Furniture that is hard to rise from
- ☐ No phone or call device nearby

The bathroom is one of the highest-risk areas — consider:

- ☐ Grab bars
- ☐ A shower chair
- ☐ A handheld shower head
- ☐ A non-slip mat
- ☐ A raised toilet seat
- ☐ A bedside commode
- ☐ A nightlight along the path from bed to bathroom

Small changes can prevent big injuries.

Near Falls Count Too

Families often remember the big fall but not the near falls — grabbing the counter suddenly, stumbling but catching themselves, losing balance while turning, or saying, "I just got dizzy for a second." Near falls are the smoke alarm before the fire. Write them down. Several near falls deserve to be taken as seriously as a fall to the ground.

When Falls May Mean More Help Is Needed

A fall does not automatically mean assisted living or memory care. But repeated falls may mean the current setup is no longer safe — especially if your parent lives alone, cannot reliably call for help, forgets to use the walker, falls at night, cannot bathe safely, is on blood thinners, has dementia, or the caregiver cannot supervise safely. The goal is not to keep someone home at all costs. It is to keep them as safe, supported, and dignified as

possible. Sometimes home still works with the right help. Sometimes home is no longer the safest place. Avoiding the question does not make the risk disappear.

WHAT TO DO THIS WEEK

1. **Write down every fall and near fall** — date, time, place, what happened, injury, whether they hit their head, and whether they used a cane or walker.
2. **Call the doctor** — especially if the fall is new, repeated, unexplained, or tied to weakness, dizziness, or confusion.
3. **Ask about home health** — request an evaluation to see whether nursing, PT, or OT is appropriate at home.
4. **Review medications** — ask the doctor or pharmacist whether any could raise fall risk. Do not stop medications on your own.
5. **Make the home safer** — start with the path from bed to bathroom, then the shower, stairs, rugs, and lighting.
6. **Ask whether more support is needed** — physical or occupational therapy, home health, private duty care, rehab, or a care-setting change.

■ MINI-TOOL — THREE QUESTIONS AFTER A FALL

1. What happened? Where was the person? What were they doing? Did they trip, slip, faint, or feel dizzy? Did anyone see it?

2. What changed? New pain? New confusion or weakness? New fear of walking? New medication? Less eating or drinking?

3. What needs to change now? Call the doctor? Home safety changes? Therapy? Home health? More supervision? Safe alone tonight?

Fall Log — record every fall and near fall. Bring this to appointments.

DATE	TIME	LOCATION	WHAT HAPPENED	INJURY / NEXT STEP

■ QUESTIONS TO ASK THE DOCTOR

1. What could be causing the falls or weakness?
2. Could medications be contributing?
3. Should blood pressure be checked sitting and standing?
4. Should we check for infection, dehydration, anemia, or pain?
5. Is physical or occupational therapy appropriate?
6. Would an evaluation for home health be appropriate?
7. Does my parent need a cane, walker, wheelchair, shower chair, or bedside commode?
8. Is it safe for my parent to live alone, bathe alone, or drive?
9. What signs should make us call again — and what signs mean call 911?

ROB'S NOTE

Families often tell me about "the fall."

But after I ask a few more questions, I usually hear about the near falls that came before it. The stumble in the hallway. The hand grabbing the kitchen counter. The bad turn in the bathroom. The moment Dad said, "I'm fine," then had to sit down for twenty minutes.

A fall is not always the beginning. Sometimes it is the first thing the family can no longer ignore.

That is why I want families to write things down early. Not to build a case against the person they love. To build a plan around what is really happening. Asking about home health care is not overreacting. It is smart. A nurse or therapist who comes to the home may notice things the family misses. The family is too close, too tired, or too used to getting by.

CHAPTER 2 TAKEAWAY

Falls, weakness, and decline are information. The next right question is: **why did this happen, and what needs to change so it does not happen again?**

CARDINAL MOMENT



A man told me his wife had "just slipped in the kitchen." That morning, she was walking on her own. A few weeks later, when I met them, she was in a wheelchair. It was not from the broken hip. That had healed. It was from all the weeks she spent lying still while it healed.

He kept saying, "It was only one fall." But that was not the part he blamed himself for. No one had told them the real danger. It was not the broken bone. It was staying still for so long after it. Once therapy started and she began to move again, some of her strength came back. Not all of it. But enough to show them both that moving sooner would have helped.

If someone you love falls, treat the days that follow like they matter. Because they do.

A composite drawn from years at the bedside. Names and details changed.

Memory Loss and Dementia

THIS CHAPTER IN 30 SECONDS

- **The rule:** memory loss is not always dementia. The first question is not "Is this dementia?" It is "Is this safe?" — medication, driving, cooking, finances, and being alone.
- **The myth:** many people with dementia can sound convincing on the phone. A parent may seem fine during a short visit but struggle when alone — the person handling the daily details often sees the truth first.
- **First move:** write down examples with dates and ask for an evaluation. Not "Dad is getting forgetful" — "Dad got lost driving home from church, missed two medication doses this week, and paid the same bill three times."
- **The script:** before accepting a dementia diagnosis as the whole story, ask "*Is there anything simple and reversible we might be missing here?*" — infection, dehydration, a thyroid problem, a vitamin B12 deficiency, depression, or medications can look exactly like dementia.
- **Watch for:** sudden confusion is delirium — it comes on over hours to days and is a medical urgency, different from dementia's slow slope over months to years. Using the word with the care team gets it taken seriously.

WHAT FAMILIES USUALLY THINK

Families often think dementia means one obvious thing: a person who does not know who or where they are. But many people with dementia can still hold good conversations, tell old stories, recognize family, and sound convincing on the phone. That is what makes it confusing. A parent may seem fine during a short visit but struggle when alone — well in the morning, confused by evening. The sibling who calls for five minutes may say, "Dad sounds fine." The daughter who manages the pillbox may say, "Something is wrong." The person handling the daily details often sees the truth first.

What Memory Loss May Look Like

- ☐ Repeating questions
- ☐ Forgetting recent conversations
- ☐ Missing appointments
- ☐ Losing important papers

- ☐ Forgetting medications or taking them twice
- ☐ Leaving food out
- ☐ Getting lost while driving
- ☐ Trouble using the phone or stove
- ☐ Confusion with dates
- ☐ Difficulty managing bills
- ☐ Falling for scams
- ☐ New suspicion or paranoia
- ☐ Neglecting hygiene

One mistake may not mean dementia. A pattern deserves attention.

Safety Comes First

The first question is not "Is this dementia?" It is "**Is this safe?**" Memory loss becomes urgent when it affects medication, driving, cooking, finances, hygiene, or nutrition — or when it leads to wandering, falls, or an inability to respond to an emergency. A person can be loving, intelligent, and still unsafe. Safety does not erase dignity. Safety protects dignity from a preventable crisis.

Medication Safety

Medication problems are one of the clearest signs that more support may be needed — a full pillbox when it should be empty, pills on the floor, duplicate bottles, taking morning pills at night, or refusing medication out of confusion. Medication mistakes can cause falls, confusion, weakness, or hospitalization. If mistakes are happening, do not just remind the person to be careful. Change the system: a pill organizer, blister packs from the pharmacy, phone alarms, family checks, a medication review, or an evaluation for home health nursing to help with medication teaching. The question is not whether your parent "should" remember. It is whether the medication system is safe.

Driving Safety

Driving is one of the hardest conversations, because for many older adults it means independence and identity. But unsafe driving can put your parent and others at risk. Warning signs:

- ☐ Getting lost on familiar roads
- ☐ New dents
- ☐ Confusing the pedals
- ☐ Drifting from the lane

- ☐ Missing stop signs
- ☐ Becoming overwhelmed in traffic
- ☐ Family feeling afraid as passengers

Ask the doctor, "Is it safe for my parent to continue driving?" and ask about a formal driving evaluation if available. If driving must stop, make a transportation plan first — for appointments, groceries, church, and social connection. Removing keys without a plan creates anger and isolation. When driving ends, support must replace what is lost.

Money, Cooking, and Wandering

Money and scams. Watch for unopened mail, late notices, duplicate donations, unusual withdrawals, new "friends" asking for money, or secrecy around finances. Handle it carefully — the goal is to prevent harm, not to seize control. Ask whether there is a financial power of attorney, who is trusted to help, whether bills can be on autopay, and whether professional guidance is needed. Financial decline may be a symptom, not a character flaw.

Cooking and the kitchen. Burned pans, a stove left on, spoiled food, little food in the house, or replacing meals with snacks. If the stove is unsafe, take it seriously — meal delivery, family-prepared meals, private duty help at meal times, stove shut-off devices, or removing the knobs. Food is independence and comfort, but safety comes first.

Wandering and getting lost. Leaving the house at night, trying to "go home" while already home, getting lost in stores, or restlessness in the evening. Wandering can become dangerous quickly. Ask whether the person can safely be alone, whether doors are secured, whether identification or a medical ID is available, and whether memory care is becoming necessary. Program the Alzheimer's Association 24/7 Helpline (1-800-272-3900) into your phone and ask about a wanderer locator program (MedicAlert, Project Lifesaver, or similar). Let trusted neighbors know proactively, and consider a heads-up call to the police non-emergency line. A person who wanders may be trying to meet a need their brain can no longer explain. The family still has to protect safety.

Mood and Behavior Changes

Dementia can bring anxiety, depression, irritability, suspicion, hallucinations, agitation, sundowning, or refusing care. These changes are painful — a daughter may say, "This is not my mother." That grief is real, but behavior is still information. Pain, infection, medication side effects, poor sleep, constipation, dehydration, and stress can all worsen confusion. When behavior changes suddenly, call the doctor. Do not assume every change is "just dementia."

Entering Their Reality

As dementia progresses, the rules of conversation change. The goal shifts from exchanging accurate information to preserving connection and dignity. When a person with dementia believes something that isn't true — that they need to go to a job they retired from decades ago, that a long-dead spouse is coming home — arguing or correcting rarely helps and often causes real pain. Respond to the feeling, not the fact. "You're worried about getting to work on time" acknowledges the emotion without a confrontation that neither of you can win.

Keep it simple and calm. Use short sentences and one idea at a time. Ask one question, not several. Give simple choices — "Would you like the blue sweater or the green one?" — rather than open-ended ones. Your tone and body language carry more than your words; a calm, warm presence reassures even when the words don't land.

A Predictable Routine Helps

A predictable routine is one of the most underrated tools in dementia care. It lowers anxiety, conserves everyone's energy, and keeps essentials from falling through the cracks. Build the day around anchors — consistent wake and sleep times, regular meals, and medication at the same times — and schedule the demanding tasks, like bathing and appointments, for the time of day your parent is at their best, which for many is the morning. Watch for "sundowning," the increased confusion and agitation that often arrives in the late afternoon and evening, and keep those hours calmer, quieter, and simpler. Leave room for what gives the day meaning, not just what keeps it functioning: a walk, music, a visit, time outside.

Rule Out the Reversible Causes

Some conditions that look exactly like dementia are treatable and even reversible. A urinary tract infection in an older adult can cause sudden, dramatic confusion. So can dehydration, a thyroid problem, a vitamin B12 deficiency, depression, or a bad combination of medications. When confusion arrives suddenly — from an infection, dehydration, or a medication — clinicians call it **delirium**: a state that comes on over hours to days, fluctuates through the day, and clouds attention. It is a medical urgency, and it is different from dementia's slow slope over months to years; using the word with the care team gets it taken seriously. Before accepting a dementia diagnosis as the whole story, insist that the doctor rule these out. The phrasing that works: **"Is there anything simple and reversible we might be missing here?"** True dementia is progressive, but the real question is always: dementia caused by what?

When to Call the Doctor

Call when memory or thinking changes are new, worsening, sudden, or affecting safety — sudden confusion, new hallucinations or paranoia, medication mistakes, getting lost, falls, weight loss, unsafe driving, wandering, or new incontinence. Ask for an evaluation, and use specific examples. Instead of "Dad is getting forgetful," say, "Dad got lost driving home from church, missed two medication doses this week, and paid the same bill three times."

When Home Health or Memory Care May Help

Home health may help when memory loss is connected with a medical need, recent decline, falls, medication concerns, weakness, hospitalization, or caregiver teaching. It is not long-term supervision, a sitter service, or private duty care — and it is not ordered simply because a family asks over the phone. For Medicare-covered home health, a clinician must evaluate the person and confirm the requirements are met, including skilled need, homebound status, and face-to-face documentation. If you are concerned, ask for an appointment: "We are seeing memory and safety changes. Can we schedule an evaluation to determine whether home health is appropriate?"

Memory care may be needed when dementia creates safety risks that cannot be managed at home — ongoing wandering, continued medication mistakes, being unsafe alone, a caregiver who cannot sleep, unsafe agitation, or increasing falls. Memory care is not a punishment. It is a care setting. Families should not wait until the caregiver collapses or the person is harmed before having the conversation.

WHAT TO DO THIS WEEK

1. **Write down examples with dates** — medication mistakes, driving concerns, safety issues, repeated questions, wandering, or behavior changes.
2. **Check safety** — medications, driving, cooking, being alone, ability to call for help, and money.
3. **Call the doctor** and ask for a memory or cognitive evaluation, using specific examples.
4. **Review medical causes** — infection, dehydration, pain, medication side effects, depression, or poor sleep.
5. **Get documents in order** — healthcare and financial power of attorney, advance directive, medication and doctor lists, emergency contacts. Do this early.
6. **Decide what support is needed now** — family check-ins, home health, private duty care, adult day care, assisted living, memory care, or a family meeting.

■ MINI-TOOL — MEMORY AND SAFETY SNAPSHOT

Use this when you are worried about memory loss but unsure what to do next.

What changed? _____

When did it start? Is it getting worse? _____

Is medication safe? _____

Is driving safe? _____

Is cooking safe? _____

Is the person safe alone? _____

Are bills and money safe? _____

Is the caregiver safe and able to continue? _____

Who needs to know? What is the next call? _____

■ DEMENTIA SAFETY CHECKLIST

Check any concern that applies. If several are checked, do not wait — call the doctor and begin a family planning conversation.

- | | |
|---|---|
| ☐ Repeats questions often | ☐ Wanders or tries to leave |
| ☐ Forgets or misuses medications | ☐ Is confused at night |
| ☐ Gets lost driving or has new driving concerns | ☐ Has unsafe anger or agitation |
| ☐ Leaves stove or appliances on | ☐ Has poor hygiene |
| ☐ Forgets to eat or has spoiled food in the home | ☐ Cannot call for help reliably |
| ☐ Misses bills or falls for scams | ☐ Caregiver is exhausted or unsafe |
| ☐ Misplaces items or accuses others of stealing | |

■ QUESTIONS TO ASK THE DOCTOR

1. Could this be dementia, delirium, depression, medication side effects, infection, dehydration, or pain?
2. Should we do a memory screening and review medications?
3. Should we check labs or urine?
4. Is my parent safe to live alone, drive, cook, and manage medications?
5. Would an evaluation for home health, nursing, therapy, or social work help?
6. Should we see a neurologist, geriatrician, or memory clinic?

7. What changes should make us call again — and what should be treated as urgent?

ROB'S NOTE

Dementia can be very hard. Families lose the person a little at a time.

One day they can still tell stories from forty years ago. The next day they cannot remember if they ate lunch. They may know who you are one minute, then blame you the next. They may sound fine on the phone, then leave the stove on after dinner.

That back-and-forth confuses families. It can also split them apart. The person who visits for an hour may think everything is fine. The person who handles the pills, the bills, and the late-night calls knows it is not.

If that is you, write things down. Not to prove everyone else wrong. To help the whole family see clearly. Memory loss needs kindness. It also needs a plan. It can need both at once.

CHAPTER 3 TAKEAWAY

Memory loss is about safety, judgment, and daily life — not only forgetting. The next right question is: **what is no longer safe to leave to memory alone?**

CARDINAL MOMENT



A woman kept asking to go home. But she was already standing in the house where she had lived for forty years. She did not mean this home. She meant a porch, a dog, her mother's cooking. She meant a place from long ago. Her son used to correct her. "Mom, you ARE home." Every time, she got more scared. She felt more sure she had been taken somewhere strange.

One tired evening, he stopped correcting her. Instead he asked, "Tell me about home. What's it like there?" She talked for twenty minutes. As she talked, she calmed down. He had not lied to her. He had just stopped arguing about a place he could no longer follow her to.

You cannot win an argument against a disease. But you can almost always answer the feeling behind the words.

A composite drawn from years at the bedside. Names and details changed.

Caregiver Burnout

THIS CHAPTER IN 30 SECONDS

- **The truth:** burnout is not weakness, selfishness, or a lack of love — it is a warning sign. When a caregiver burns out, everyone becomes less safe.
- **The myth:** "I should be able to do this." Love does not remove exhaustion. Asking for help is not failure — it is planning.
- **The check:** take the burnout self-check, then ask the two most important questions: Is the caregiver safe? Is the person receiving care safe under the current plan? If either answer is no, the plan needs to change.
- **This week:** write down the actual workload, then ask by task — not "I need help," but "*Can you take Mom to her appointment Thursday at 10?*" Specific requests are harder to ignore.
- **If you are at the edge:** call or text 988 any hour, and ask the doctor, a social worker, or the Eldercare Locator (1-800-677-1116) about respite options today.

WHAT FAMILIES USUALLY THINK

Families often think caregiving should be manageable because it is done out of love. "She's my mother." "I should be able to do this." "My siblings are busy." "I don't want strangers in the house." Sometimes love gives people strength. But love does not remove exhaustion. Love does not lift a person safely from the bathroom floor, make dementia predictable, or turn one worn-out daughter into a full care team. Families often wait too long because asking for help feels like failure. It is not failure. It is planning.

What Burnout Looks Like

Burnout can look like sadness, anger, control, or silence — a daughter who snaps at everyone, a son who stops answering calls, a spouse who cries in the shower. Watch for constant exhaustion, irritability, resentment, anxiety, depression, trouble sleeping, ignoring personal health, pulling away from friends, feeling trapped, anger toward the parent or siblings, guilt for wanting relief, making more mistakes, and dreading the phone. The caregiver may not use the word burnout. They may say, "I'm fine," or "I don't have a choice." Listen closely. Sometimes "I'm fine" means "I have no room left to fall apart."

IF YOU ARE AT THE EDGE

If you are having thoughts of harming yourself, or you are afraid of what exhaustion might make you do, call or text **988** — the Suicide & Crisis Lifeline — any hour. And tell your own doctor what caregiving is costing you: caregiver depression is a treatable medical condition, not a character flaw. Respite care exists for exactly this moment — ask the doctor, a social worker, or the Eldercare Locator (1-800-677-1116) about respite options today.

It Has a Name

If you have felt all of this and quietly wondered whether you were just weak, hear this plainly: burnout is real, and it has a name. The psychologist Herbert Freudenberger named it in the 1970s, and in 2019 the World Health Organization formally recognized it — a state of deep exhaustion that comes from chronic stress that was never given a chance to ease. The WHO describes it in the language of the workplace, but every hospice nurse, and every daughter sleeping in a recliner outside her father's door, knows the truth: unpaid caregiving is some of the most relentless work there is. What you are feeling is not a character flaw. It is a known, named response to carrying too much for too long without relief.

How Burnout Builds

Burnout rarely arrives in a single day. The psychologists Herbert Freudenberger and Gail North described it as a slow slide of about a dozen stages. You do not need to memorize them — you need to recognize the shape, so you can catch yourself sliding and step off early. For a family caregiver, the slide often looks like this:

1. **The drive to prove** you can handle it all yourself.
2. **Working harder** — taking on every task instead of asking.
3. **Neglecting yourself** — your sleep, your meals, your own doctor.
4. **Pushing down the conflict** — insisting the resentment isn't there.
5. **Rewriting what matters** — friends, hobbies, and your own life quietly drop away.
6. **Denying the strain** — blaming your body or your patience, never the load.
7. **Withdrawing** — pulling back from the very people who would help.
8. **Changing** — becoming short, numb, not quite like yourself.
9. **Feeling unreal** — going through the motions, detached from them and from you.
10. **Emptiness** — a hollow you try to fill with anything.
11. **Depression** — the flatness where nothing feels worth it.
12. **Collapse** — the wall, body and mind at once.

You do not have to reach the bottom of this list to act. The earlier you can name where you are on the slide, the shorter the step back to solid ground.

The Hidden Load

Caregiving is not only the visible tasks. It is the hidden load:

- ☐ Remembering refills
- ☐ Watching for decline
- ☐ Scheduling appointments
- ☐ Handling insurance
- ☐ Tracking bills
- ☐ Anticipating falls
- ☐ Keeping peace between siblings
- ☐ Reading discharge papers
- ☐ Worrying at night

This is why one caregiver may seem "too emotional" to relatives who are less involved. They are not reacting to one task. They are carrying the whole weather system. A family cannot fix burnout until it names the actual load.

The Promise That Traps Families

Many families carry a promise: "I promised Mom I would never put her in a nursing home." Those promises usually come from love — but they are often made before anyone understands what care may require: before dementia causes wandering, before falls happen at night, before bathing becomes unsafe, before the caregiver has not slept in months. A promise made in one season may need wisdom in another. The deeper promise is usually not "I will keep you in this exact place no matter what." It is "I will make sure you are cared for. I will protect your dignity. I will not abandon you." Sometimes keeping the deeper promise means changing the plan. That is not betrayal. That is love growing up under pressure.

Resentment Is Information

Caregivers often feel ashamed of resentment. But resentment is information. It may mean the workload is unfair, there is no relief, boundaries are unclear, others are not helping, or the current plan no longer works. Instead of "I'm a terrible person for feeling this way," try "What is this resentment telling me about the care plan?" The need it points to may be rest, backup, money, a schedule, private duty care, adult day care, or a family meeting. Resentment is not the final answer. It is the smoke under the door.

The Family Pattern Problem

Caregiving does not create family patterns. It reveals them. The responsible sibling becomes more responsible. The avoidant sibling becomes more avoidant. Old wounds walk into the family meeting and sit down. This is why caregiving decisions turn emotional so quickly — families are deciding about medication and money, but also about history. You may not be able to fix the whole family system. But you can make the next decision clearer: write down facts, name the care needs, choose **one point person**, assign specific tasks, and set deadlines. Do not try to heal thirty years of history before deciding who takes Dad to the doctor on Tuesday.

When to Ask for Help

Ask for help before the caregiver breaks. Help may come from:

- ☐ Family
- ☐ Friends
- ☐ Community or faith groups
- ☐ The primary doctor
- ☐ Home health (if there is a skilled need and criteria are met)
- ☐ Private duty care
- ☐ Adult day care
- ☐ Respite care
- ☐ Assisted living
- ☐ Memory care
- ☐ Palliative care
- ☐ Hospice if appropriate
- ☐ A social worker
- ☐ A counselor

Do not wait until you are desperate to learn what exists.

How to Ask Family for Help

Many caregivers ask too generally — "I need help." That is honest, but hard to act on. Name the specific task instead. Not "I need help with Mom," but "Can you take Mom to her appointment Thursday at 10?" Not "I'm overwhelmed," but "I need someone to handle medication refills every month." Specific requests are harder to ignore, and they reveal who is truly willing to help. End the family meeting with names, tasks, and dates — not vague kindness.

When the Parent Refuses Help

Many caregivers are trapped by one sentence: "They won't let me." Refusal is hard, but it does not automatically mean the current plan is safe. Ask whether the parent understands the risk, whether memory loss is affecting judgment, and whether a doctor needs to evaluate capacity or safety. Sometimes help gets in more easily with different words — "Someone is coming to help with laundry and lunch" instead of "You need a caregiver," or "The therapist wants you to use this so you can keep moving safely" instead of "You need a walker." The goal is not to win an argument. It is to lower the risk.

You Cannot Save Someone by Destroying Yourself

In a long stretch of caregiving, it is easy to start believing that if you just try harder, sleep less, and carry more, you can hold everything together on your own. It feels like love. But it does not hold — and a caregiver who collapses does not give more care. They become a second emergency on top of the first.

Resting is not quitting, and asking for help is not failing. Paying for help, or moving your parent somewhere with more hands, is not abandoning them — it is often the most caring thing you can do. The best thing you can do for the person you are caring for is to still be standing next week, and next month, and however long this takes. Taking care of yourself is part of taking care of them. It is the reason they tell you to put your own oxygen mask on first.

PERMISSION TO REST

A nurse, Linda Liner, RN, shared something worth passing on. Over the years, she has heard family members say the same thing: *"I wish someone had given me permission to go home and get some rest."*

So here it is, in writing: you have permission. Go home. Sleep. Rest is not giving up on them — it is how you keep showing up.

Finding Your Way Back

Burnout's opposite is not happiness — it is simply feeling like yourself again. The sociologist Corey Keyes gave a name to the gray middle so many caregivers live in: *languishing* — not depressed, exactly, but not flourishing either; flat, and going through the motions. The writer Adam Grant brought the word to millions because so many people recognized it at once. The way back is rarely one big change. It is small, deliberate steps:

- **Name it.** Say out loud, "I am languishing," or "I am close to burning out." Naming a feeling loosens its grip.
- **Find one thing that absorbs you.** A task that makes you lose track of time — a walk, a recipe, music, the garden — pulls you out of the fog for a while.
- **Protect small blocks of time.** One hour that is yours, guarded like an appointment, does more than you would think.
- **Stay connected.** Isolation deepens the flatness. One honest conversation, one person who knows, is a lifeline — not a luxury.
- **If the flatness does not lift,** tell your doctor. Languishing can slide into depression, and depression is treatable. Asking for help is the strongest move on this page.

WHAT TO DO THIS WEEK

1. **Write down the actual workload** — every task the caregiver handles. The family cannot divide work it has not named.
2. **Take the burnout self-check** on the next page. Do not argue with the score. Use it as information.
3. **Hold a family meeting** (Chapter 5) that produces specific assignments, not vague promises.
4. **Add one form of relief this week** — a covered appointment, meals, help with bills, paid care, or exploring adult day care or respite.
5. **Protect the caregiver's health** — keep their own appointments, sleep, eat, and take time away. That is maintenance, not selfishness.
6. **Recheck the care setting** — is home still safe, is one caregiver enough, is nighttime care breaking the caregiver?

■ MINI-TOOL — CAREGIVER BURNOUT SELF-CHECK

This is not a diagnosis. It is a warning-light checklist. Use it when the caregiver says, "I'm fine," but everything around them suggests otherwise. Check any statement that feels true most days, then total each section.

Physical exhaustion (___ / 8)

- | | |
|---|---|
| ☐ Tired even after sleeping | ☐ Skipping my own appointments |
| ☐ Not sleeping enough | ☐ Eating worse than usual |
| ☐ Waking at night over caregiving | ☐ Getting sick more often |
| ☐ More headaches, tension, or pain | ☐ Physically worn down |

Emotional strain (___ / 8)

- | | |
|---|---|
| ☐ Anxious much of the time | ☐ Sad, numb, or hopeless |
|---|---|

- ☐ Crying more easily
- ☐ Angry or irritated often
- ☐ Guilty when I want a break

- ☐ Feeling trapped
- ☐ Resentful that others don't help
- ☐ Dreading calls and new problems

Caregiving load (___ / 8)

- ☐ Managing most appointments
- ☐ Managing medications
- ☐ Managing bills and insurance
- ☐ Doing most transportation

- ☐ Doing bathing, meals, or daily care
- ☐ The main person everyone calls
- ☐ No reliable backup
- ☐ If I stop, everything falls apart

Safety concerns (___ / 8)

- ☐ Parent should not be left alone long
- ☐ Parent has fallen or nearly fallen
- ☐ Parent is confused about medication
- ☐ Parent is unsafe cooking, driving, or walking

- ☐ I've lifted my parent when it felt unsafe
- ☐ Afraid something bad happens if I leave
- ☐ Making more mistakes because I'm tired
- ☐ The current plan does not feel safe

Total score: _____ / 32

SCORE	WHAT IT MAY MEAN, AND THE NEXT STEP
0–7 · Watch & Plan	Managing for now. Keep a written care list, name one backup, review emergency contacts, organize documents.
8–15 · Stress Building	Hold a family meeting. Assign tasks. Ask what can come off the caregiver's plate. Consider private duty, adult day support, or a home health evaluation.
16–23 · High Risk	Schedule a family meeting soon. Ask for concrete help. Consider paid support. Review whether the parent is safe alone.
24–32 · Crisis	Do not wait. Call a family meeting. Contact the doctor, social worker, or care team. Consider urgent respite or a care-setting change. If anyone is in immediate danger, call 911.

The two most important questions: Is the caregiver safe? Is the person receiving care safe under the current plan? If the answer to either is no, the plan needs to change — not because anyone failed, but because the need has outgrown the plan.

■ CAREGIVER LOAD LIST

Bring this to the family meeting. The goal is to make the invisible work visible, then assign a helper and a start date to each task.

TASK	CURRENTLY DONE BY	NEW HELPER / DATE
------	-------------------	-------------------

Appointments & doctor calls		
Medications & refills		
Bathing, dressing, daily care		
Meals, laundry, housekeeping		
Transportation		
Bills, mail, insurance		
Overnight supervision		
Emotional support & family updates		

What must come off the caregiver's plate this week? _____

ROB'S NOTE

I have met caregivers who could tell me every pill, every diagnosis, every appointment, and every change in their parent's health.

Then I would ask, "How are you?" And the room would go quiet. Not because they did not know the answer. Because no one had asked them in a long time.

Caregivers often end up doing it all. The schedule. The pills. The late nights. The worrying. But everyone has a limit. When the caregiver breaks down, that is not a small, private problem. It is an emergency for the whole family.

If you are the caregiver, please hear this. You are allowed to ask for help before you break down. You are allowed to say the plan is not working. You are allowed to love someone and still be worn out.

CHAPTER 4 TAKEAWAY

Caregiver burnout is not weakness. It is information. The next right question is: **is the care plan protecting both the parent and the caregiver?**

CARDINAL MOMENT



A son had slept in a recliner outside his mother's bedroom door for eight months. That way, he would hear her if she got up in the night. He never chose to become the night shift. It just happened one night, and then it never stopped.

He told me he was fine. But his hands were shaking as he said it. We set up paid help for two nights a week. The morning after the first night, he slept until ten. He woke up crying. Part of it was relief. Part of it was sadness that he had waited so long to let anyone help.

Needing rest never made him a worse son. But believing he could not have any had almost made him sick.

A composite drawn from years at the bedside. Names and details changed.

The Family Meeting

THIS CHAPTER IN 30 SECONDS

- **The point:** a family meeting moves the family from reacting to deciding — one place to name what changed, what is unsafe today, who is responsible for what, and what happens next.
- **The myth:** "We can just text." Avoiding the conversation does not remove the problem. It only pushes the decision into a future crisis.
- **The structure:** bring written facts, include the parent whenever possible, choose one point person, assign tasks with names, dates, and deadlines, and schedule the next check-in before the meeting ends.
- **The script (the Velvet Hammer):** *"Thank you for sharing your thoughts. What part of the plan can you take responsibility for this week?"*
- **When emotion rises:** slow down and come back to one question — "What needs to happen in the next seven days?"

WHAT FAMILIES USUALLY THINK

Families often think a meeting will make things worse. "We don't want drama." "My brother will never agree." "We can just text." Sometimes those concerns are real — family meetings can be emotional. But avoiding the conversation does not remove the problem. It only pushes the decision into a future crisis. A meeting is not about forcing everyone to feel the same way. It is about making the facts visible: what changed, what is unsafe, what the caregiver needs, what documents exist, what care options should be explored, and who is doing what this week.

When a Family Meeting Is Needed

Hold a family meeting when:

- ☐ A parent has fallen
- ☐ Memory loss is affecting safety
- ☐ Medications are being missed
- ☐ A caregiver is overwhelmed
- ☐ Living alone may not be safe

- ☐ A hospital discharge or rehab discharge is coming
- ☐ Driving is becoming unsafe
- ☐ The family disagrees
- ☐ One person is doing almost everything
- ☐ A care-setting change may be needed
- ☐ Hospice or palliative care has been mentioned
- ☐ The current plan no longer works

Do not wait until everyone agrees a meeting is necessary. The person carrying the most responsibility usually sees the need first. If that person says, "We need to talk," believe them.

The Conversation Nobody Wants to Start

Most families wait too long to talk — not because they do not care, but because starting the conversation feels like admitting the thing they are most afraid of. To say out loud, "Dad can't keep living alone," or "Mom isn't safe driving anymore," feels like a betrayal, or like calling down the future by naming it. So everyone stays quiet, hoping someone else goes first, and the risk keeps growing in the silence.

Someone has to be the one to start it, and it is usually the person already carrying the most. You do not need the perfect words. You do not need the whole plan. You only need one honest sentence to open the door: "I'm worried about Mom, and I don't want to figure this out alone — can we talk this week?" Name one specific thing you have seen, keep the focus on safety rather than blame, and pick a time when no one is already angry or exhausted. The conversation nobody wants to start is almost always easier than the crisis that arrives when no one ever started it.

Who Should Be There

Invite the people directly involved in care decisions: the parent (if appropriate and able), spouse or partner, adult children, the main caregiver, the healthcare and financial powers of attorney, a trusted family member, and, if involved, a care manager, social worker, clergy, hospice or palliative team member, or facility representative. Not everyone needs to be in every meeting. Some people create more heat than help. The guiding question is: who needs to know, who needs to decide, and who needs to help?

Should the Parent Be Included?

Whenever possible, the person receiving care should be included in decisions about their own life. They are not a project. They are a person. Consider whether they can follow the

conversation, whether the meeting would overwhelm them, and whether a smaller conversation would be kinder. A parent may not manage every detail but can still answer what matters most: What matters most to you? Where do you feel safest? Who do you trust? What are you afraid of? What do you not want? Do not confuse limited capacity with no voice. Listen for what the person can still tell you.

Choose One Point Person

Every care plan needs one main point person. That does not mean they do all the work — it means they coordinate communication, so no one assumes someone else handled it. The point person keeps the medication list updated, communicates with doctors, shares updates, tracks appointments, and knows where documents are. They do not have to be the oldest child or the loudest voice. They need to be the person most able to help the plan hold together.

Separate Roles From Opinions

Everyone may have an opinion. Not everyone has a role. An opinion sounds like, "I think Mom should stay home," or "We should wait." A role sounds like, "I'll take Mom to the doctor Tuesday," or "I'll call three assisted living communities by Friday," or "I'll pay for two mornings of private duty care this week." Opinions without roles exhaust the caregiver. A family meeting should gently move people from commentary to contribution.

THE VELVET HAMMER

Use this sentence: **"Thank you for sharing your thoughts. What part of the plan can you take responsibility for this week?"** Soft handle, real impact.

The Family Meeting Agenda

Use a simple agenda. Do not let the conversation wander into every old family wound.

1. **What changed?** Name the reason for the meeting in one sentence.
2. **What is unsafe today?** Falls, wandering, missed medications, unsafe driving or cooking, caregiver exhaustion, no overnight coverage.
3. **What matters most to the person receiving care?** Name the priority — safety, comfort, or staying home as long as it is safe.
4. **What support is needed?** Family help, a home health evaluation, private duty, adult day care, assisted living, memory care, palliative care, hospice, or caregiver relief.
5. **Who is responsible for what?** Names. Dates. Deadlines. No fog.

6. **When will we meet again?** Set the next check-in. One meeting is not the whole plan.

Ground Rules, Siblings, Distance, and Money

Ground rules: speak respectfully, use facts not accusations, let the main caregiver speak without interruption, stay on the next decision, and if you have a strong opinion, take a role. If someone becomes cruel or disruptive, pause the meeting. A family meeting is not a fight to be won. The purpose is care.

When siblings disagree, return to facts: "Mom fell Monday and Friday." "Dad missed medication four times this week." "The caregiver has not slept through the night in three weeks." Facts do not remove emotion, but they give the conversation a floor.

The long-distance family member often underestimates the situation after a cheerful five-minute call. Ask better questions — Is she taking medication correctly? Has she fallen? Is the caregiver sleeping? — and offer specific help: bills, research, scheduling, paying for respite, or covering a weekend. Distance changes the kind of help you can give, not whether you help.

When money enters the conversation, start with clarity: what income, savings, and insurance exist; whether there is long-term care insurance or VA eligibility; who has authority to access financial information; and whether professional guidance is needed. Financial confusion damages both care and relationships. Do not let money conversations happen in the shadows.

“One parent can take care of eight children, but eight children cannot take care of one parent.”

— An old proverb, often quoted by consumer advocate Clark Howard

When One Child Does Everything

In most families, the work is not shared evenly. One person — often the one who lives closest, or the daughter, or simply the one who cannot look away — slowly becomes the appointments, the medications, the phone calls, the nights. Everyone else means well from a distance, and because the primary caregiver keeps managing, the family assumes it is handled. The quiet cost is resentment on one side and guilt on the other, and it can fracture a family long after the caregiving ends.

If you are that one child, you are allowed to say plainly, "I can't keep doing this alone," and you are allowed to name specific help instead of waiting to be offered it. If you are the sibling who has been at a distance, do not ask "Let me know if you need anything" — that puts the work of delegating onto the person already drowning. Instead, take something off the plate and own it: "I'll handle the medications and the pharmacy." "I'll take the Tuesday appointments." "I'll pay for two mornings of help a week." The goal is not to keep score of who did what. It is to make sure the family does not spend the one caregiver like a resource that never runs out — because it does.

DIVIDE BY STRENGTHS, NOT GUILT

When you hand out the work, match it to what each person is actually good at — not to who feels guiltiest or who happens to live closest. The sibling who stays calm with numbers takes the bills, insurance, and benefit applications. The one who is steady in a doctor's office manages the medical calls and appointments. The far-away one who is warm on the phone takes the daily check-in calls and the online research. Roles assigned by strength get done; roles handed out by guilt get dropped.

And do not save the talking for emergencies. A five-minute update every Sunday — what changed, what is coming up, who is handling what — keeps the 2 a.m. crisis call from being the only time the family talks. Care runs on steady, ordinary communication, not dramatic rescues.

When the Meeting Gets Emotional

It probably will. That does not mean the meeting failed. Tears do not mean the family is weak; silence does not mean nobody cares. These conversations are hard because they involve loss, fear, duty, money, old history, and love. When emotion rises, slow down and say, "Let's come back to the next decision," or "We do not have to solve the whole future today," or "What needs to happen in the next seven days?" That question pulls the conversation out of the storm and back onto the table.

What If No One Agrees?

Sometimes the meeting ends and the family is still split — one sibling wants Mom to stay home, another wants assisted living, and a third does not want to discuss it at all.

Deadlock feels like failure, but it is common, and it does not have to stop a decision from getting made. When opinions collide, go back to two anchors that outrank any single vote: **what is safe** and **what the person receiving care actually wants**. A preference to "stay home" does not survive a parent who is falling weekly and cannot call for help — the facts decide, not the loudest voice.

When a family still cannot agree, remember who holds the authority. If your parent still has capacity, the choice is theirs, even if the family dislikes it. If they do not, the named healthcare or financial power of attorney carries the decision — that is the entire reason those documents exist (Chapters 20 and 22). And when the conflict runs too deep to settle at the kitchen table, a neutral third party can break the logjam: a doctor, a social worker, a geriatric care manager, a mediator, or clergy. Bringing in help is not surrender. It is what families do when they love someone more than they need to be right.

WHAT TO DO THIS WEEK

1. **Name the reason for the meeting** in one sentence: "Mom has fallen twice, and we need a safer plan."
2. **Invite the right people** — decision-makers, caregivers, and those who need assignments.
3. **Bring written facts** — logs, medication lists, discharge papers, examples.
4. **Choose one point person** and a backup.
5. **Assign tasks** — each with a person, a deadline, and a next step.
6. **Schedule the next check-in** before the meeting ends.

■ **Worksheet — The Family Meeting** — fill it in from the printable companion workbook · TheCardinalsPromise.com/resources

ROB'S NOTE

Family meetings are hard. Families are people, not numbers on a list.

Every family has a past. People fall back into old roles. Old hurts come back up. The one who always fixes things will try to fix it all. The one who hates conflict may go quiet or leave. The person who gives the most care may seem upset. That's because they are the closest to the pain.

That doesn't mean the meeting is a waste. It means the meeting needs a plan. Facts matter. Names matter. Dates matter.

The goal is not to win a fight with your family. The goal is to protect the person who needs care. It's also to keep one person from doing all the work alone. When you feel stuck, ask one simple question: what needs to get done in the next seven days?

CHAPTER 5 TAKEAWAY

A family meeting makes the facts visible, assigns real tasks, and protects both the person receiving care and the caregiver. The next right question is: **who is doing what by when?**

CARDINAL MOMENT



Three grown-up siblings sat in a small room. They would not look at each other. One of them had done everything for two years. The other two lived in far-off cities. They had watched from a distance, and they had opinions about all of it.

We did not fix their past that day. We just made a list — who does what, and by when. The sister who had done it all alone finally took a deep breath. The brother lived four states away and had felt useless. Now he was in charge of the medicine and the pharmacy. For the first time in two years, he felt like a son again, not just someone watching from the side.

The meeting did not turn them into a perfect family. It made them a team for the one job that mattered most. Sometimes that is the most love a family can give at one time. And it is enough.

A composite drawn from years at the bedside. Names and details changed.

BEHIND THIS BOOK

There's a story behind this guide. Rob tells it in his memoir, *The Cardinal's Promise*. Read more at thecardinalspromise.com.

Aging Whispers Before It Shouts

Nobody picks the day they become a caregiver.

It starts with a fall. Or a diagnosis. Or a hospital stay. Or a discharge planner on the phone asking where your parent is going next, like you're supposed to already know. One day you're a son or a daughter. The next you're making decisions nobody prepared you for.

I know how that sneaks up on you, because looking back, my parents had been aging for years before I let myself see it.

In 2019, my mother and I were walking through Central Park. She was sixty-eight. She stepped toward a curb and went down. It didn't seem like much at the time — people trip, people fall. We got her up and went on with our day. But it stuck with me. It wasn't that she fell. It was *how* she fell. I remember thinking, my mom is getting old. Then life moved on and I let it. I didn't ask questions. I didn't look closer. I didn't connect anything.

Three years later she was visiting us in Pensacola, standing in our kitchen, and I noticed her hand shaking. She looked down at it and said, almost offhand, "I need to get in with an acupuncturist."

That night I described it to Hope, my wife, a pharmacist. She didn't hesitate. "Rob, your mom has Parkinson's."

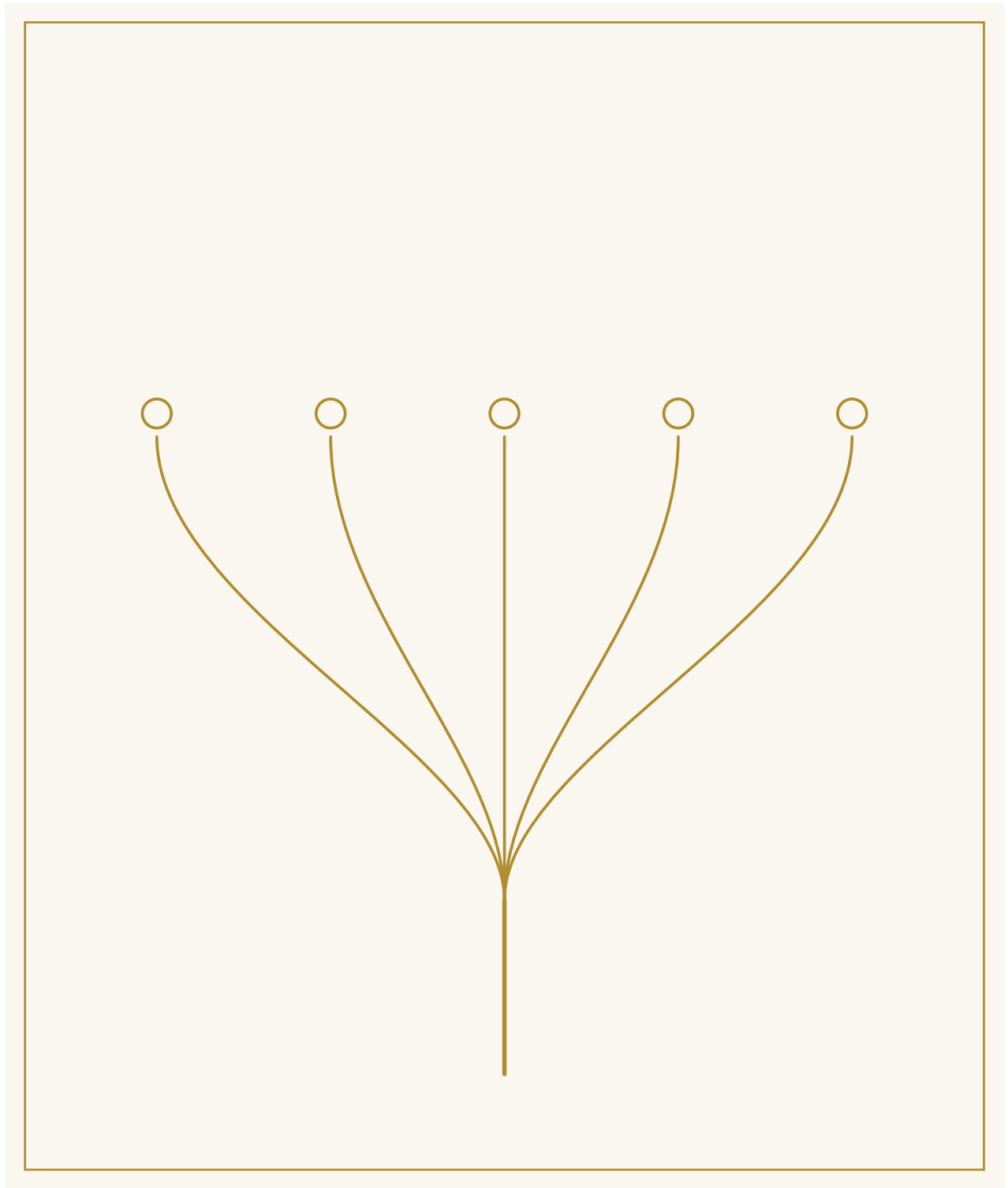
I started crying. Not because a doctor had confirmed anything — none had, yet — but because somewhere in me I already knew she was right. A neurologist confirmed it later.

Here's what I understand now: my silence those three years came from love and fear and denial, all tangled together. I wasn't afraid of being wrong. I was afraid of being right. Knowing something and being able to look straight at it are two different things, and when it's your own parent, looking straight at it is the hardest thing there is.

Aging almost never announces itself all at once. It whispers before it shouts. The curb in Central Park was a whisper. The shaking hand in my kitchen was another. I only heard them once I looked back.

The whispers aren't only physical. Consumer advocate Clark Howard has spent years urging families to stay almost nosily present in an aging parent's money — the bills, the accounts, the paperwork — long before anything looks wrong. There's wisdom in that for hearing the whisper: a checkbook often says what a face still hides. A missed payment, a withdrawal your parent can't explain, a scam that finally gets through — these are rarely just money problems. They are often the first quiet word that something is changing.

So hear this before anything else: you're not behind. Nobody teaches this. You're walking into the middle of the storm, same as the rest of us did. You don't have to know everything today. You just have to know the next right step. Keep this book close. It's meant to make sense even on the worst day.



Help has many doors. This part walks you through the right one.

Home Health

Part 2 — Care Options. Once a family sees that something has changed, the next question is usually simple and overwhelming: what kind of help do we need? These chapters explain the major options — home health, private duty care, assisted living, memory care, skilled nursing and rehab, palliative care, and hospice — each in the same pattern, so families can compare without getting lost. You do not need to memorize every service. You need to know what each one is, what it is not, when it may help, what to ask, and what to do next.

“If you want to go fast, go alone. If you want to go far, go together.”

— AFRICAN PROVERB

THIS CHAPTER IN 30 SECONDS

- **What it is:** medical care at home — nursing, PT, OT, speech, social work, limited aide visits — ordered by a clinician for people who meet the requirements.
- **What it is not:** all-day supervision, housekeeping, meals, or a sitter. That is private duty care (Chapter 7).
- **When to ask:** after a fall, hospitalization, surgery, new weakness, new wound, medication change, or a decline in bathing, dressing, or walking.
- **How to ask:** not “order home health” — say “*Can we schedule an appointment or provider evaluation to discuss this recent change and determine whether home health is appropriate?*”
- **If the answer is no:** ask what criteria are not met and what support they recommend instead.

“Home health is like raising your hand in the classroom. It takes humility to ask for help, but those who do often learn faster, recover stronger, and reach their goals sooner. Sometimes, asking for help is the smartest step you can take.”

— Nick Adams, PTA

Follow the Insurance

When my father was being referred to home health, a social worker asked an important question: "What insurance does he have?" At first, it seemed routine. But because my father was still working, Medicare was not his primary insurance. That detail mattered.

Years in healthcare taught me that not all insurance plans reimburse providers the same way. Some are easier to work with. Some pay differently. Some require additional approvals. During my career, I once overheard a provider discussing a referral and saying something to the effect of: "I'll take this private-insurance patient if you send me your next two Medicare patients."

Whether said jokingly or seriously, it revealed something many families never see: behind every referral is a business reality. The lesson is not to become cynical. The lesson is to become informed. Ask questions. Why was this agency selected? What other agencies are available? Does this agency accept my insurance? Do I have a choice? Follow the insurance — not because money is more important than care, but because knowing how it works helps you make sure the care is right.

WHAT FAMILIES USUALLY THINK

Families often think home health means someone will stay with Mom, that Medicare will send an aide every day, that it will handle bathing, meals, and supervision, or that the doctor can just order it over the phone. Some of that comes from confusion, some from hope.

Families are tired and scared. They hear "home health" and imagine a safety net large enough to catch everything. But home health is a specific service. It can be valuable and even powerful — but it is not every kind of help. A family needs to know the difference so they do not build a plan on the wrong promise.

What Home Health May Include

Skilled nursing — medication and disease teaching, wound care, symptom monitoring, safety teaching, caregiver instruction, and communication with the doctor.

Physical therapy — strength, balance, walking, transfers, fall risk, stairs, safe use of a walker or cane, and recovery after hospitalization or decline.

Occupational therapy — bathing, dressing, toileting, transfers, kitchen safety, energy conservation, equipment, and home setup for safer independence.

Speech therapy — swallowing concerns, communication, cognition, memory strategies, and changes after illness or stroke.

Medical social work — community resources, long-term planning, caregiver stress, advance care planning, and understanding care options.

Home health aide — limited personal care (such as bathing help) when it is part of the skilled plan of care. Aide visits are usually limited — not the same as daily private duty caregiving.

What Home Health Usually Does Not Provide

Home health usually does not provide all-day supervision, overnight or long shifts, housekeeping, meal preparation, transportation, grocery shopping, companionship, routine daily medication reminders without a skilled need, ongoing bathing help as the only need, sitting services so the caregiver can leave, or twenty-four-hour care. This is where families often get frustrated — they thought home health would solve the whole home situation. It is not meant to replace family care, private duty care, assisted living, memory care, or long-term supervision. It may be one part of the care plan. It is rarely the whole plan.

Home Health Is Not Private Duty Care

The names sound similar; the services are different. A family may need both — home health to strengthen, teach, evaluate, and treat, and private duty to help with daily support and supervision. The question is not which one sounds better. It is: what does my parent actually need at home?

HOME HEALTH	PRIVATE DUTY CARE
Medical, intermittent, clinician-ordered	Non-medical daily help, often paid privately
Nursing, therapy, social work, limited aide	Bathing, meals, companionship, supervision
After illness, wounds, weakness, or decline	When the person cannot safely be alone
Not long blocks of supervision	Can cover longer blocks and overnight

When Home Health May Help

Ask about a home health evaluation after:

- ☐ A fall
- ☐ New weakness

- ☐ Trouble walking
- ☐ Surgery
- ☐ Hospitalization
- ☐ Rehab discharge
- ☐ A new wound
- ☐ Medication changes
- ☐ A new diagnosis
- ☐ A disease flare-up
- ☐ Shortness of breath
- ☐ Increased confusion with a medical concern
- ☐ A decline in bathing, dressing, or transfers

Home health may also help when the family needs trained eyes inside the home — a therapist may notice unsafe transfers, a nurse may catch medication confusion, a social worker may see caregiver strain, an occupational therapist may spot a bathroom setup creating risk. Sometimes the home tells a different story than the doctor's office.

Home Health Requires Evaluation and Documentation

HOW TO ASK, THE RIGHT WAY

Home health is not usually something a family simply requests over the phone and automatically receives. For Medicare-covered home health, a clinician must determine whether the person meets the requirements — a skilled need, homebound status, and the required face-to-face encounter documentation. So the better request is not "Can you just order home health?" It is: **"Can we schedule an appointment or provider evaluation to discuss this recent change and determine whether home health is appropriate?"**

At the visit, bring clear examples:

- ☐ Falls and near falls
- ☐ New weakness
- ☐ Walking changes
- ☐ Trouble getting out of bed or chairs
- ☐ Medication concerns
- ☐ Bathing or dressing difficulty
- ☐ Recent hospitalization
- ☐ Wounds
- ☐ Shortness of breath

- ☐ New confusion
- ☐ Caregiver or home-safety concerns

Then ask: "Based on what you are seeing, does my parent meet criteria for home health nursing, physical therapy, occupational therapy, speech therapy, social work, or other skilled services?" The goal is not to demand a service. It is to give the clinician enough information to evaluate whether home health is medically appropriate.

What "Homebound" Means in Plain English

Families often hear "homebound" and think it means the person can never leave the house. In plain language, homebound means leaving home is difficult, taxing, unsafe, or requires help, equipment, or considerable effort. A person may still leave for medical appointments or occasional reasons and still be considered homebound if they otherwise meet the requirements. Do not try to decide this alone — describe what is happening ("She needs help leaving the house," "He uses a walker and gets exhausted," "She cannot manage stairs safely," "Leaving home causes significant weakness or shortness of breath") and let the clinician evaluate it.

When Home Health Is Not Enough — or Not Ordered

Home health alone may not be enough when the person cannot safely be alone, dementia causes wandering, medications require daily supervision, the caregiver cannot sleep, falls continue despite therapy, or daily bathing, dressing, and toileting are unsafe. When it is not enough, the family may need private duty care, adult day care, assisted living, memory care, skilled nursing, palliative care, or hospice. That does not mean home health failed — it means the need is bigger than intermittent skilled visits.

Sometimes a clinician says home health is not appropriate — because the person does not meet the requirements (no skilled need, not homebound), or needs a different kind of care. If so, ask two questions: "What criteria are not being met?" and "What support do you recommend instead?" The answer "not home health" should not end the conversation. It should lead to the next question — outpatient therapy, private duty care, adult day care, a medication review, a specialist, a care manager, or a palliative or hospice evaluation if appropriate.

WHAT TO DO THIS WEEK

1. **Write down what changed** — use facts: "Mom fell twice in two weeks," "He gets short of breath walking to the bathroom."

2. **Decide whether there may be a skilled need** — fall risk, weakness, wounds, therapy, medication or disease teaching, caregiver teaching, or home safety.
3. **Ask for a provider evaluation** using the script in this chapter — not "order home health," but "determine whether home health is appropriate."
4. **Bring your notes** — fall log, medication list, discharge papers, symptoms, and safety concerns.
5. **Ask what services may be appropriate** — nursing, PT, OT, speech, social work, or aide as part of the plan.
6. **Make the home ready** — clear walkways, gather medications, and show the clinician the bathroom, bedroom, stairs, and walking paths.

■ **Worksheet — Home Health Evaluation** — fill it in from the printable companion workbook
 · TheCardinalsPromise.com/resources

■ QUESTIONS FOR THE PROVIDER

1. Does my parent meet criteria for home health, and is there a skilled need?
2. Is my parent considered homebound, and was the required face-to-face encounter completed or scheduled?
3. Would nursing, physical therapy, occupational therapy, speech therapy, or social work help?
4. Would a home health aide be part of the care plan?
5. What documentation is needed?
6. What should we do if home health is not appropriate — what criteria are not being met?
7. What support is needed beyond home health?

■ HOME HEALTH SCRIPT

Calling the provider's office: "My parent has had a recent change in function and safety. We are seeing concerns with _____. Can we schedule an appointment or provider evaluation to determine whether home health is appropriate and whether the required documentation can be completed if they meet criteria?"

At the visit: "Based on what you are seeing today, does my parent meet criteria for home health services such as nursing, PT, OT, speech, social work, or aide support as part of the care plan?"

If the answer is no: "What criteria are not being met, and what support would you recommend instead?"

ROB'S NOTE

Home health can be one of the best services a family gets. But it only helps when the family knows what it is.

I have seen families feel let down. They thought someone would stay for hours, cook meals, watch over a person with dementia, and take over the whole day. That is not what home health does.

I have also seen families miss how much home health can help. They didn't know how much a nurse, therapist, or social worker could see once inside the home. The doctor's office sees a patient sit in a chair for fifteen minutes. Home health sees more. It sees the narrow hallway. The bathroom with no grab bars. The pill bottles on the counter. A caregiver lifting the wrong way. Stairs that are now too hard to climb.

A home can look fine at first. But it can be unsafe for someone who is weak, confused, short of breath, or healing. Home health is not the whole answer. But it can be the first trained person to spot the problem. That helps a family figure out what to do next.

CHAPTER 6 TAKEAWAY

Home health is medical care in the home for people who meet the requirements — not private duty care, housekeeping, or all-day supervision. The next right question is: **can we schedule a provider evaluation to determine whether home health is appropriate, and what support is needed if it is not?**

CARDINAL MOMENT



A daughter thought she had just two choices. She could quit her job to care for her dad. Or she could move him into a place he did not want to go. In her head, she was already writing her goodbye letter to work. No one had told her something important. After his time in the hospital, Medicare would send a nurse and a physical therapist to his house for a while. And it would cost them nothing.

Home health gave her six weeks. It was not forever, and it was never meant to be. But it gave her time. She got to keep her paycheck. And her dad got to stay in his own kitchen a little longer.

Sometimes the help you need most is the help no one thinks to mention. So ask the hospital in plain words: "Does he qualify for home health?" Make them answer before he goes home.

A composite drawn from years at the bedside. Names and details changed.

Private Duty Care

THIS CHAPTER IN 30 SECONDS

- **What it is:** non-medical help in the home, usually longer blocks of hands-on help and supervision — may include bathing, dressing, meals, light housekeeping, companionship, transportation, errands, and safety checks.
- **What it is not:** medical care. Home health is medical, intermittent, and ordered when a person meets the requirements (Chapter 6) — a family may need both.
- **Who pays:** often paid privately — long-term care insurance, VA benefits, or certain Medicaid programs may help; Medicare generally does not pay for long-term custodial private duty care.
- **How to introduce it:** soft, practical, time-limited — *"Let's try it for two weeks and see what helps"* — and if they refuse: *"I understand. This is also to help me — I need some support so I can keep doing this well."*
- **The real question:** not "Are we ready for caregivers?" but what tasks are no longer safe or realistic for the family to handle alone?

WHAT FAMILIES USUALLY THINK

Families often think private duty care means giving up. "We're not ready for strangers in the house." "Mom will never allow it." "Medicare should cover this." "We promised we'd keep her home." These concerns are common — care that enters the home and touches daily life can feel personal. But help at home is not failure. Sometimes it is the support that makes staying home possible. A caregiver coming in for a few hours may prevent a crisis, protect the main caregiver, and give the family enough breathing room to make better decisions.

The Yardstick: Activities of Daily Living

Professionals measure how much help a person needs using "activities of daily living" (ADLs) — the basic self-care tasks: bathing, dressing, eating, toileting, transferring in and out of a bed or chair, and continence. Alongside them are the "instrumental" activities of daily living (IADLs) — managing medications, handling money, cooking, shopping, using the phone, and getting around the community. Decline often shows up first in the instrumental tasks — a missed bill, a burnt pot — before it reaches the basic ones. Tracking

ADLs and IADLs tells you where your parent actually stands. A steady rise in how much help they need signals that more in-home support, a different living arrangement, or a higher level of care may be needed soon.

Help only as much as needed. The instinct to take over completely is understandable, but doing too much can accelerate decline and wound dignity. If your parent can still button their own shirt slowly, let them, even when it would be faster to do it for them. Reserve your help for what they truly cannot do safely — and protect dignity most carefully in bathing and toileting, where resistance is most common, especially with dementia. Go slowly, explain what you are doing, preserve privacy, and stay calm. A matter-of-fact, respectful manner does more than any technique.

What It May Include — and What It Usually Does Not

Private duty care may include bathing, dressing, grooming, toileting and incontinence care, meal preparation, light housekeeping, laundry, transportation, errands, companionship, dementia supervision, medication reminders (depending on state rules and agency policy), overnight care, and respite for family caregivers. Services vary by agency, caregiver training, and state rules. Always ask what the agency can and cannot do — do not assume.

It usually does not include skilled medical care unless the agency specifically provides licensed nursing. Many private duty caregivers cannot give injections, manage complex wounds, adjust medications, provide skilled nursing assessments, or replace a doctor, nurse, therapist, or hospice team. Some agencies offer licensed nursing separately; some do not. Ask directly.

Private Duty Care vs. Home Health

Families often mix these up (Chapter 6 draws the full distinction), and a family may need both — home health physical therapy helps Dad regain strength after a hospital stay, while private duty care helps him shower safely, eat lunch, and avoid being alone during the day. One supports medical recovery; the other supports daily life.

HOME HEALTH	PRIVATE DUTY CARE
Medical and intermittent	Non-medical daily help and supervision
Nursing, PT, OT, speech, social work, limited aide	Bathing, meals, errands, companionship, overnight care
Requires eligibility and clinical documentation	Often private pay; can be scheduled for longer blocks

Not long blocks of daily supervision	Can cover mornings, evenings, overnight, and respite
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When Private Duty Care May Help

It may help when the person needs help with bathing, dressing, or meals; needs supervision or transportation; should not be alone for long; is at risk for falls; has dementia or wanders; forgets meals or medications; is lonely; needs overnight support; is coming home after a hospital or rehab stay; or has a caregiver who needs relief. It may also help when the family is deciding whether home is still realistic — a few weeks of help can reveal whether the person can manage with support, whether the caregiver is sleeping, whether falls are reduced, and whether the cost is sustainable. Private duty care can be a bridge. Sometimes it becomes the long-term plan; sometimes it reveals that more care is needed.

How Much Care Is Needed?

Families often underestimate, starting with "Can someone come once a week?" when the real need is daily bathing help, meal setup, medication reminders, and supervision during unsafe hours. Match the schedule to the risk, not to the hope that everything will be fine. Ask when risk is highest — morning (getting up, bathing, dressing, medications), midday (missed meals, loneliness, wandering, falls), evening (worsening confusion, unsafe cooking), and overnight (getting up, wandering, toileting).

Cost and Who Pays

Private duty care can become expensive, so families need clear numbers. Ask about:

- ☐ the hourly rate
- ☐ minimum shift length
- ☐ weekend and holiday rates
- ☐ overnight billing
- ☐ deposits and fees
- ☐ cancellation policy
- ☐ what happens if a caregiver calls out

It is often paid privately — from savings, retirement income, family contributions, long-term care insurance, certain VA benefits, or some Medicaid waiver or state programs depending on eligibility. **Medicare generally does not pay for long-term custodial private duty care.** This is one of the most important points: if the need is daily

supervision, bathing, meals, and companionship, do not assume Medicare will cover it. Ask specific questions before building the plan.

Choosing an Agency — or Hiring Privately

A good agency is more than a warm body in the house. Ask about:

- ☐ licensing
- ☐ background checks
- ☐ training
- ☐ dementia and fall-prevention experience
- ☐ caregiver supervision
- ☐ backup coverage
- ☐ care plans
- ☐ documentation
- ☐ medication and transportation policies
- ☐ minimum hours
- ☐ rates
- ☐ how caregiver matching and replacement work

The cheapest option is not always the safest, and the most expensive is not automatically the best — look for reliability, communication, supervision, and fit.

Some families hire privately instead. That may lower cost and offer flexibility, but it can also mean no backup if the caregiver is sick, payroll and tax responsibilities, liability concerns, less supervision, and difficulty finding a replacement. A friendly arrangement can become complicated once money, bathing help, medications, keys, and family trust are involved. When in doubt, ask an elder law attorney, accountant, or care manager what risks to consider.

Introducing Help When a Parent Refuses

Many parents refuse at first — "I don't need a babysitter," "I don't want strangers here." Do not begin with the word "caregiver" if it creates resistance. Try softer, practical language: "Someone is coming Tuesday to help with lunch and laundry" instead of "We're hiring a caregiver"; "We're trying extra help for two mornings so everyone feels safer" instead of "You can't be alone"; "Let's try it for two weeks and see what helps" instead of "This is permanent." The first goal may not be full acceptance. The first goal may be getting through the door.

When Private Duty Care Is Not Enough

It may not be enough when the person needs two-person transfers, is unsafe even with scheduled visits, has severe dementia behaviors or frequent wandering, needs twenty-four-hour supervision that is not financially sustainable at home, or when the caregiver cannot continue despite the help. At that point, families may need to consider assisted living, memory care, skilled nursing, palliative care, or hospice if appropriate. Changing the plan does not mean private duty care failed. It means the needs changed or became clearer.

WHAT TO DO THIS WEEK

1. **List the daily tasks that are no longer safe or realistic** — bathing, meals, transportation, supervision, overnight safety, caregiver respite.
2. **Identify the highest-risk times of day** — morning, midday, evening, overnight, weekends, shower days.
3. **Decide how many hours to try first** — often a few hours a few days per week to start.
4. **Call at least three agencies** and compare rates, minimums, training, and backup coverage.
5. **Decide who will manage the schedule** and name a backup.
6. **Review after two weeks** — is the parent safer, is the caregiver getting relief, are more or fewer hours needed?

■ **Worksheet — Private Duty Care Planning** — fill it in from the printable companion workbook · TheCardinalsPromise.com/resources

■ QUESTIONS TO ASK A PRIVATE DUTY AGENCY

1. Are you licensed or registered as required in this state, and are caregivers employees or contractors?
2. Do you perform background checks, and what training do caregivers receive (including dementia)?
3. Can caregivers help with bathing, toileting, transportation, and medication reminders — and what are they not allowed to do?
4. What is the minimum shift length, the hourly rate, and are nights, weekends, or holidays billed differently?
5. What happens if the caregiver calls out, and can we request a different caregiver if needed?
6. Who supervises the caregiver, and how do you communicate with the family?

7. Do you help with long-term care insurance paperwork, and how quickly can care start?

■ HOME SETUP NOTES FOR THE CAREGIVER

Emergency contact _____

Primary doctor / preferred hospital _____

Pharmacy _____

Allergies _____

Mobility & bathroom safety notes _____

Meal preferences _____

Dementia / behavior notes _____

Things that calm the person _____

Things that upset the person _____

House rules _____

First-week care plan

TASK	PERSON RESPONSIBLE	DATE
List care needs & call agencies		
Compare costs & review budget		
Choose agency & schedule first shift		
Prepare home notes & update family		
Review after first week		

Two-week reviewIs the current plan working? Yes ☐ No ☐ Somewhat ☐ Unsure ☐**What needs to change?**☐ More hours☐ More family help or supervision☐ Fewer hours☐ Assisted living should be explored☐ Different caregiver or agency☐ Memory care should be explored

Final decision — agency chosen & start date _____

Schedule _____

Main goal of private duty care _____

Next review date _____

■ INTRODUCTION SCRIPT — WHEN A PARENT RESISTS

"We are going to try having someone come in to help with _____. This is not about taking over. It is about making the day easier and safer. We'll try it for _____ and then see what is working."

If they say, "I don't need help," try: "I understand. This is also to help me — I need some support so I can keep doing this well."

If they say, "I don't want a stranger," try: "That makes sense. We'll meet them together, and if it's not a good fit, we'll speak up."

■ ROB'S NOTE

Private duty care can be hard for families. It can feel like letting a stranger in too close.

Before this, the family may have done all the care. Then all at once, someone from outside is helping. They help with showers, meals, laundry, and the private parts of daily life. That can feel strange.

But I have seen private duty care give families a break. The daughter who never sleeps gets a morning off. The husband or wife who is scared to leave can go to the doctor. The parent who kept skipping meals starts eating lunch again.

It is not always perfect. The first caregiver may not be the right fit. The cost may be more than you thought. But asking for help is not a failure to love. It can keep love from turning into burnout. The right question is not, "Should we be able to do this on our own?" It is, "What help would make this safer for everyone?"

■ CHAPTER 7 TAKEAWAY

Private duty care is daily support, supervision, and non-medical help in the home. The next right question is: **what tasks are no longer safe or realistic for the family to carry alone?**

CARDINAL MOMENT



A family hired four hours of help each day. They felt a quiet shame about it. They thought their love should have been enough by itself. For weeks, they kept saying sorry to each other for needing help.

The aide's name was Denise. She learned that their father liked his coffee with two sugars. She learned he liked the ballgame on low. Within a month, he called her by name. He even told her the same three stories he only told people he trusted.

Bringing in a stranger did not replace the family. It gave the family back to him. Now they were rested. Now they were glad to see him again, instead of worn out by him.

A composite drawn from years at the bedside. Names and details changed.

Assisted Living

Margaret's daughter didn't decide anything the day of the fall. She decided three weeks later, standing in her mother's kitchen at 7 a.m., counting pill bottles and realizing she'd driven over before work every day that month. Her mother hadn't eaten a real dinner in days. The neighbor had found her outside at dusk, looking for a dog that died in 2019. The daughter wasn't failing. She was doing the work of three people, and there was still only one of her. (*A composite, drawn from years around senior living — details changed.*)

That morning is what this chapter is for. Not the fall. The kitchen, three weeks after.

What You Need to Understand

Assisted living is a residential care setting — not a nursing home, not home health. Meals, medications, bathing, laundry, safety checks, staff around the clock. Many communities also have a separate, secured memory care wing if dementia is part of the picture (Chapter 9).

Two beliefs stop families at the door, and both are usually wrong. The first is that a move means giving up. For many families it means more safety, more connection, staff at 2 a.m. — and it gives the family back its relationship. The daughter gets to go back to being a daughter instead of a full-time nurse.

The second is that Medicare pays for it. It almost never does. Assisted living is generally private pay, though long-term care insurance, certain VA benefits, and some state programs can help (Part 3 shows you how families actually pay).

The question that tells you whether it's time isn't on any brochure: **"Who is with them at the key times?"** If your family can't answer that, the answer is the conversation.

What Families Commonly Miss

Do your research. The tour shows you the lobby — but the staffing, the medication process, and the fall response are what you're actually buying, and none of them are on the tour unless you ask.

And the price on the website is not the price. The base rent is where the cost *starts*. Care is priced in levels on top of it, and the level can change after move-in — usually at the first

care reassessment.

\$ THIS COULD SAVE YOU THOUSANDS

The base rent gets all the attention. The **care level cost** is what breaks budgets. Before you sign anything, ask three questions and get the answers in writing: What level is she today, and what does it cost? What would move her to the next level? What does *that* cost? A community quoting \$4,200 a month at “Level 1” can honestly become \$5,600 at Level 3 — and the family that never asked finds out at the reassessment, after the furniture is moved in. Get the numbers in writing before you fall in love with a place.

The Solution

Work in this order, and the decision gets simpler.

Needs first, tours second. Be honest about bathing, transfers, medication, nighttime, memory. Assisted living meets many needs, not all — two-person transfers, frequent wandering, or complex medical care may mean memory care or skilled nursing instead. Ask each community what they *cannot* manage and what would force a move-out later. A community that answers that honestly is telling you something good about itself.

Tour three if you can, with your eyes open. Watch how staff treat residents. Ask to see care areas, not just the dining room. Notice whether call buttons get answered. Red flags: vague answers about staffing or cost, pressure to sign quickly. Green flags: staff who greet residents by name, costs explained in writing, honest talk about limits.

Then bring it to the family (Chapter 5), compare in writing, and decide the next step — not the whole future.

WHAT NOBODY TELLS YOU

Tour at mealtime or shift change — that’s when you see the real staffing, not the brochure’s version. And more is negotiable than families assume: the one-time community fee and the first months’ rate often move, especially when a building has open apartments. You will never get a discount you didn’t ask about.

WHO TO CALL

- **Not sure what level of care fits** — a local placement advisor, or **Cardinal Care Bridge** at TheCardinalsPromise.com — no cost to you
- **Paying for it** — an elder law attorney (protecting assets, Medicaid timelines — Chapter 14); your SHIP office for insurance questions (Chapter 13)
- **Veteran or surviving spouse** — a Veterans Service Officer about Aid & Attendance (Chapter 18)
- **Suspect it's more than assisted living can handle** — ask the doctor for a care-needs evaluation before you tour

FROM SOMEONE WHO'S BEEN THERE

“[Reserved for a real, permissioned family quote.]”

— Name, relationship · used with permission

Need the comparison sheet? The one-page tour worksheet — five questions, three communities, the care-level trap built in — is in the printable companion workbook.

Your One Next Move

Tonight, answer one question honestly: **who is with them at the key times?** If the answer is “no one” or “only me” — call one community tomorrow and book a tour. Not a commitment. A look.

**KEEP GOING**

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TheCardinalsPromise.com/resources

CARDINAL MOMENT



For years, a mother swore she would never go into "one of those places." Her daughter went and toured seven of them anyway, quietly. She kept looking until she found one that did not smell like anything bad. It even had a piano that someone actually sat down and played.

Her mother cried on move-in day. Her daughter drove home sure she had made a terrible mistake. A month later, her mother cried again. But this time it was because she had made a friend at dinner. She had not expected to have a life again.

The fear is almost always about the idea, not the real place. So go tour a real one. The right building is not an ending. Sometimes it is the first company she has had in years.

A composite drawn from years at the bedside. Names and details changed.

Memory Care

THIS CHAPTER IN 30 SECONDS

- **What it is:** a residential setting for people with dementia or Alzheimer's who need more structure, supervision, and safety than regular assisted living can provide.
- **What it is not:** not a punishment, not "locking someone away," not giving up — and not measured by whether the person still recognizes family or has moments of clarity.
- **When to consider it:** a pattern of risk — usually not one bad day — wandering, unsafe stove or medication, falls, nighttime confusion, refusing care, caregiver exhaustion. Write down the pattern.
- **The one question:** is this person unsafe because they cannot remember how to stay safe? If yes, memory care belongs in the conversation.
- **How to say it:** not "We have to put you in memory care" — say "*We are looking at a place with more support and a safer routine.*"

WHAT FAMILIES USUALLY THINK

Families often think memory care means the end. "Mom is not that bad." "Dad still knows who I am." "She can still have a normal conversation." But dementia is not measured only by whether someone recognizes family. A person can smile, tell old stories, and recognize loved ones while also being unsafe with medication, cooking, wandering, driving, bathing, or nighttime confusion. Memory care is not about whether the person still has moments of clarity. It is about whether the current setting can safely support them during the moments when clarity disappears.

Memory Care vs. Assisted Living

Assisted living and memory care may be in the same building, but they are not the same level of support. Assisted living may work when a person needs help with meals, bathing, dressing, medications, and general safety checks but can still live safely in a less secure setting. Memory care may be needed when dementia creates safety risks that require more structure — secured or monitored exits, dementia-trained staff, more frequent cueing, support for wandering, and calm redirection for confusion or agitation. Ask one question:

is this person unsafe because they cannot remember how to stay safe? If yes, memory care belongs in the conversation.

When Memory Care May Be Needed

Consider memory care when the person:

- ☐ wanders or tries to leave
- ☐ gets lost
- ☐ is unsafe with the stove or medication
- ☐ falls or forgets the walker
- ☐ cannot reliably call for help
- ☐ is confused or frightened at night
- ☐ becomes suspicious or agitated
- ☐ refuses bathing or medication
- ☐ needs supervision throughout the day
- ☐ or when assisted living or home care is no longer enough and the caregiver can no longer safely continue

Memory care is usually not about one bad day. It is about a pattern of risk. Write down the pattern. Families make better decisions when they can see what is actually happening.

The Wandering Risk

Wandering is one of the clearest reasons to consider memory care, and it does not always look dramatic at first:

- ☐ walking outside "just for a minute"
- ☐ trying to go home while already home
- ☐ leaving at night
- ☐ looking for a deceased spouse
- ☐ exiting during appointments
- ☐ becoming restless in the evening

A person who wanders may not understand danger; they may be looking for something that feels real to them. Arguing usually does not fix it, and instructions may not be remembered ten minutes later. If wandering is happening, ask whether the person can safely be alone, whether the caregiver can sleep, and whether a secured memory care setting is needed. Wandering turns a home into a watchtower — families need to be honest about whether they can keep watch.

Nighttime, Behavior, and Caregiver Collapse

Nighttime often reveals how unsafe the plan has become — getting up repeatedly, wandering, falling on the way to the bathroom, not recognizing home, or the caregiver no longer sleeping. Dementia can also change mood and behavior: agitation, suspicion, hallucinations, refusal of care, or fear of being alone. Behavior is communication and can have treatable causes — pain, infection, constipation, dehydration, poor sleep — so report sudden changes to a clinician and do not assume everything is "just dementia." And memory care is sometimes needed not only because the person is declining but because the caregiver is. A care plan that depends on one exhausted caregiver staying awake forever is not a plan. It is a fuse.

How to Talk About It

Families often avoid the phrase "memory care" because it feels harsh. Lead with honesty and gentleness. Instead of "We have to put you in memory care," try "We are looking at a place with more support and a safer routine." Instead of "You cannot live alone anymore," try "We are worried the current setup is not keeping you safe." With dementia, one conversation may not settle it — you may need repetition, reassurance, and professional support. Do not turn every conversation into a courtroom. The goal is care, not winning the argument.

Tour Beyond the Locked Door

A secured door is the easy part of memory care. What matters is the quality of care inside it. On a tour, ask how they manage wandering and exit-seeking, how they handle agitation and refusal of care, how medications and falls are managed, how families are updated, and what care needs would require a transfer out. Watch how staff speak to residents: do they rush, correct harshly, or redirect calmly? Do they know names? Green flags include warm, name-by-name greetings, a calm and structured environment, a safe supervised outdoor space, dementia-adapted activities, and staff who can clearly explain how they handle the hard days. A beautiful building does not guarantee good dementia care. The care culture does.

WHAT TO DO THIS WEEK

1. **Write down the safety risks** — wandering, unsafe cooking or medication, falls, nighttime confusion, refusing care, caregiver exhaustion.
2. **Ask whether assisted living is enough** or whether dementia-related risks need a secured setting.
3. **Tour more than one memory care community** and ask about the hard days, not just the lobby.

4. **Talk with the doctor or neurologist** about sudden changes and treatable causes.
5. **Hold a family meeting** (Chapter 5) focused on safety, caregiver capacity, and cost.
6. **Decide the next step** — more supervision, private duty, adult day care, assisted living, memory care, or a provider evaluation.

■ **Worksheet — Memory Care Decision** — fill it in from the printable companion workbook · TheCardinalsPromise.com/resources

■ QUESTIONS TO ASK MEMORY CARE

1. How do you manage wandering or exit-seeking, and are exits secured or monitored?
2. What dementia training do staff receive, and how many are on during the day, evening, and overnight?
3. How do you handle agitation, fear, aggression, or refusal of bathing and medication — what non-drug approaches do you try first, and what is your approach to antipsychotic medications?
4. How are medications and falls managed, and how quickly is family notified of a change?
5. How are activities adapted for different stages of dementia?
6. How do you learn the resident's routines, history, triggers, and preferences?
7. What happens if care needs increase, and what would require a transfer or discharge?

Decision summary

Main reasons memory care may be needed _____

Main concerns _____

Questions still unanswered _____

Next step / person responsible / due date _____

■ CONVERSATION SCRIPT

With family: "We need to talk about memory care not because we are giving up, but because dementia is creating safety concerns. The question is whether the current setting can still protect safety, dignity, and the caregiver."

With the person, if appropriate: "We are looking at a place with more support and a safer routine. The goal is not to take your life away. It is to make sure your days are safer, calmer, and better supported."

ROB'S NOTE

Memory care is one of the hardest tasks a family can have. It feels like a door that only opens one way.

Families often wait, because they want to respect the person they love. I get that. But dementia changes the rules bit by bit. At first, the family helps with small things. Then they manage the pills. Then they hide the car keys. Then they sleep with one ear open. Then they are scared to shower, because the person might walk out the door.

No one planned for this. Memory care should never be treated lightly. But it is not a failure either.

Sometimes the most loving thing a family can say is, "This is bigger than what we can safely handle at home." That sentence hurts. It can also keep everyone safe. The goal is not to save the old plan. The goal is to care for the person in front of you right now.

CHAPTER 9 TAKEAWAY

Memory care may be needed when dementia creates safety risks that cannot be reliably managed at home or in regular assisted living. The next right question is: **is the current setting safe during the moments when memory cannot protect them?**

CARDINAL MOMENT



A husband kept his wife at home two years longer than was safe. Moving her felt like breaking the promise he had made to her. He propped chairs against the doors and slept in his clothes.

One night in February, she got out anyway. He found her shaking on the neighbor's steps. Something in him finally let go. Memory care did the one thing his love could not do alone. It locked the doors, so he could go back to being her husband instead of her guard.

Choosing more help is not choosing less love. Sometimes it is the most loving thing left to do.

A composite drawn from years at the bedside. Names and details changed.

Skilled Nursing and Rehab

THIS CHAPTER IN 30 SECONDS

- **What it is:** short-term medical care, nursing, and therapy after a hospital stay when a person is not ready to safely return home — a decision point, not just a place to "get stronger."
- **Inpatient vs. observation:** before the skilled nursing stay, ask whether the hospital stay was inpatient or observation — it can decide Medicare coverage (Chapter 13). And Medicare generally does not pay for long-term custodial nursing home care.
- **The money clock:** with a qualifying stay, Medicare covers a skilled stay on a clock:
 - **Days 1–20:** Medicare pays in full.
 - **Days 21–100:** a daily coinsurance (\$217 in 2026 — confirm at [medicare.gov](https://www.medicare.gov)).
 - **After day 100:** Medicare pays nothing.

Start the "what happens after" conversation early (Chapters 13–15).

- **The "plateaued" trap:** under the Jimmo settlement, Medicare coverage does not legally require the person to keep improving — skilled care to maintain function or to prevent or slow decline can also qualify. Ask for the written notice and ask how to appeal.
- **The fast appeal:** if coverage or the stay is ending and you believe it is unsafe, Medicare patients have a formal fast-appeal right through the Quality Improvement Organization (QIO) — the number is on the written notice, and the clock is short, so act the same day. Lost the notice? Find your QIO at qioprogram.org ("Locate Your QIO") or call 1-800-MEDICARE.
- **How to ask:** request a care conference early and get plain language — *"What does that mean at home at 2 a.m. when one person is helping?"*

WHAT FAMILIES USUALLY THINK

Families often think, "Rehab will get Mom back to normal," "Dad will stay until he is ready," "Medicare will cover everything," or "They cannot discharge her if we are not ready." Sometimes rehab helps a person recover well. But not every person returns to their prior level. Older adults can lose strength quickly during illness, hospitalization, bedrest, or confusion, and a person may improve and still need more help than before. Families get into

trouble when they assume rehab will solve the whole situation. Rehab may answer important questions — but it may not give the answer the family hoped for.

What Rehab May Provide — and What It Does Not Mean

A skilled nursing or rehab facility may provide:

- ☐ nursing care
- ☐ medication management
- ☐ physical, occupational, and speech therapy
- ☐ wound care
- ☐ monitoring after illness or surgery
- ☐ help with bathing, dressing, toileting, and transfers
- ☐ meals
- ☐ discharge planning
- ☐ social work
- ☐ equipment recommendations
- ☐ caregiver training
- ☐ coordination with doctors

The exact services depend on the person's needs, the facility, insurance coverage, and the care plan. Ask what is being provided — do not assume.

Rehab does not always mean the person will return home independent. It does not guarantee daily progress, a long stay, or that insurance will keep paying, and it does not replace family planning or automatically solve dementia, frailty, advanced illness, caregiver burnout, or unsafe housing. Begin discharge planning early. That conversation should not start the day before discharge.

Short-Term Rehab vs. Long-Term Care

This distinction matters. Short-term rehab is usually focused on recovery after a hospital stay, surgery, illness, or fall; the goal is improvement, and the stay may be covered by insurance if requirements are met (coverage depends on the payer, medical need, eligibility, and plan rules). One protection families should know: **Medicare coverage does not legally require the person to keep improving.** Under the *Jimmo* settlement, skilled care to maintain function or to prevent or slow decline can also qualify. If you are told coverage is ending because the person has "plateaued," ask for the written notice and ask how to appeal. Long-term care is custodial or ongoing care for someone who cannot safely live alone over time; it is often paid differently, and **Medicare generally does not**

pay for long-term custodial nursing home care. Ask early: "Is this stay considered short-term rehab, long-term care, or something else?" That one question can prevent a lot of confusion.

Two more protections. Before the skilled nursing stay, ask whether the hospital stay was **inpatient or observation** — it can decide Medicare coverage (Chapter 13 explains the trapdoor). And when you are told coverage or a stay is ending and you believe it is unsafe, Medicare patients have a formal fast-appeal right through your Medicare Quality Improvement Organization (QIO) — the written notice the facility hands you includes the phone number, and the appeal clock is short, so act the same day.

Rehab Is a Handoff, Not a Holding Place

Families sometimes think rehab buys time to avoid the next decision, but time moves quickly and discharge planning begins almost immediately. During rehab, ask: What could the person do before the hospital, and what can they do now? Are they improving and participating in therapy? What help do they need with bathing, dressing, toileting, walking, and transfers? Can they safely return home, and what equipment will be needed? Will home health be appropriate after discharge? Will private duty care be needed? Is assisted living, memory care, or long-term care more realistic? Is palliative care or hospice appropriate? A rehab stay should give the family information. Use it.

Therapy and Nursing: What to Ask

Ask the therapy team how far the person can walk and whether it is independent, with supervision, or hands-on; whether they need a walker, cane, or wheelchair; whether they can get in and out of bed, transfer to a chair, get on and off the toilet, manage stairs, and bathe and dress safely; whether they are improving; and what would make home unsafe. Ask for plain language. Not just "She is min assist," but **"What does that mean at home at 2 a.m. when one person is helping?"** That is the real question.

Ask the nursing team what medical needs are being managed, whether medications are stable, whether there is wound care, pain, confusion, incontinence, poor eating, or shortness of breath, and what needs to be monitored after discharge. Nursing sees things therapy may not — therapy knows how the person walks, but nursing may know whether they are confused at night, refusing care, or eating poorly. Both views matter.

Social Work, Discharge Planning, and the Care Conference

The discharge planner or social worker is your key contact. Ask about:

- ☐ the expected discharge date

- ☐ the recommended next level of care
- ☐ what equipment is needed
- ☐ whether home health will be arranged if appropriate
- ☐ whether private duty care is needed
- ☐ what safety concerns exist about returning home
- ☐ what documents need signing
- ☐ what the options are if home is not safe — including what happens if the family disagrees with the discharge plan

Do not wait for the discharge planner to call you. Ask for a care conference.

A care conference is a meeting of the rehab team — nursing, therapy, social work, sometimes a dietitian — with the family and the patient if appropriate. Ask for one early, and use it to learn what has and has not improved, what help the person still needs, the discharge goal and expected date, what setting is safest next, and what training the family needs before discharge. The care conference should produce a plan, not just reassurance.

When Going Home May Not Be Safe

Going home may not be safe if the person cannot transfer safely or get to the bathroom, falls or is at high risk for falls, needs more help than one caregiver can provide, needs two-person assistance, or lives in a home with stairs that cannot be managed. It may also be unsafe if dementia creates supervision needs that cannot be met at home, if medications or wounds require skilled care the family cannot provide, if the person is not eating or drinking safely, or if the caregiver cannot physically or safely continue. If home is not safe, that is not a failure — it is information. It means the next setting needs to match the care the person actually needs now.

WHAT TO DO THIS WEEK

1. **Ask whether this is short-term rehab or long-term care** and how it is being paid for.
2. **Request a care conference early** — do not wait for the discharge call.
3. **Get plain-language answers** from therapy and nursing about walking, transfers, toileting, bathing, and what home would require.
4. **Ask what happens after discharge** — home health evaluation, private duty care, equipment, and monitoring.
5. **Compare the recommended setting honestly** — home, assisted living, memory care, or long-term care.
6. **Hold a family meeting** (Chapter 5) before discharge, not after.

■ **Worksheet — Rehab & Discharge Planning** — fill it in from the printable companion workbook · TheCardinalsPromise.com/resources

■ QUESTIONS TO ASK BEFORE DISCHARGE

1. Is this stay short-term rehab or long-term care, and how is it paid for?
2. Can my parent transfer, toilet, walk, and bathe safely enough for the planned setting?
3. What would make home unsafe, and what would we need to make it work?
4. Will home health be evaluated, and what skilled needs qualify?
5. What medications, wounds, or conditions must be monitored after discharge?
6. What should we watch for, and who do we call if things get worse?
7. What happens if we do not feel the discharge is safe?

ROB'S NOTE

Rehab is one of the most misread moments in the whole journey.

Families hear the word "rehab" and picture their mother walking back through the front door the way she left. Sometimes that happens. Often it does not.

I have sat with families on day three of a rehab stay who were already being asked about going home. They had not even caught their breath from the hospital. That is why I tell people: rehab is not a pause. It is a countdown. Ask for the care meeting early. Ask the therapists what "min assist" means at 2 a.m. Ask the nurses what the nights are like.

The goal is not to rush the person home to prove the crisis is over. The goal is to send them to a place that can actually keep them safe. Sometimes that is home with help. Sometimes it is not. Either way, you want to be the family that asked the hard questions early. You do not want to be the one handed a go-home date with nowhere safe to go.

CHAPTER 10 TAKEAWAY

Rehab is a decision point, not just a place to get stronger. The next right question is: **what level of care will actually be safe after rehab ends?**

CARDINAL MOMENT



A man went to rehab after his stroke "for a week or two." The whole family thought he would be home by the end of the month. But progress came slower than the movies had promised. Everyone, including him, started to lose hope.

What turned it around was not pushing harder. It was showing up. They sat in on therapy. They learned the exercises. They cheered the small wins out loud: a spoon lifted, a word that came back, a step without the rail.

Rehab is not a machine you drop someone into and pick them up from later. It works best when the family is in the room, counting each inch right beside them.

A composite drawn from years at the bedside. Names and details changed.

Palliative Care

THIS CHAPTER IN 30 SECONDS

- **What it is:** specialized support for serious illness — symptoms, stress, goals of care, and quality of life — that can often begin earlier and be used alongside treatment.
- **What it is not:** not hospice, not stopping treatment, not a signal that death is close — hospice is generally for people nearing the end of life who are no longer pursuing curative treatment for the terminal condition.
- **When to ask:** advanced heart, lung, kidney, or liver disease, cancer, dementia with increasing complications, repeated hospitalizations or ER visits, uncontrolled pain or shortness of breath, or difficult treatment decisions.
- **How to ask:** *"Would palliative care be appropriate to help with comfort, quality of life, goals of care, and family support?"* — and if they say it is not needed, ask what support they recommend for symptom management, caregiver stress, and planning.
- **If the family is unsure:** ask *"Are we talking about palliative care for serious illness support, or hospice care because the illness may be in its final stage?"* That question can clear the room.

WHAT FAMILIES USUALLY THINK

Families often hear "palliative care" and think it means hospice, that death is close, that the doctor is giving up, or that treatment stops. This confusion causes families to miss helpful support. Palliative care is not a punishment or a signal that the family did something wrong, and it is not the same as stopping treatment. It is an extra layer of support for serious illness. Many families do not need fewer doctors — they need someone to help connect the dots and ask: What is the goal now? What symptoms are making life harder? What does the patient understand? What decisions are coming? What matters most if the illness gets worse?

What Palliative Care May Provide

Services vary by setting and program, but may include:

- ☐ symptom management (pain, shortness of breath, nausea, fatigue)
- ☐ emotional support
- ☐ goals-of-care conversations

- ☐ medication review
- ☐ care planning
- ☐ family meetings
- ☐ advance care planning
- ☐ help understanding treatment options
- ☐ coordination with other doctors
- ☐ social work
- ☐ spiritual support if available

Palliative care may be provided in hospitals, clinics, nursing facilities, assisted living communities, or at home, depending on local programs — home-based palliative care is growing quickly, so ask specifically whether a home-based program serves your area.

What Palliative Care Usually Does Not Mean

It does not usually mean the person must stop all treatment, that they must be dying soon, that hospice is automatically starting, that the family failed, that the doctor is abandoning the patient, that comfort is the only goal, or that no more hospital visits or specialists are allowed. Palliative care is not a single decision. It is a support service and a conversation partner that helps families choose well as illness changes.

Palliative Care vs. Hospice

Families need this difference in plain language. Both care about comfort, dignity, communication, and support — what differs is timing and eligibility.

PALLIATIVE CARE	HOSPICE
Can often begin earlier in a serious illness	For those in the last stage who meet eligibility
May be used alongside treatment	Comfort-focused when cure is no longer the goal
Focuses on symptoms, stress, quality of life, goals	A full team and a specific benefit structure

IF THE FAMILY IS UNSURE

Ask: **"Are we talking about palliative care for serious illness support, or hospice care because the illness may be in its final stage?"** That question can clear the room.

When Palliative Care May Help

Consider asking about palliative care with:

- ☐ advanced heart, lung, kidney, or liver disease
- ☐ cancer
- ☐ neurologic illness
- ☐ dementia with increasing complications
- ☐ repeated hospitalizations or ER visits
- ☐ uncontrolled pain or shortness of breath
- ☐ weight loss
- ☐ declining function
- ☐ difficult treatment decisions
- ☐ family disagreement about goals
- ☐ confusion about prognosis
- ☐ concern that treatments are becoming harder than the illness

It may be especially helpful when everyone is working hard but no one is asking what the patient actually wants most.

The Goals-of-Care Conversation

A goals-of-care conversation is not only about medical treatment — it is about what kind of life the person wants as illness changes. Important questions: What does the person understand about the illness, and what does the family understand? What matters most right now? What symptoms are hardest? What is the person hoping for, and what are they afraid of? What treatments feel acceptable, and what feel too burdensome? If time is shorter than the family hoped, what would make the days better? These conversations are hard, but they are how a family makes sure the care matches the person — not just the disease.

WHAT TO DO THIS WEEK

1. **Write down current symptoms** and what helps or worsens each.
2. **List hospital and ER visits** in the last six months and what keeps sending you back.
3. **Ask the provider about palliative care** using the script in this chapter.
4. **Clarify palliative vs. hospice** — ask which one is being recommended and why.
5. **Review advance care documents** — power of attorney, advance directive, DNR/POLST if applicable.
6. **Invite key family members** into the conversation and, if needed, hold a family meeting.

■ **Worksheet — Palliative Care Conversation** — fill it in from the printable companion workbook · TheCardinalsPromise.com/resources

■ QUESTIONS TO ASK THE PALLIATIVE CARE TEAM

1. What symptoms can you help manage, and how can we improve comfort and quality of life?
2. What should we understand about this illness, and what changes should we expect next?
3. Are current treatments helping, or becoming too burdensome?
4. How can we reduce unnecessary crisis trips to the hospital, and who do we call after hours?
5. What support is available for the caregiver, and can you help with advance care planning or a family meeting?
6. Is hospice appropriate now, or should we continue palliative care — and what would tell us it is time to consider hospice?

Advance care planning — documents to find or update

- | | |
|---|--|
| ☐ Healthcare Power of Attorney | ☐ Medication list & doctor list |
| ☐ Living Will / Advance Directive | ☐ Emergency contacts |
| ☐ DNR / MOST / POLST (if applicable) | ☐ Funeral home information (if known) |

■ PALLIATIVE CARE REQUEST SCRIPT

With a doctor or care team: "We are trying to better manage symptoms, stress, and decisions related to this serious illness. Would palliative care be appropriate to help with comfort, quality of life, goals of care, and family support?"

If they say it is not needed: "What support would you recommend for symptom management, caregiver stress, and planning?"

If hospice is mentioned: "Can you explain whether you are recommending hospice now, or palliative care while treatment continues?"

ROB'S NOTE

Palliative care is one of the most misunderstood words in medicine.

Families hear it and brace themselves, like they have just been told the worst. But palliative care is not the end of the road. It is someone walking beside you on the road. They ask the questions no one else has slowed down to ask.

I have watched a palliative care team change a family's whole experience. They did not add another treatment. They finally asked, "What matters most to you now?" Then they built the plan around the answer.

You do not have to be dying to deserve comfort. You do not have to give up to want less pain. Palliative care means taking your symptoms, your stress, and your dignity seriously, while there is still a lot of living to do.

CHAPTER 11 TAKEAWAY

Palliative care is extra support for serious illness — comfort and clarity alongside treatment, not instead of it. The next right question is: **would extra support help us manage symptoms, stress, and decisions?**

CARDINAL MOMENT



A daughter said no to palliative care for months. She was sure it meant giving up, and her mother was still going to every chemo appointment. She thought that accepting comfort was the same as accepting defeat.

When she finally said yes, the palliative nurse just asked what was making the days so hard. The nausea. The sleepless nights. The pain no one had fully treated. Then the nurse fixed what could be fixed. Her mother kept fighting her cancer. And for the first time in months, she also felt like a person again.

Palliative care is not the opposite of hope. It is comfort walking next to the fight, so the fight stays bearable.

A composite drawn from years at the bedside. Names and details changed.

Hospice

THIS CHAPTER IN 30 SECONDS

- **What it is:** comfort-focused care for people who meet eligibility requirements and are nearing the end of life — a full team (nurse, aide, social worker, chaplain, volunteers, medical director) wherever the person lives. It is not giving up.
- **The trade-off:** while on hospice, Medicare generally stops paying for treatment intended to cure the terminal illness — care for other health conditions continues, comfort treatment continues, and the person can leave hospice at any time and return to regular Medicare coverage.
- **What it is not:** usually not a full-time bedside caregiver — the family, and sometimes private duty caregivers, still provide much of the hands-on daily care, so plan that coverage early.
- **Cost and the clock:** under the Medicare Hospice Benefit families usually pay almost nothing (often just a small copay of up to \$5 per comfort prescription); the roughly six-month prognosis is renewable with recertification, though room and board in a facility continues separately.
- **How to ask:** *"Given the changes we are seeing, would a hospice evaluation be appropriate?"* — an evaluation commits you to nothing, and the family always chooses.

WHAT FAMILIES USUALLY THINK

Families often think hospice means death is days away, that all treatment and medication stop, that they are abandoning their loved one, or that choosing hospice means they have failed. None of that is true. Hospice continues care — it simply changes the goal from cure to comfort. One thing does change, and families should know it going in: **while on hospice, Medicare generally stops paying for treatment intended to cure the terminal illness.** Care for other health conditions continues, comfort treatment continues, and the person can leave hospice at any time and return to regular Medicare coverage. Many families later say the same thing: "I wish we had called sooner." They discover that hospice did not shorten the time. It made the time better — the pain controlled, the fear lowered, the family finally able to be family again instead of a frightened medical crew.

What Hospice Provides

Hospice usually provides a team and a plan of care:

- ☐ regular nurse visits for symptom and comfort management
- ☐ aide visits for personal care
- ☐ a social worker for support and planning
- ☐ chaplain or spiritual care if wanted
- ☐ volunteers
- ☐ bereavement support for the family — typically for about 13 months after death

It typically covers medications related to the terminal condition, equipment such as a hospital bed or oxygen, and supplies tied to the plan of care. Hospice can be provided wherever the person lives — at home, in assisted living, in memory care, or in a skilled nursing facility — and inpatient hospice may be available when symptoms require it. Services and visit frequency vary by agency and plan of care, so ask what each agency actually provides.

What Hospice Does Not Mean

Hospice does not usually mean a full-time bedside caregiver. This surprises families. Hospice supports the family and manages comfort, but it does not typically sit at the bedside twenty-four hours a day — the family, and sometimes private duty caregivers, still provide much of the hands-on daily care. Hospice also does not mean the person can never change their mind. If someone improves or wants to pursue treatment again, they can leave hospice. And it does not mean stopping all care — comfort medications, personal care, and support continue. Knowing this early lets a family plan the hands-on coverage they will actually need.

Two Things Families Get Backwards

First, families fear hospice will be a financial burden. Under the Medicare Hospice Benefit, it is usually the opposite. It covers the hospice team, pain and symptom medications, equipment and supplies, short-term inpatient and respite care, and grief support for the family — and most families pay almost nothing, often just a small copay of up to \$5 per comfort prescription. The one real gap is room and board if the person lives in a facility, which is billed separately. Coverage depends on eligibility and the plan, so confirm the specifics — but do not let the fear of cost keep you away. For most families, that fear is exactly backwards.

Second, families think hospice is a one-way door with a hard six-month clock.

Hospice is generally for a prognosis of about six months or less, but it is reversible and renewable: a person can leave hospice and return, and the benefit can continue past six

months with recertification if they remain eligible. There is also a level of care most families never hear about — continuous or "crisis" care, where a nurse can stay at the bedside for hours during a hard stretch of symptoms. If symptoms spike, ask for it by name. It is short-term and criteria-based — the hospice determines whether a crisis level of care is met — so ask what the criteria are when you call.

When to Ask About Hospice

Consider asking for a hospice evaluation when a serious illness is progressing and you are seeing a pattern of decline:

- ☐ more hospitalizations or ER visits
- ☐ more weakness and time in bed
- ☐ more sleeping
- ☐ less eating and drinking
- ☐ weight loss
- ☐ more falls or infections
- ☐ more confusion
- ☐ more shortness of breath
- ☐ wounds that will not heal
- ☐ difficulty swallowing
- ☐ treatment that is no longer helping or has become too hard on the person

Also consider it when the doctors have mentioned comfort care, when a palliative team has raised it, or when the family finds itself quietly asking, "What are we doing this for?" Write down the pattern with specific examples. Patterns, not single bad days, tell the real story.

The Question Families Are Afraid to Ask

THE WORDS THAT HELP

If someone is afraid that hospice means giving up, the words that help are simple: **"We are not trying to give up. We are trying to understand whether comfort-focused support would help now."**

Asking for a hospice evaluation does not commit anyone to anything. It is a conversation and an assessment. The family always chooses. And choosing hospice is not abandoning the person — it is deciding that their comfort, their dignity, and their time matter more than one more hard treatment that may not help.

WHAT TO DO THIS WEEK

1. **Write down the decline with specific examples** — eating, weight, walking, sleeping, breathing, confusion, infections, hospital visits.
2. **Ask the provider for a hospice evaluation** using the script in this chapter — an evaluation commits you to nothing.
3. **Call more than one hospice agency** and compare team, visit frequency, after-hours support, and services.
4. **Plan the hands-on caregiving coverage** — hospice supports the family; it is usually not a full-time bedside caregiver.
5. **Gather the documents** — power of attorney, advance directive, DNR/POLST, medication and doctor lists.
6. **Name a family point person** and set how the family will be kept updated.

■ **Worksheet — Hospice Questions & Decision** — fill it in from the printable companion workbook · TheCardinalsPromise.com/resources

■ HOSPICE EVALUATION & AGENCY QUESTIONS

1. Does my loved one appear hospice appropriate, and for what diagnosis?
2. What decline and symptoms are you seeing, and what would hospice help with most?
3. What treatments would continue, and what medications, equipment, and supplies would hospice provide?
4. How often would the nurse and aide visit, and what support is available after hours?
5. Who do we call in a crisis at night, and what would make inpatient hospice appropriate?
6. What hands-on care is the family still responsible for, and would private duty care still be needed?
7. Can hospice support care at home, assisted living, memory care, or skilled nursing?
8. What happens if my loved one improves, or if we change our mind?

■ HOSPICE REQUEST SCRIPT

With a doctor, facility nurse, or hospice agency: "We are seeing decline and are wondering whether hospice would be appropriate. Can we talk honestly about whether the focus should shift toward comfort, dignity, and support?"

Or simply: "Given the changes we are seeing, would a hospice evaluation be appropriate?"

If family members fear it means giving up: "We are not trying to give up. We are trying to understand whether comfort-focused support would help now."

ROB'S NOTE

I have walked into a lot of rooms after the word "hospice" was finally said out loud.

Almost always, the fear in the room is bigger than the truth. Families think they are signing a surrender. What they are really doing is finally letting someone help.

I have watched pain come down. I have watched a daughter stop being a nurse and go back to being a daughter, holding her father's hand instead of his medication chart. I have watched families say the things they were afraid they would never get to say.

Hospice did not take the time away. It gave the time back — softer, closer, more honest. So if you are standing at that door, afraid to open it, hear me. Choosing hospice is not choosing to stop loving someone. It is choosing to love them all the way to the end, on their terms, with their comfort at the center. That is not giving up. That is showing up.

CHAPTER 12 TAKEAWAY

Hospice is not the family choosing death. It is the family choosing support when the illness is already changing life. The next right question is: **would comfort-focused care help us care better now?**

CARDINAL MOMENT



A family fought the word hospice for weeks. Saying yes felt like signing something they could never take back. So they waited. And their father spent those days gripping the bed rails in pain.

When they finally called, the nurse came that same night. The pain was under control by morning. For the first time in a week, their father slept. Their only regret, they told me later, was every day they had waited.

Hospice is not the day you stop fighting. It is the day comfort becomes the goal. And most families say afterward that they wish they had called sooner.

A composite drawn from years at the bedside. Names and details changed.

MORE TOOLS

Free worksheets, printable state guides, and checklists are waiting at
thecardinalspromise.com.

OUR MISSION

Mission

To help families avoid unnecessary suffering by understanding the healthcare system before a crisis forces them to.

If this book helps one family make one better decision, it has done its job.



The money questions have answers, too.

Medicare Basics

Part 3 — Paying for Care. Care decisions often become money decisions. Families may know what help is needed but not who pays for it, and that confusion creates panic, arguments, and false expectations. These chapters are not meant to turn you into a Medicare expert, Medicaid planner, or elder law attorney. They are meant to help you ask better questions. Rules can change, and coverage depends on the person's situation, plan, medical need, eligibility, provider participation, and documentation. When in doubt, ask the plan, provider, social worker, or a qualified professional — and do not build a care plan on assumptions.

“An investment in knowledge pays the best interest.”

— ATTRIBUTED TO BENJAMIN FRANKLIN

THIS CHAPTER IN 30 SECONDS

- **The one sentence:** Medicare is health insurance, not long-term care insurance — it generally does not pay for long-term custodial care (bathing, dressing, toileting, meals, supervision) when that is the only care needed.
- **What it may cover:** hospital care, doctor visits, a limited skilled nursing facility stay when requirements are met, some home health, hospice, preventive care, outpatient services, and prescription drugs depending on coverage.
- **Know the plan:** Original Medicare (Parts A and B, often with Part D and a Medigap supplement) follows Medicare rules; Medicare Advantage adds networks, referrals, prior authorization, and plan-specific rules — and leaving Advantage later may mean medical underwriting to buy a Medigap supplement.
- **The trapdoor question:** ask early and more than once — *“Is my parent admitted as an inpatient, or under observation status?”* — and write down the answer and who gave it.
- **The next right question:** what exact care is needed — skilled or custodial — and does Medicare cover that specific care in this situation?

WHAT FAMILIES USUALLY THINK

Families often think Medicare pays for nursing homes, assisted living, someone to stay with Dad, long-term dementia care, or private duty care — or that if the doctor says it is needed, Medicare automatically pays. These misunderstandings cause real problems: a family delays

planning believing Medicare will cover care that is actually private pay, a caregiver burns out waiting for help Medicare does not provide, or a family is shocked when a rehab stay ends sooner than expected. Medicare is important, but it has limits, and families need those limits explained plainly.

The Main Parts of Medicare

PART	WHAT IT GENERALLY HELPS COVER
Part A	Hospital insurance — inpatient hospital care, skilled nursing facility care under certain conditions, hospice, and some home health.
Part B	Medical insurance — doctor visits, outpatient care, preventive services, durable medical equipment, and some home health.
Part C	Medicare Advantage — private plans through which you receive your Medicare benefits. You are still in Medicare, but the plan's networks, prior authorizations, and copays apply. Call the plan directly.
Part D	Prescription drug coverage — depends on the plan, formulary, pharmacy, and cost-sharing.

Original Medicare vs. Medicare Advantage

This distinction matters. Original Medicare includes Part A and Part B, often with a Part D drug plan and a Medigap supplement, and families deal with Medicare rules and participating providers. Medicare Advantage plans are offered by private companies approved by Medicare; they may include drug coverage and extra benefits but also networks, referrals, prior authorization, and plan-specific rules. If your parent has Medicare Advantage, always ask: Is this provider in network? Is prior authorization required? What is the copay? What is covered and not covered? What happens if the plan denies coverage, and what appeal rights exist? The Medicare card alone does not tell the whole story — look at the actual plan.

One more caution: leaving an Advantage plan to return to Original Medicare later may mean medical underwriting to buy a Medigap supplement — a person with health problems can be turned down or charged more. Ask before switching, not after.

What Medicare Usually Does Not Cover

THE NEED CAN BE REAL AND STILL NOT COVERED

Medicare generally does not cover long-term custodial care when that is the only care needed — ongoing help with bathing, dressing, toileting, meals, housekeeping, supervision, long-term dementia care, assisted living or memory care room and board, custodial nursing home care, or private duty care for daily support. There may be other paths (Medicaid waivers, VA benefits, long-term care insurance, state resources), but do not assume Medicare will pay for daily care simply because the need is real.

Home Health, Skilled Nursing, and the Inpatient Trapdoor

Home health. Medicare may cover home health — skilled nursing, therapy, medical social services, and part-time or intermittent aide care in the plan of care — when requirements are met. But it is not private duty care or all-day supervision, and it requires a clinician's evaluation, a skilled need, homebound status, and documentation. Ask, "Can we schedule a provider evaluation to determine whether home health is appropriate?" — not "Can Medicare send someone to stay with Mom?"

Skilled nursing facility care. Medicare Part A may cover a limited stay if requirements are met: Part A days available, a qualifying inpatient hospital stay, a Medicare-certified facility, and a need for skilled care or therapy. The qualifying stay generally means at least three consecutive days as an admitted inpatient — observation days and the discharge day do not count — though many Medicare Advantage plans and some programs waive this. Ask early whether the hospital stay was **inpatient or observation**, how many covered days are available, what you could owe, what happens when coverage ends, and what appeal rights exist. The 2026 structure: days 1–20 generally paid in full, days 21–100 with a \$217-per-day coinsurance, and nothing after day 100 — confirm current amounts at [medicare.gov](https://www.medicare.gov).

Inpatient vs. observation status. This is a major trapdoor: a person may spend the night in the hospital and still be considered outpatient under observation status, which can affect whether Medicare covers a later skilled nursing stay. Ask early and more than once: **"Is my parent admitted as an inpatient, or under observation status?"** Write down the answer and who gave it.

Hospice, Assisted Living, and Equipment

Hospice. When eligibility requirements are met, Medicare covers hospice, including team support, medications related to the hospice plan, equipment, supplies, and bereavement support. But room and board in a facility is generally not covered unless the team determines short-term inpatient care is needed for symptom management, and hospice is a benefit — not a full-time caregiver.

Assisted living and memory care. Medicare usually does not pay the monthly residential room and board, though it may still cover medical services there — doctor visits, covered therapy, home health if requirements are met, hospice if eligible, equipment, or medications. The residential cost is usually paid another way: private funds, long-term care insurance, VA benefits, a Medicaid waiver in some states, or family contribution.

Durable medical equipment. Medicare may cover items like a walker, wheelchair, hospital bed, oxygen, or bedside commode when medically necessary, prescribed, and supplied by a Medicare-approved supplier. Ask:

- ☐ whether it is covered
- ☐ who orders it
- ☐ which supplier to use
- ☐ whether prior authorization is needed
- ☐ what you will owe
- ☐ who teaches you to use it

WHAT TO DO THIS WEEK

1. **Identify the type of coverage** — Original Medicare or Medicare Advantage — and find the plan name and number.
2. **Name the exact care needed** — hospital, rehab, home health, hospice, equipment, or residential care.
3. **Ask whether the care is skilled or custodial** — this is the question that decides most coverage.
4. **Ask what Medicare may cover** for this specific need, what requirements apply, and what you may owe.
5. **Ask what Medicare will not cover** so you can plan those costs another way.
6. **Ask who can help** — case manager, social worker, SHIP counselor, or benefits counselor.

■ **Worksheet — Medicare Coverage Questions** — fill it in from the printable companion workbook · TheCardinalsPromise.com/resources

■ QUESTIONS TO ASK WHENEVER SOMEONE SAYS "MEDICARE SHOULD COVER IT"

1. Which part of Medicare — Original or Advantage — and is the provider in network?
2. Is prior authorization required, and what requirements must be met?
3. Is there a skilled need, and is this short-term or long-term care?

4. What does Medicare pay, and what do we owe?
5. What happens if coverage ends, and can we appeal a denial or discharge?
6. Who can explain this in writing?

ROB'S NOTE

Medicare confusion can make families feel foolish. They are not foolish.

The system is complicated. The words sound like they should mean more than they do. Skilled care. Custodial care. Observation. Inpatient. Benefit period. Prior authorization. Advantage plan. Supplement. It's a lot of hard words. And most families are trying to make sense of them while someone they love is falling, getting worse, or being sent home.

So don't try to remember it all. Ask better questions. Write down names and dates. Ask what is covered, what is not, and what happens next.

Medicare can be a big help. But it is not a free pass for aging. The clearer the family gets about that, the fewer surprises they will face.

CHAPTER 13 TAKEAWAY

Medicare is health insurance, not long-term care insurance. It may cover certain medical or skilled services when requirements are met, but it usually does not pay for long-term daily custodial care. The next right question is: **what exact care is needed, and does Medicare cover that specific care in this situation?**

CARDINAL MOMENT



A son was sure Medicare would pay for his mother's long-term care. He found out it would not the way most families do. A bill showed up months in, and it scared him.

What Medicare did cover, it covered well. It paid for the hospital, the rehab after, the home-health nurse, and later, hospice. If he had known where that line was before the crisis, he could have skipped a month of worry.

Medicare pays for a lot. But it does not pay for the everyday, long-term care most families think it does. Learn where the line is before you are standing on it.

A composite drawn from years at the bedside. Names and details changed.

Medicaid Basics

THIS CHAPTER IN 30 SECONDS

- **What it is:** a needs-based program, administered by states, that may help pay for long-term care — nursing facility or home and community-based services — for people who meet both financial and functional or medical requirements.
- **What it is not:** Medicare, automatic, or the same everywhere — program names, waiting lists, and applications vary by state, so do not rely on what worked for a neighbor in another state.
- **The look-back:** transfers and gifts may be reviewed during a look-back period — currently five years (60 months) in most states — and can create penalties or eligibility problems.
- **Before moving money:** ask a qualified elder law attorney or Medicaid planner before spending down, transferring assets, changing titles, or making gifts — good planning can protect care options; bad guessing can create expensive delays.
- **Also ask:** what spousal impoverishment protections may let the spouse at home keep, and whether estate recovery applies — get the answer in writing.

WHAT FAMILIES USUALLY THINK

Families often think Medicaid and Medicare are the same, that Medicaid only pays for the poor, that it will take the house right away, that they must spend every dollar first, that they can just give money to the kids, or that they should wait until the money is gone. Most of these are too simple. Medicaid is not a single thing that works the same everywhere — it is a federal and state program, and state rules matter. One family may be dealing with nursing facility Medicaid, another with home and community-based services, another with a waiver or help paying Medicare premiums (ask for "Medicare Savings Programs" by name). Families need the right doorway.

Medicaid vs. Medicare

Keep this distinction clear. A person can have both (often called "dual eligible"), but having Medicare does not automatically mean Medicaid is available, and having Medicaid does not mean every setting or service is covered.

MEDICARE

MEDICAID

Health insurance	Needs-based program
Hospital, doctor, short-term SNF, home health, hospice	May help pay for long-term services and supports
Generally does not pay for long-term custodial care alone	May cover nursing facility or home/community services if you qualify
Same federal rules nationwide	Administered by states — rules vary by state

Medicaid and Long-Term Care

Medicaid is the nation's primary payer for long-term services and supports, delivered in institutions such as nursing facilities or in community-based settings such as the home, depending on the program and eligibility rules. It may help people who need ongoing assistance because of chronic illness, disability, frailty, or functional limits. The important point: Medicaid may help with long-term care for people who qualify, but qualifying is not automatic. The person usually must meet both financial and functional or medical requirements, and those requirements — along with the program names, waiting lists, and application process — differ by state. Do not rely on what worked for a neighbor in another state. Medicaid is not gossip-friendly. It is paperwork-friendly.

Nursing Facility, Home & Community, Assisted Living, and Memory Care

Nursing facility Medicaid may help pay for that level of care for people who meet state rules, but qualifying usually involves a detailed financial review and clinical eligibility. Not every facility accepts Medicaid, and not every bed is a Medicaid bed — ask early.

Home and community-based services (HCBS) let some people receive care at home or in the community instead of an institution, often through state waiver programs. They can be valuable but are not always immediate — waiting lists can exist, and they may not cover everything. Plan early.

Assisted living and memory care coverage varies widely. In some states a waiver may help pay for certain services but not room and board; in others these settings stay mostly private pay. Ask the community and the state Medicaid office before move-in — not after the money is gone — whether Medicaid helps, whether there are limited slots or a waiting list, and what happens if private funds run out.

Spend-Down, Transfers, and the Look-Back Period

DO NOT MOVE MONEY WITHOUT ADVICE

"Spend-down" generally means reducing countable income or assets to meet Medicaid rules — but it does not mean giving money away or making random purchases. For long-term care Medicaid, transfers and gifts may be reviewed during a look-back period — currently five years (60 months) in most states — and can create penalties or eligibility problems. Before spending down, transferring assets, changing titles, or making gifts, ask a qualified elder law attorney, Medicaid planner, or the appropriate state resource. Good planning can protect care options. Bad guessing can create expensive delays.

The Spouse at Home and Estate Recovery

The spouse at home. When one spouse needs long-term care and the other stays home, special rules (often called spousal impoverishment protections) may let the spouse at home keep certain income or assets within program rules. Do not assume both spouses must become completely broke before Medicaid helps — and do not assume everything is protected. Ask the right professional what the spouse can keep, how the home counts, and what documentation is needed.

Estate recovery. State Medicaid programs are required to seek recovery from certain enrollees' estates. For people age 55 or older, states must do so for nursing facility services, home and community-based services, and related hospital and prescription drug services. Rules, exemptions (for a spouse, disabled child, caregiver child, and others), deferrals, and hardship waivers vary. Do not panic — and do not ignore it. Estate recovery is a planning issue. Ask someone to explain, in writing, whether it applies in your situation.

When to Ask for Help

Ask for help early if:

- ☐ long-term care may be needed
- ☐ private funds are running low
- ☐ nursing facility or memory care is being discussed
- ☐ a spouse remains at home
- ☐ the person owns a home
- ☐ assets were transferred or gifted
- ☐ there is a trust
- ☐ family members disagree about money
- ☐ you are close to discharge with no payment plan

Possible helpers include:

- ☐ the state Medicaid office
- ☐ the local Department of Social Services
- ☐ a facility business office
- ☐ a hospital or rehab social worker
- ☐ the Area Agency on Aging
- ☐ a SHIP counselor
- ☐ an elder law attorney
- ☐ a Medicaid planner
- ☐ a care manager
- ☐ a veterans service officer if VA benefits may apply

WHAT TO DO THIS WEEK

1. **Identify the state and program question** — nursing facility, HCBS, waiver, assisted living, memory care, or help with Medicare costs.
2. **Find out what level of care is needed** and whether it matches a Medicaid program.
3. **Gather financial documents** — bank and income statements, insurance cards, property info, power of attorney.
4. **Ask about timing** — application length, waiting lists, retroactive coverage, and what to pay while pending.
5. **Do not transfer assets without advice** — talk to a qualified professional first.
6. **Ask about estate recovery** in calm, plain language and get the answer in writing.

■ **Worksheet — Medicaid Planning** — fill it in from the printable companion workbook · TheCardinalsPromise.com/resources

■ KEY QUESTIONS TO ASK

1. Which Medicaid program may apply, and what are the income and asset limits?
2. What medical or functional level of care is required, and is there a waiting list?
3. What documents are needed, how long does the application take, and can coverage be retroactive?
4. What does the person pay from monthly income, and what happens to the spouse at home?
5. How is the home treated, and do transfers or gifts create problems?
6. Does estate recovery apply, and what protections or hardship waivers exist?

7. Does the facility accept Medicaid, are there limited slots, and what happens if private funds run out?

ROB'S NOTE

Medicaid talks make families nervous. People worry about the house, the bank account, the spouse still at home, the look-back, and the application. They fear that one wrong move will ruin everything.

That fear makes sense. But fear is not a plan.

The best thing a family can do is slow down. Gather your papers. Ask questions about your own state's rules. Get good help before you make any money moves. Medicaid may become the bridge that keeps care going when private money runs out. But it is a bridge with rules. Don't cross it blindfolded.

Ask early. Write everything down. And before you move money, change names on titles, or give gifts, get advice.

CHAPTER 14 TAKEAWAY

Medicaid may help pay for long-term care for people who meet financial and functional requirements, but the rules vary by state and program. The next right question is: **which Medicaid program might apply, what are the rules in this state, and who can help us apply correctly?**

CARDINAL MOMENT



A daughter was scared that applying for Medicaid meant her mother would lose the house she had lived in her whole life. She almost didn't apply at all, all because of a rumor.

A good elder-law lawyer walked her through the rules. They were far kinder than the stories she had heard. The home was often protected. The healthy spouse was not left with nothing. The paperwork was hard. But the disaster she feared never came.

Medicaid is a maze, not a trap. Get a guide who knows the way, and start before the money is gone, not after.

A composite drawn from years at the bedside. Names and details changed.

Private Pay

THIS CHAPTER IN 30 SECONDS

- **What it is:** the family or the person receiving care pays directly from personal funds — savings, retirement income, pensions, Social Security, investments, home sale proceeds, or family contributions.
- **What it buys:** what insurance does not — private duty care, assisted living, memory care, adult day care, respite, transportation, home modifications — but money creates options; it cannot guarantee a perfect caregiver or unlimited time.
- **The key number:** the monthly burn rate — a number, not a vague guess — then divide available funds by the monthly gap (costs minus income) to see how long the plan can last.
- **Protect two people:** do not spend down without understanding how the spouse at home will live, and put family contributions in writing — who pays, how much, gift or loan, and the backup plan if someone stops.
- **The next right question:** what care is needed now, what does it cost, how long can we sustain it, and when do we need another plan?

WHAT FAMILIES USUALLY THINK

Families often think Medicare will cover more than it does, that they should not have to use savings, that Mom worked hard for that money, or that Dad wanted to leave something to the kids. These thoughts are common — and dangerous if they delay planning. Money is not the most important thing, but it affects options. A family may want twenty-four-hour care at home that is not sustainable. A parent may want assisted living whose rate rises with care needs. A family may protect an inheritance so tightly that the person who earned the money does not receive the care they need. Care decisions and money decisions are connected. Money should not make every decision — but it should be named clearly enough that the family can plan honestly.

What Private Pay Covers — and What It Doesn't Solve

Private pay can cover what insurance does not:

- ☐ private duty caregivers

- ☐ assisted living and memory care fees
- ☐ adult day care
- ☐ respite
- ☐ care managers
- ☐ transportation
- ☐ meal delivery
- ☐ home safety modifications
- ☐ supplies

It is often the glue between formal services — home health for skilled needs plus private duty for meals and bathing, hospice plus private duty help at night. But money can buy help; it cannot solve every problem. Private pay does not guarantee a perfect caregiver, a safe home, family agreement, unlimited time, or no future decline. Money creates options, and it can disappear quickly without a plan. The family needs to understand the monthly burn rate: how much is care costing each month, and how long can this plan last?

The Monthly Burn Rate

The monthly burn rate is the amount spent on care each month. Families need this as a **number, not a vague guess**. Include:

- ☐ housing or facility base rate
- ☐ care level fees
- ☐ private duty care
- ☐ medications
- ☐ supplies
- ☐ transportation
- ☐ insurance premiums
- ☐ food
- ☐ home maintenance
- ☐ utilities
- ☐ copays
- ☐ equipment
- ☐ care manager fees

Then compare it to monthly income and available savings. A family may discover the plan is sustainable, works only for a limited period, or needs adjustment — or that Medicaid planning, long-term care insurance, VA benefits, a home sale, or a less expensive setting should be explored. Numbers do not remove emotion. But they turn fog into a map.

Home, Assisted Living, and Memory Care Costs

At home, a few hours a week may be manageable, but daily help costs more, overnight costs more, and twenty-four-hour care can become extremely expensive. Ask how many hours are needed, the hourly rate, minimum shifts, night and weekend rates, and how long it can be sustained. The question is not whether home is emotionally preferred — it is whether home is safe, supported, and financially sustainable.

Assisted living bills may include base rent, care level fees, medication management, community or move-in fees, supply and transportation charges, and rate increases over time. Ask for a written cost breakdown — not just the brochure number — and ask what is included, what costs extra, how care levels are priced, how often rates rise, and what happens if funds run out.

Memory care is often more expensive because it provides more supervision and dementia-specific support. Ask the total monthly cost today, what could make it increase, what needs would require added private duty care or a move, and whether Medicaid options exist in that state or community. Memory care can bring safety and structure — and drain funds quickly if not planned carefully.

Family Contributions and the Spouse at Home

Family contributions should be handled carefully, not built on vague promises. Write down who is contributing, how much, how often, for how long, whether it is a gift or a loan, who pays the bill, who tracks payments, and the backup plan if someone stops. Money between family members can become emotional; a written plan protects the relationships and keeps the care plan from collapsing because everyone "thought someone else was handling it."

WATCH CLOSELY — THE SPOUSE AT HOME

The spouse at home needs extra care in private-pay planning. Do not spend down money without understanding how the spouse at home will live — what income they need, what bills, housing, and insurance costs remain, what assets are available, and whether Medicaid planning is likely later. Care planning should not protect one person by making the other financially unsafe. Both lives matter.

When to Ask for Professional Guidance

Ask for guidance when care costs are high, funds may run out within one to two years, a spouse remains at home, a home may need to be sold, Medicaid may be needed later,

long-term care insurance exists, family members disagree about money, someone wants to transfer assets, or there are trusts or complex accounts. Possible professionals: an elder law attorney, financial advisor, accountant, Medicaid planner, long-term care insurance representative, veterans benefits counselor, facility business office, or care manager. A good question asked early can prevent a very expensive mistake later.

WHAT TO DO THIS WEEK

1. **Write down the current care costs** — every recurring cost. Do not guess.
2. **Identify monthly income** — Social Security, pension, retirement distributions, and other income.
3. **Estimate available savings** — list accounts and approximate values. Gather information; do not make changes yet.
4. **Calculate the monthly gap** — care cost minus income equals what must come from savings or another source.
5. **Ask how long the current plan can last** — divide available funds by the monthly gap.
6. **Decide when another plan is needed** — review insurance and VA benefits, ask about Medicaid planning, or compare settings before the money is gone.

■ **Worksheet — Private Pay Planning** — fill it in from the printable companion workbook · TheCardinalsPromise.com/resources

■ COST QUESTIONS TO ASK A FACILITY OR AGENCY

1. What is the base monthly cost, what is included, and what costs extra?
2. How are care levels priced, and how often can rates increase?
3. Are there move-in fees or deposits, and are supplies, medication, and transportation included?
4. What happens if care needs increase, or if private funds run out?
5. Do you accept long-term care insurance, or Medicaid / Medicaid waiver if applicable?
6. What is the move-out policy, and can we get all costs in writing?

ROB'S NOTE

Talking about paying out of pocket can feel cold, because it puts numbers next to love. But the numbers are not the enemy. The numbers are there so the family can protect the care plan.

I have seen families put off the cost talk until the crisis hits. Then everyone is shocked, rushed, and angry. That is when bad choices happen.

Talking about money does not mean you care less. It means you are trying to make care last. It means you are trying to protect the parent, the caregiver, the spouse at home, and the whole family from a surprise.

Don't let guilt write the budget. Don't let fear hide the numbers. Put the costs on paper. Then decide what the next right step should be.

CHAPTER 15 TAKEAWAY

Private pay means using personal or family funds to pay directly for care that insurance or public benefits may not cover. The next right question is: **what does the current care plan cost each month, and how long can we safely sustain it?**

CARDINAL MOMENT



A family paid for care out of their own pocket. They watched their savings drop, faster every month. The not-knowing was its own kind of fear. Every time they took money out, it felt like the last safe time.

What helped them was a single page. It showed what the care cost, how many months the money would last at that rate, and exactly what came next when it ran low. The number was scary. But not knowing the number had been worse.

Paying out of pocket is not failure. It is a stage, and it has a way out. Map how long the money lasts and what comes next — before an empty account, not a plan, makes the choice for you.

A composite drawn from years at the bedside. Names and details changed.

Long-Term Care Insurance

THIS CHAPTER IN 30 SECONDS

- **The starting line:** you either have a policy or you do not — coverage usually has to be purchased before care is needed, so do not try to create it in the middle of a crisis. (Planning ahead for yourself? Traditional and hybrid life/LTC policies are typically bought in one's 50s or 60s — premiums rise with age and health changes can close the door — so compare options with a fee-only advisor well before care is on the horizon.)
- **First job:** find the policy — check documents, bank statements for premium drafts, employer or retirement benefit files, life insurance policies with long-term care riders, hybrid policies, and financial advisor records.
- **If no policy:** move on to the next payment option — no policy is not a moral failure; it is just information.
- **If a policy exists:** call the company and ask about covered settings and providers, the benefit amount, the elimination period, reimbursement or cash benefit, and what claim documentation is required.
- **Then:** get the benefit details in writing, match the provider or setting to the policy requirements before choosing care, and assign one person to manage the claim.

WHAT FAMILIES USUALLY THINK

Families often think, "I'm sure Dad has something," or "Mom was organized, so there must be a policy," or "We'll just buy a policy now." Maybe. Maybe not. The first job is not to guess — it is to find the policy.

What to Look For — and Where

Search for:

- ☐ a long-term care insurance policy
- ☐ premium notices
- ☐ annual statements
- ☐ insurance company letters
- ☐ bank drafts for premiums
- ☐ employer or retirement benefit documents

- ☐ life insurance policies with long-term care riders
- ☐ hybrid life/long-term care policies
- ☐ insurer emails
- ☐ financial advisor records
- ☐ an agent's contact information

Check:

- ☐ the important-documents folder
- ☐ filing cabinet
- ☐ safe
- ☐ desk drawers
- ☐ bank statements
- ☐ email
- ☐ financial advisor portal
- ☐ tax files
- ☐ insurance binder
- ☐ mail pile
- ☐ power of attorney files

If There Is No Policy

If there is no policy, do not get stuck here. Move to the next payment questions: Can care be paid privately? Are VA benefits possible? Could Medicaid become part of the plan? Is there a state program or waiver? Does the care setting offer financial options? No policy means no long-term care insurance benefit to activate. That is not a moral failure. It is just information.

If There Is a Policy

If a policy exists, call the insurance company. Ask what is covered and how to start a claim — and do not assume every setting or service qualifies. Ask specifically about:

- ☐ home care, assisted living, memory care, adult day care, and nursing facility care
- ☐ caregiver and licensed-agency requirements
- ☐ the daily or monthly benefit amount
- ☐ the elimination period
- ☐ the maximum benefit period
- ☐ inflation protection
- ☐ required assessments and documentation
- ☐ whether it is reimbursement or a cash benefit

- ☐ who can submit claims

Key Terms Families Need to Know

Benefit amount — how much the policy may pay, listed as a daily amount, monthly amount, or pool of money.

Elimination period — the waiting period before benefits begin, during which the family may need to pay privately. Ask whether it is counted in calendar days, service days, or another rule.

Covered setting — not every policy covers every type of care; ask whether it covers the setting you are considering.

Covered provider — some policies require a licensed agency or approved facility, and privately hired caregivers may not qualify.

Claim documentation — the insurer may require care notes, invoices, assessments, or provider forms. Without the right paperwork, payment may be delayed.

WHAT TO DO THIS WEEK

1. **Find out whether a policy exists** — check documents, bank statements, financial records, and advisor files.
2. **If no policy exists, move on** to the next payment option. Do not linger.
3. **If a policy exists, call the company** and ask how to start a claim and what documents are needed.
4. **Get the benefit details in writing** — ask for a benefits summary.
5. **Match the policy to the care plan** — make sure the provider or setting meets the policy requirements before choosing care.
6. **Assign one person to manage the claim** — claims involve ongoing invoices, forms, and follow-up.

■ **Worksheet — Long-Term Care Insurance Policy Finder** — fill it in from the printable companion workbook · TheCardinalsPromise.com/resources

■ STEP 6: CLAIM REQUIREMENTS TO ASK THE INSURER

1. How do we start a claim, and what forms are needed?
2. Is a provider assessment or doctor statement required?
3. Are invoices or care notes required, and must the provider be licensed?

4. How long does approval usually take?
5. Who can speak on behalf of the policyholder, and what power of attorney paperwork is needed?

Final decision

Where this lands: No policy — move to other payment options ☐ Policy exists, start a claim ☐
 Policy exists but current care may not qualify ☐ More information needed ☐

Next step _____

Person responsible _____

Due date _____

ROB'S NOTE

Long-term care insurance is not something families should spend three weeks arguing about during a crisis.

Find the policy. If it exists, call and start the process. If it does not exist, move on.

That may sound blunt, but it is kind to the family's energy. When care is needed now, the family does not need pretend coverage. They need real choices.

Paper first. Facts second. Decision third.

CHAPTER 16 TAKEAWAY

Long-term care insurance starts with one question: does a policy already exist? If yes, find out what it covers and how to start a claim. If no, move to the next payment option.

CARDINAL MOMENT



A man found out, only after his father moved into care, that the old man had quietly paid for long-term-care insurance for twenty years. The policy was in a drawer no one had thought to open.

It covered most of what he needed. But first the family had to learn how to file the claim, log the hours of care, and wait out the waiting period before the payments began. The coverage was real. The paperwork almost buried it.

If a policy might exist, find it early and read it twice. The coverage is only ever as good as the claim someone actually files.

A composite drawn from years at the bedside. Names and details changed.

Prescription Assistance

THIS CHAPTER IN 30 SECONDS

- **The core rule:** *"Never disqualify yourself. Ask the question. Complete the application. Let the program tell you no."* — many pharmaceutical assistance programs focus primarily on income, not the value of a person's home.
- **The cap:** as of this writing, Medicare drug coverage caps yearly out-of-pocket costs for covered Part D drugs — \$2,100 in 2026, adjusted annually — and the Medicare Prescription Payment Plan can spread those costs into predictable monthly bills.
- **Doors to try:** Extra Help, manufacturer patient assistance programs, State Pharmaceutical Assistance Programs, and disease-specific foundations — it is a question tree: if one door closes, try the next.
- **Ask the pharmacist:** the counter price is a starting point, not a verdict — check the cash price, another pharmacy, a 90-day supply, a generic, and whether a discount card means the cost does not count toward the plan's deductible or out-of-pocket tracking.
- **Ask the prescriber:** *"Is there a lower-cost alternative that is safe and appropriate?"* — and ask for a medication review; do not stop or change medications without clinical guidance.

The Million-Dollar House and the Free Medication

Years ago, while working for a large pharmaceutical company, I was helping a physician's office find a way for a patient to afford a medication that cost more than \$850 per month.

The physician's office told me she would never qualify for assistance. "She lives in a multi-million-dollar home," they said.

I asked a different question: "What is her income?"

After reviewing the details, I explained that many pharmaceutical assistance programs focus primarily on income, not the value of a person's home. The office was skeptical. The patient applied anyway. She was approved. Her medication was provided at no cost.

That experience taught me a lesson I have seen repeated throughout healthcare: many families assume they do not qualify for help before they ever ask. **Never disqualify yourself. Ask the question. Complete the application. Let the program tell you no.** One application can sometimes save more money than months of worrying.

WHAT FAMILIES USUALLY THINK

Families often think the pharmacy price is the only price, that Medicare covers all prescriptions, that they make too much to qualify for help, or that coupons always (or never) work. Some of these may be true in a specific case, but families should not guess. Prescription assistance is a question tree: you try one door; if it closes, you try the next.

Medicare Part D and Extra Help

Part D and drug coverage. Medicare drug coverage depends on the specific Part D or Medicare Advantage plan — formularies, tiers, preferred pharmacies, deductibles, copays, prior authorization, and step therapy. The same medication may cost different amounts under different plans. Ask whether it is covered, what tier it is on, whether prior authorization or step therapy applies, whether a preferred pharmacy, mail order, or 90-day supply costs less, and whether there is a covered alternative. Review the medication list at least once a year, especially during open enrollment.

The yearly cap on drug costs. As of this writing, Medicare drug coverage caps what a person pays out of pocket for covered Part D drugs each year — \$2,100 in 2026, with the amount adjusted annually. Once the cap is reached, covered drugs cost nothing for the rest of the year. Families can also ask about the **Medicare Prescription Payment Plan**, which spreads those out-of-pocket costs across the year in predictable monthly bills instead of large amounts at the pharmacy counter. Ask the plan or a SHIP counselor for the current cap amount and how the payment plan works — for a family staring at a large pharmacy bill, these two protections can change the whole picture.

Extra Help. Extra Help is a Medicare program that helps people with limited income and resources pay Part D costs — premiums, deductibles, and copays. Some people qualify automatically; others apply — before or after enrolling in Part D. Do not assume income is too high. Check whether your parent qualifies, and ask who can help with the application.

Manufacturer, State, and Foundation Programs

Manufacturer patient assistance programs (PAPs). Some drug companies offer programs that may provide financial assistance or free medication, sometimes for people with Part D and sometimes only for those without insurance. They vary and may require proof of income, prescription information, doctor forms, denial letters, or out-of-pocket records. Medicare has a tool to search assistance programs by drug. Ask the manufacturer or program directly.

State Pharmaceutical Assistance Programs (SPAPs). Some states offer programs to help with drug costs; availability varies. Ask whether this state has one, who qualifies, whether it works with Part D, and whether it helps with premiums, copays, or specific medications.

Disease-specific foundations. Some nonprofits help with medication costs for specific diagnoses — certain cancers, rare diseases, heart, lung, or kidney conditions. Funding opens and closes, so check current availability, covered medications, income limits, and whether the program works with Medicare.

Ask About Alternatives, Coupons, and a Medication Review

Alternatives. Sometimes the easiest savings come from one question: "Is there a lower-cost alternative that is safe and appropriate?" Ask about:

- ☐ a generic
- ☐ a therapeutic alternative
- ☐ a different dose or formulation
- ☐ a covered formulary option
- ☐ a 90-day supply
- ☐ whether the medication is still needed

Doctors do not always know the price at the counter; pharmacists often do. Do not stop or change medications without clinical guidance — but do ask.

Coupons and discount cards. Some discount programs may lower costs, but they may not apply with Medicare or Medicaid, and using one can mean the cost does not count toward the plan's deductible or out-of-pocket tracking. Sometimes the cash price is lower than the insurance price; sometimes it is not. Ask the pharmacist the insurance price, the cash price, whether a discount affects insurance tracking, and whether another pharmacy costs less.

Medication review. Prescription assistance is not only about finding money — it is also about reducing burden. Ask for a medication review if there are many medications, duplicates, multiple prescribers, or if the person is falling, confused, dizzy, sleepy, or nearing end of life. A medication that made sense five years ago may not make sense now. Do not stop medications on your own, but do ask whether the list still matches the person's goals, condition, and safety.

WHAT TO DO THIS WEEK

1. **Make a complete medication list** — name, dose, frequency, prescriber, pharmacy, and monthly cost.
2. **Ask the pharmacist for a cost review** of the most expensive medications and lower-cost options.
3. **Ask the prescriber about alternatives** — a generic, formulary option, or safe substitute.
4. **Check Medicare Extra Help eligibility.**
5. **Search manufacturer and state assistance** using Medicare's tool and the drug company.
6. **Assign one person to track medication costs** — savings require forms, calls, and follow-up.

■ **Worksheet — Prescription Assistance** — fill it in from the printable companion workbook
 · TheCardinalsPromise.com/resources

■ QUESTIONS FOR THE PHARMACIST

1. Which medication is costing the most, and is it running through the correct insurance?
2. Is this a preferred pharmacy — and would another pharmacy, mail order, or a 90-day supply be cheaper?
3. Is there a generic version or a discount option?
4. If we use a discount card, does it affect insurance tracking?
5. Should we ask the doctor about an alternative?

■ QUESTIONS FOR THE PRESCRIBER

1. Is this medication still needed, and is there a generic or lower-cost/formulary alternative?
2. Can the dose or formulation be changed safely to lower cost, and are any medications duplicated?
3. Could any medications be increasing falls, confusion, dizziness, or sleepiness?
4. Would a medication review be appropriate?
5. Is there a manufacturer assistance program, and can your office help complete the forms?

Assistance checks

Medicare Extra Help — could it apply? Yes ☐ No ☐ Unsure ☐ — who will check/apply?

Also check: Manufacturer patient assistance ☐ State Pharmaceutical Assistance Program ☐
 Disease-specific foundation ☐

Documents often needed for manufacturer programs: prescription information, proof of income, insurance/Part D card, a doctor form, patient signature, and denial or prior-authorization information.

Medication safety review (check any concern)

- | | |
|--|--|
| ☐ Too many or duplicate medications | ☐ Multiple prescribers |
| ☐ Missed or double doses | ☐ Old medications still in the home |
| ☐ Dizziness, falls, or sleepiness | ☐ Hospice or palliative care involved |
| ☐ New confusion or poor appetite | |

HOPE'S NOTE — FROM THE PHARMACIST

Two things I wish every family knew. First, your pharmacist can see what your doctor often cannot: every prescription, from every prescriber, on one screen — and we are the easiest person in healthcare to reach. No appointment needed; just ask for a minute at the counter.

Second, never let the price at the register be the end of the conversation. Ask us to check the cash price, a different pharmacy, a 90-day fill, and whether the prescriber should hear about an alternative. The counter price is a starting point, not a verdict. And ask us to run a discount program — GoodRx is the one most people know. Patients are often unsure about these, but the discount price is sometimes lower than your insurance copay. The trade-off: a discount purchase usually does not count toward your deductible or out-of-pocket cap, so ask us which way wins for each drug.

ROB'S NOTE

Drug costs can make families feel ashamed. People don't always want to say, "We can't afford this medicine."

Say it anyway. Tell the pharmacist. Tell the doctor. Tell the social worker. Say it before pills get skipped, doses get stretched, or refills get put off.

A medicine can't help if it's stuck behind a price the family can't pay. There isn't always an answer, but there's almost always a better question. Ask what is covered. Ask what is cheaper. Ask what is still needed. Ask what help is out there.

A quiet drug bill can turn into a loud crisis if no one looks at it.

CHAPTER 17 TAKEAWAY

Prescription assistance starts with one practical step: make the medication list, find the expensive drugs, and ask the pharmacist and prescriber what can be done safely.

CARDINAL MOMENT



A widow was quietly cutting her heart pills in half to make them last all month. She was sure nothing could be done about the cost, and she was too proud to say so.

When her daughter finally told the pharmacist, he knew of a program that helped patients pay. It brought the price down to almost nothing. For most of a year, she had been going without her own medicine, and no one knew.

No one should have to choose between medicine and food, alone and unheard. Ask the pharmacist, the doctor, the drug maker. The help is there, but only for the family that asks.

A composite drawn from years at the bedside. Names and details changed.

VA Benefits

THIS CHAPTER IN 30 SECONDS

- **The first question:** "*did they serve?*" — if the person is a Veteran — or a Veteran's surviving spouse or dependent — VA benefits may matter. Do not assume. Check.
- **Several doors, not one bucket:** VA health care, VA pension, Aid and Attendance, Housebound benefits, survivor benefits, long-term services and supports, caregiver support, and burial benefits.
- **Aid and Attendance is not automatic:** the person generally must first qualify for VA Pension or Survivors Pension, then meet additional requirements — the doctor documents the need on VA Form 21-2680.
- **Do not move money here either:** VA pension has its own net worth limit and its own look-back period — currently 36 months — with penalty periods for gifts made to qualify; be wary of anyone selling financial products to "help you qualify."
- **How to start:** find the DD214, gather care cost records, call a Veterans Service Officer — for claims, ask whether the helper is VA-accredited — and assign one person to manage the follow-up.

WHAT FAMILIES USUALLY THINK

Families often think, "Dad was in the military, so VA will pay," or "We make too much," or "He never used VA before, so it is too late," or "Aid and Attendance pays for assisted living automatically." Some of these may be partly true in certain situations, but many are too simple. VA benefits are not one bucket — they are several different doors. The family's first job is to find the right door.

First Question: Did They Serve?

Start here: did the person receiving care serve in the U.S. military? If yes or unsure, gather what you can:

- ☐ branch and dates of service
- ☐ discharge status
- ☐ DD214 or separation papers
- ☐ any VA claim number

- ☐ VA health care enrollment status
- ☐ any service-connected disability rating
- ☐ prior VA benefits
- ☐ surviving spouse status if applicable

Do not stop if you cannot find everything immediately. A Veterans Service Officer or VA contact may help identify what is needed.

Aid and Attendance, Housebound, and Pension

Aid and Attendance / Housebound. These are monthly payments added to a VA pension for qualified Veterans and survivors who need help with daily activities or are housebound. This is often the benefit families hear about when someone needs care. But it is not automatic — the person generally must first qualify for VA Pension or Survivors Pension, then meet additional requirements. The doctor completes VA Form 21-2680 to document the need, and it accompanies the pension application (VA Form 21P-527EZ, or 21P-534EZ for survivors). Ask whether your parent qualifies for pension, whether Aid and Attendance or Housebound could be added, what income and asset rules apply, what medical forms are needed, and who can help apply.

VA Pension and Survivors Pension. VA Pension may provide monthly payments to eligible wartime Veterans who meet age or disability requirements and have limited income and net worth; Survivors Pension may help eligible surviving spouses or children. Do not guess eligibility — ask a VA-accredited representative or Veterans Service Officer. Key questions: Was service during a qualifying wartime period? Is the discharge status qualifying? Do income, net worth, and unreimbursed medical expenses fit the rules? Does assisted living or home care cost affect the calculation? Good paperwork matters here.

DO NOT MOVE MONEY HERE EITHER

VA pension has its own net worth limit and its own look-back period — currently 36 months — on asset transfers, with penalty periods for gifts made to qualify. Do not move money to qualify for Aid and Attendance without advice from a VA-accredited representative, and be wary of anyone selling financial products to "help you qualify."

VA Health Care, Long-Term Services, and Caregiver Support

Health care and long-term services. VA health care may provide access to geriatric and extended care programs — home-based primary care, homemaker and home health aide services, adult day health care, respite, skilled home health, community living centers, contract nursing home care, state Veterans homes, geriatric evaluation, palliative care,

and hospice-related support. Availability and eligibility vary by location, clinical need, and program. Ask the local VA medical center, VA social worker, or Veterans Service Officer what applies.

Caregiver support. VA has caregiver support programs for caregivers of eligible Veterans — education, training, resources, respite connection, and support groups, plus the Program of Comprehensive Assistance for Family Caregivers for those who qualify. Ask whether the Veteran is enrolled in VA health care, whether there is a caregiver support coordinator, and what programs are available. Do not assume caregiver support only means money — sometimes the value is guidance, training, respite, or knowing whom to call. You can contact the VA Caregiver Support Line or a local team.

VA Benefits and Care Settings

VA does not simply pay for assisted living or memory care in every situation, but certain benefits may help — Aid and Attendance can add monthly pension support that the family can put toward care, and some state Veterans homes or community programs may be relevant. Even a monthly benefit that does not cover everything may help the care plan last longer. For hospice or palliative care, if the person is connected to VA, ask whether VA palliative or hospice support is available, whether it can be coordinated with Medicare hospice, and who coordinates care. Many Veterans also have Medicare, Medicaid, or private insurance — the family needs to understand how benefits work together. Do not rely on a sales brochure or hallway rumor. Verify.

Documents and Who Can Help

Start gathering:

- ☐ DD214 or separation papers
- ☐ Social Security number
- ☐ any VA claim number
- ☐ VA health care enrollment info
- ☐ discharge status
- ☐ marriage and death certificates if a survivor benefit is involved
- ☐ medical records
- ☐ a doctor statement
- ☐ VA Form 21-2680 if applicable
- ☐ income and asset information
- ☐ care expense records
- ☐ power of attorney documents

For help, consider a local Veterans Service Officer, county Veterans Services office, VA medical center social worker, VA caregiver support coordinator, an accredited VA claims agent or attorney, or the state Veterans affairs office. A Veterans Service Officer can often identify which benefits apply and help complete forms — and for claims, ask whether the helper is VA-accredited.

WHAT TO DO THIS WEEK

1. **Ask the service question** — did the person serve, in what branch, and roughly when?
2. **Look for discharge papers** — DD214 or separation documents.
3. **Identify the current VA connection** — health care enrollment, disability rating, or benefits already received.
4. **Call a Veterans Service Officer or VA contact** to review eligibility.
5. **Gather care cost records** — home care, assisted living, memory care, medications, and other unreimbursed medical expenses.
6. **Assign one person to manage VA follow-up** — paperwork takes time and needs one owner.

■ **Worksheet — VA Benefits Screening** — fill it in from the printable companion workbook · TheCardinalsPromise.com/resources

■ QUESTIONS FOR THE VETERANS SERVICE OFFICER OR VA CONTACT

1. Which VA benefits might apply, and is the Veteran enrolled in VA health care?
2. Is VA Pension, Survivors Pension, Aid and Attendance, or Housebound possible?
3. What income, asset, service-record, and medical-form requirements apply — and is VA Form 21-2680 needed?
4. Are unreimbursed medical expenses important to the calculation?
5. Could caregiver support, home-based services, respite, or a state Veterans home apply?
6. Could VA help with palliative care or hospice, and how does it work with Medicare?
7. What forms should be filed first, who can help complete them, and how long may it take?

Final decision

Where this lands: VA benefits do not appear relevant ☐ May apply — explore ☐ Review Aid and Attendance / Housebound ☐ Review VA health care enrollment ☐ Review caregiver support ☐ More information needed ☐

Next step _____

Person responsible _____

Due date _____

ROB'S NOTE

VA benefits are easy to miss. Families are usually focused on the crisis right in front of them — the fall, the hospital stay, the assisted living bill, the worn-out spouse, the tour of a memory care home.

*But one question can open another door: **did they serve?***

If the answer is yes, don't guess and don't brush it off. Check. The benefit may not apply. The steps may take time. It may not cover everything.

But if a Veteran or a surviving spouse earned the right to help, the family should at least know what is possible. Find the papers. Call the right person. Ask the question.

CHAPTER 18 TAKEAWAY

VA benefits may help some Veterans, surviving spouses, or caregivers, but eligibility depends on the person, the program, and the situation. The next right question is: **did they serve, and who can help us check what VA benefits may apply?**

CARDINAL MOMENT



A daughter had no idea her father qualified for a VA benefit called Aid and Attendance. It would have paid for years of the care he had gone without. He had never made much of his service. He thought it "wasn't a big deal."

Once the claim was finally filed, it changed what he could afford. The only sad part was the years it could have helped, if someone had known to ask sooner.

If anyone in the family ever wore a uniform, ask about VA benefits. A war fought long ago can still help pay for care today.

A composite drawn from years at the bedside. Names and details changed.

THE STORY BEHIND IT

Rob's memoir *The Cardinal's Promise* is the story that started this work —
thecardinalspromise.com.



The documents that protect the people you love.

The Important Documents Folder

Part 4 — The Documents That Matter. When a crisis hits, the family that can find the right paper moves; the family that cannot gets stuck. These chapters help you gather the medical, legal, and financial documents that decisions actually depend on — one trusted folder, the words on the key legal documents, and the information a family needs the moment someone can no longer speak for themselves. You do not need to become a lawyer. You need to be able to find the right document quickly.

“The palest ink is better than the best memory.”

— CHINESE PROVERB

THIS CHAPTER IN 30 SECONDS

- **What it is:** one folder — paper, digital, or both — so the right person can put a hand on the right paper in minutes, not hours.
- **Why now:** the hospital will want the medication list, the facility will need to know who has authority, the doctor will ask about advance directives — almost always at the worst possible time.
- **The step:** start with the first five — medication list, doctor list, insurance cards, healthcare decision documents, and emergency contacts. A useful folder beats a perfect folder that never gets built.
- **Watch for:** add a HIPAA release — without one, even a devoted daughter can be told “we can’t discuss that.”
- **Keep it alive:** tell the main caregiver, backup caregiver, and healthcare decision maker where it lives, and review it every three months and after any major change.

WHAT FAMILIES USUALLY THINK

Families often think the paperwork can wait until things settle down, that one person “has it handled,” or that everything is somewhere in the house if they ever need it. But things rarely settle down, the one person who has it handled is not always reachable, and “somewhere in the house” is not a plan at 2 a.m. in an emergency room. Building the folder is not a sign that something bad is coming. It is a gift to the family for the day something does.

What Belongs in the Folder

One folder — paper, digital, or both — should hold the essentials in a few clear categories:

- ☐ emergency and medical contacts
- ☐ the medication list and doctor list
- ☐ insurance and benefit cards
- ☐ healthcare decision documents (power of attorney, advance directive, DNR or POLST if applicable)
- ☐ financial and legal documents (financial power of attorney, will, trust, deeds)
- ☐ care and facility paperwork
- ☐ benefits and military or VA documents
- ☐ end-of-life and funeral information

You do not need every item on day one. You need a folder that exists and grows.

One small document worth adding while you are at it: a **HIPAA release**, which lets doctors and hospitals share medical information with the family members named on it. Without one, even a devoted daughter can be told "we can't discuss that."

Start With the First Five

Do not wait for the whole folder to be perfect. **A useful folder beats a perfect folder that never gets built.** Start with the five documents a family reaches for most: the medication list, the doctor list, the insurance cards, the healthcare decision documents, and the emergency contacts. Once those five are in one place, the folder is already doing its job — and you can keep adding.

Keep It Alive, and Tell People Where It Is

A folder only helps if the right people know where it lives. Tell the main caregiver, the backup caregiver, and the healthcare decision maker, and store it somewhere secure. Make copies of important cards and lists, and ask whether copies of legal documents are acceptable or whether originals are needed. Then review it every three months, and after any hospitalization, medication change, doctor change, insurance change, or move. Documents change; a folder that never gets updated becomes a museum. This folder should be alive.

WHAT TO DO THIS WEEK

1. **Choose one folder location** — paper, digital, or both — and one document point person.
2. **Gather the first five documents** — medication list, doctor list, insurance cards, healthcare decision documents, and emergency contacts.

3. **Make copies of important cards and lists**, and confirm whether legal documents need originals.
4. **Tell the right people where the folder is** — main caregiver, backup caregiver, and healthcare decision maker.
5. **List what is missing** and assign who will find each item and where to look.
6. **Set a review date** — every three months and after any major change.

■ **Worksheet — Important Documents Folder Checklist** — fill it in from the printable companion workbook · TheCardinalsPromise.com/resources

ROB'S NOTE

The documents folder isn't exciting. No one brags about a binder.

But when the hospital asks for the medication list, when the facility asks who's in charge, when the doctor asks about advance directives, when the family is tired and scared — that binder becomes a real gift.

These papers aren't busywork. When a hard moment comes, a family with the right documents can act — a family without them gets stuck.

Start with the first five. Then keep going.

CHAPTER 19 TAKEAWAY

The important documents folder gives the family one trusted place to find medical, legal, financial, and emergency information. The next right question is: **can the right person find the right document in less than ten minutes?**

CARDINAL MOMENT



When her mother collapsed, a daughter spent the first frantic hour not at the bedside, but on her knees in a closet. She was tearing through drawers, looking for an insurance card and some idea of what medicines her mother took.

Months later, a neighbor showed her a single folder that held all of it — cards, lists, documents, phone numbers — in one place. The next emergency came, because there always is one. This time she was ready in ninety seconds.

The folder is boring right up until the day it is everything. Build it now, on a calm afternoon, so a crisis never has to build it for you.

A composite drawn from years at the bedside. Names and details changed.

Healthcare Power of Attorney

THIS CHAPTER IN 30 SECONDS

- **What it does:** names the person — called a healthcare agent, proxy, or surrogate; the term varies by state — who can make medical decisions if your parent cannot.
- **What it does not do:** it does not usually give someone control over money (that is a Financial Power of Attorney), and the agent must honor the person's known wishes, not personal preference.
- **Why now:** complete or update it while the person still has decision-making capacity — once capacity is lost, families may face asking a court to appoint a guardian: slower, more expensive, and more public.
- **The step:** name a backup agent, have the wishes conversation, and share copies — then ask: *"We have a Healthcare Power of Attorney. What is the best way to make sure it is added to the medical record?"*
- **Watch for:** a move to another state, a changed agent or relationship, a serious diagnosis, or an old form — do not assume an old form still does the job; ask an attorney.

WHAT FAMILIES USUALLY THINK

Families often think, "I'm the oldest, so it's me," or "I'm the spouse, so I decide everything," or "Everyone agrees, so we don't need paperwork," or "The hospital will know what to do." Sometimes the law provides default rules, and sometimes families agree. But crisis has a way of turning soft assumptions into hard arguments. The Healthcare Power of Attorney tells the medical team who should speak when the patient cannot. It gives the family a clear lane.

What It Does — and Does Not Do

Depending on the document and state law, a Healthcare Power of Attorney may cover decisions about:

- ☐ doctors
- ☐ hospitals
- ☐ surgery
- ☐ medications
- ☐ tests

- ☐ treatments
- ☐ rehab
- ☐ care-related placement
- ☐ palliative care
- ☐ hospice
- ☐ artificial nutrition or hydration
- ☐ life-sustaining treatment
- ☐ comfort-focused care
- ☐ access to medical records

The basic goal is simple: the right person can speak with the medical team when the patient cannot.

It does not usually give someone control over money — that is a Financial Power of Attorney. And it does not mean the named person can ignore the patient's wishes. The agent's job is not personal preference; it is to honor the person's known wishes, values, faith, goals, and best interests. A Healthcare Power of Attorney is not a crown. It is a responsibility.

Capacity Matters — Do Not Wait

The best time to complete or update this document is while the person still has decision-making capacity — the ability to understand the decision, understand the consequences, and communicate a choice. If memory is changing, do not wait. If dementia has been diagnosed, do not assume it is automatically too late, but do not delay either — ask the doctor and an attorney what is appropriate. Once a person loses capacity, signing new legal documents may no longer be possible, and families may face harder options — such as asking a court to appoint a guardian — a slower, more expensive, and more public process. The door is easiest to open before it locks.

Choosing the Agent — and a Backup

The best healthcare agent is not always the loudest, closest, or oldest. It is someone who can:

- ☐ stay calm under pressure
- ☐ listen to doctors
- ☐ ask good questions
- ☐ honor the person's wishes
- ☐ communicate with family
- ☐ be reachable

- ☐ make hard decisions without making themselves the center
- ☐ put the patient's values ahead of personal guilt

This is not a popularity contest. It is a trust decision.

Every Healthcare Power of Attorney should name a backup agent if possible. The first person may be sick, traveling, overwhelmed, or unreachable. Ask who is the first choice, who is the backup, whether each person knows they are named, whether they understand the person's wishes, and whether they have a copy. A backup is not an insult. It is a safety net.

The Conversation Matters More Than the Form

The document matters, but so does the conversation. A healthcare agent should know what matters most to the person, what quality of life means to them, what treatments they would and would not want, where they would prefer to receive care, who they want involved, what faith or spiritual beliefs matter, whether they would want hospice or comfort-focused care, what they fear most, and what they hope for. Do not make the agent guess in the hallway outside an ICU room. Talk while there is time.

When Family Disagrees

A Healthcare Power of Attorney can reduce conflict, but it may not erase emotion. Family members may still disagree, push for more or less care, or accuse the agent of giving up. The agent should stay grounded in the person's wishes. Helpful language: "I hear that this is hard. My job is to follow what Mom wanted, not what is easiest for any of us," or "We are not making this decision from fear — we are trying to honor what Dad told us mattered most," or "Let's ask the medical team to explain the options clearly, then come back to what Mom would choose if she could speak." The agent does not need to win every argument. The agent needs to stay faithful to the patient.

Give Copies to the Right People, and Review It

Once complete, share copies with the healthcare agent, backup agent, primary doctor, hospital system, any facility, hospice or palliative team, main caregiver, and attorney, and put one in the important documents folder. Keep the original in a safe but accessible place — a document no one can find is a paper ghost. Review it if the person moved to another state, the named agent died or is no longer appropriate, relationships changed (marriage, divorce, widowhood), wishes changed, a serious diagnosis occurred, dementia is progressing, or the document is old or missing. Do not assume an old form still does the job — ask an attorney whether it is valid and current.

WHAT TO DO THIS WEEK

1. **Find out whether one exists** — check the documents folder, safe, attorney files, and hospital records.
2. **Identify who is named** — the healthcare agent and the backup agent.
3. **Make sure the agent knows** they are named and understands the responsibility.
4. **Share copies** with the agent, doctor, facility, and care team.
5. **Have the wishes conversation** — what care would be wanted, and what would not.
6. **Update if needed** — if the document is missing, outdated, or no longer reflects the person's wishes, ask an attorney.

■ **Worksheet — Healthcare Power of Attorney** — fill it in from the printable companion workbook · TheCardinalsPromise.com/resources

■ **THE WISHES CONVERSATION (ASK WHILE THE PERSON CAN STILL ANSWER)**

What matters most to you if you become seriously ill? _____

What kind of care would you want — and not want? _____

Where would you prefer to receive care if possible? _____

Who do you want involved in medical decisions? _____

What role should faith or spiritual beliefs play? _____

What would you want your family to remember if decisions get hard?

■ **CONVERSATION SCRIPTS**

With the person: "We want to make sure your wishes are honored if there is ever a time when you cannot speak for yourself. Who would you trust to make medical decisions, and what would you want that person to know?"

With family: "This is not about control. It is about making sure the doctors know who is legally named to speak if Mom or Dad cannot."

With the doctor or facility: "We have a Healthcare Power of Attorney. What is the best way to make sure it is added to the medical record?"

ROB'S NOTE

The Healthcare Power of Attorney is one of those documents families don't think about until the room gets serious. Then, all at once, everyone wants a clear answer.

Who can talk to the doctor? Who can make the decision? Who knew what Mom wanted? Who has the paper? Who is the backup?

Don't leave that to memory. Don't leave it to family fights. Don't leave it to the loudest person in the room. Put it in writing. Talk about it while there is still time.

The best healthcare agent isn't the person who wins the argument. It's the person who can carry out the wishes.

CHAPTER 20 TAKEAWAY

A Healthcare Power of Attorney names the person who can make medical decisions if your parent cannot. The next right question is: **who is legally named to speak for this person if they cannot speak for themselves?**

CARDINAL MOMENT



Three grown children stood in an ICU. A doctor gently asked who was allowed to speak for their father, who was unconscious. No one had ever been named. Scared and grieving, the siblings disagreed about almost everything, and the hospital couldn't just pick one of them.

The days that followed were harder than they needed to be. Not because they loved him less, but because one short form — the kind you could have signed on any ordinary afternoon years earlier — had never been signed.

Naming a healthcare agent isn't planning to die. It's deciding, calmly and ahead of time, who speaks for you on the day you can't speak for yourself.

A composite drawn from years at the bedside. Names and details changed.

Living Will / Advance Directive

THIS CHAPTER IN 30 SECONDS

- **What it does:** explains what care a person would or would not want — CPR, ventilators, feeding tubes, comfort care — while the Healthcare Power of Attorney names who speaks; a family should try to have both.
- **What it does not do:** it usually applies only in specific medical situations defined by state law — often a terminal condition or permanent unconsciousness certified by physicians — so ask what triggers it.
- **Watch for:** a Living Will may express wishes about CPR, but a medical order such as a DNR, MOST, or POLST may also be needed depending on the state and setting — do not assume one form does everything.
- **The step:** read it before the crisis, talk with the healthcare agent, and share copies with the doctor, agent, facility, and care team.
- **How to ask:** *"Can you help us understand whether the advance directive matches the medical situation we are facing now?"*

WHAT FAMILIES USUALLY THINK

Families often think, "We know what Mom would want," "Dad told me once," "Talking about this will scare them," or "The Healthcare Power of Attorney is enough." The problem is not that families do not care — it is that love does not always remember clearly under pressure. One sibling remembers one conversation; another remembers something different. One says, "Mom would fight." Another says, "Mom said she never wanted machines." Both may be grieving, both sincere, and both wrong if no one wrote it down. A Living Will or Advance Directive helps turn memory into guidance.

Healthcare Power of Attorney vs. Living Will

These often work together, but they are not the same. The Healthcare Power of Attorney names the person who can make decisions — it answers **who speaks**. The Living Will or Advance Directive explains the person's wishes about care — it answers **what they would want**. A family should try to have both: the agent needs authority, and the agent also needs guidance.

What a Living Will May Cover

Depending on the state and document, it may address:

- ☐ CPR
- ☐ ventilator use
- ☐ feeding tube
- ☐ artificial hydration
- ☐ dialysis
- ☐ surgery
- ☐ antibiotics
- ☐ hospital transfer
- ☐ intensive care
- ☐ comfort-focused care
- ☐ palliative care
- ☐ hospice
- ☐ organ donation
- ☐ spiritual preferences
- ☐ preferred place of care
- ☐ who should be involved
- ☐ what quality of life means
- ☐ what treatments would feel too burdensome

The document may not answer every question — that is why the conversation matters too. The paper is the map. The conversation is the lantern.

One more thing to know: a Living Will usually applies only in specific medical situations defined by state law — often a terminal condition or permanent unconsciousness certified by physicians. Ask the attorney or doctor what triggers it, so the family does not over-read its reach.

CPR and Code Status

CPR is often misunderstood. On television it looks quick, clean, and successful. Real life is different, especially for someone who is frail, seriously ill, or near the end of life. Ask the doctor: What would CPR look like for this person? What is the chance it would help? What injuries or complications could happen? Would it lead to a ventilator or ICU stay? Would it match the person's goals? Is a DNR or medical order appropriate? A Living Will may express wishes about CPR, but a medical order such as a DNR, MOST, or POLST may also

be needed depending on the state and setting. Do not assume one form does everything. Ask.

Ventilators, Nutrition, and Hydration

Ventilators and intensive care. A ventilator helps someone breathe. For some it is temporary support during a reversible illness; for others it may prolong dying or lead to care they would not want. Ask whether it is expected to be temporary, what you are hoping it will fix, what happens if the person cannot come off it, whether they would want ICU care, and what comfort-focused care would look like instead.

Artificial nutrition and hydration. Feeding tubes and IV fluids feel personal — food means love, water means care. But near the end of life or in advanced illness, the body may not process food and fluids the same way; they may help in some situations and create burdens in others. Ask what problem you are trying to solve, whether it is temporary or long-term, whether it will improve comfort or strength, whether it could cause complications, what comfort feeding means, and what hospice or palliative care would recommend. Do not let guilt make the decision alone. Ask what helps the person.

Comfort-Focused Care

A Living Will can help clarify when the goal should shift from cure to comfort. Comfort-focused care may include:

- ☐ pain, breathing, anxiety, and nausea relief
- ☐ mouth and skin care
- ☐ gentle repositioning
- ☐ a quiet room
- ☐ family presence
- ☐ spiritual support
- ☐ hospice
- ☐ palliative care
- ☐ avoiding burdensome treatments

Comfort care is not "doing nothing." It is doing the things that matter when cure is no longer the goal. It is active, focused care — care with the noise turned down.

The Conversation, Memory, and Disagreement

The document should not appear out of nowhere during a crisis — talk while the person can still answer. Ask what matters most if they become seriously ill, what would make life

still feel meaningful, what would feel like too much, what treatments they would and would not want, whether they would want to be home, whether they would want hospice, who they trust to speak for them, and what to remember if decisions get hard. Write down the answers; they do not have to be perfect, only honest.

If memory is changing, advance directives become urgent — a person with early dementia may still be able to share wishes and sign documents, depending on capacity, but waiting can close the door. And when family disagrees, the document lets everyone return to the person's stated wishes: "Let's come back to what Dad wrote down." "Mom named comfort as her priority — we need to honor that." "This is not about what we want; it is about what she told us she wanted." The document does not remove grief, but it can reduce guessing.

Give Copies to the Right People, and Review It

Once completed, copies should go to the healthcare agent and backup, the primary doctor, the hospital system, any facility, the palliative or hospice team, the important documents folder, and the attorney. Keep the original in a safe but accessible place. Review it if the person moved to another state, a serious diagnosis occurred, dementia is progressing, wishes changed, the named agent changed, the care setting changed, hospice or palliative care has been discussed, or the document is old or missing. Ask an attorney or qualified legal resource whether it is valid, current, and appropriate for the state.

WHAT TO DO THIS WEEK

1. **Find out whether one exists** — check the documents folder, safe, attorney files, and hospital records.
2. **Read it before the crisis** — do not wait until the hospital asks.
3. **Identify what it says** — CPR, ventilators, feeding tubes, comfort care, hospice — and write down the main points.
4. **Talk with the healthcare agent** so they understand the person's wishes.
5. **Share copies** with the doctor, agent, facility, and care team.
6. **Update if needed** — if it is missing, outdated, or unclear, ask an attorney or legal resource.

■ **Worksheet — Living Will / Advance Directive** — fill it in from the printable companion workbook · TheCardinalsPromise.com/resources

■ QUESTIONS FOR THE DOCTOR

CPR / code status

1. What would CPR likely look like for this person, and what is the chance it would help?
2. Would CPR likely lead to ICU care or a ventilator, and would it match the person's goals?
3. Is a DNR or medical order (MOST / POLST) appropriate, and does the current document match the situation?

Ventilator / feeding tube

1. Would a ventilator be temporary or likely long-term, and what happens if the person cannot come off it?
2. What problem would artificial nutrition or hydration solve, and would it improve comfort or cause burden?
3. What would comfort-focused care look like instead?

Comfort-focused care — what matters most (check all that apply)

- | | |
|--|---|
| ☐ Pain relief | ☐ Avoiding hospital transfer |
| ☐ Breathing comfort | ☐ Staying at home if possible |
| ☐ Anxiety relief | ☐ Hospice / palliative support |
| ☐ Quiet room / family present | ☐ Music, prayer, or familiar items |
| ☐ Spiritual support | |

■ CONVERSATION SCRIPTS

With the person: "We want to understand what kind of care you would want if there ever came a time when you could not speak for yourself. What would matter most to you?"

With family: "This conversation is not about giving up. It is about making sure we do not have to guess later."

With the doctor: "Can you help us understand whether the advance directive matches the medical situation we are facing now?"

ROB'S NOTE

Most families don't skip these talks because they don't care. They skip them because they care so much that it hurts.

But staying quiet doesn't protect anyone. It leaves the people you love standing in a hospital hallway. They have to make huge decisions while they're tired and heartbroken.

A Living Will or Advance Directive is not about choosing death. It is about being clear. It says, "Here is what matters to me."

That is a kindness — not only to the doctors, but to the family who would otherwise have to guess.

CHAPTER 21 TAKEAWAY

A Living Will or Advance Directive helps explain what care a person would or would not want if they could not speak for themselves. The next right question is: **what did Mom or Dad say they would want if the decision became hard?**

CARDINAL MOMENT



A son sat by his mother's bed. A doctor asked if they should keep her on the breathing machine. She had never written down what she wanted. She had never really said it out loud, either. So he made the kindest guess he could.

Later, it wasn't the grief that stayed with him. It was the guessing. Did he pick what she would have picked? Now there was no one left who could answer that.

A living will doesn't make the loss any smaller. But it makes sure the choice was hers. Then no one who loved her has to carry a guess for the rest of their life.

A composite drawn from years at the bedside. Names and details changed.

Financial Power of Attorney

THIS CHAPTER IN 30 SECONDS

- **What it does:** names who can manage money, bills, accounts, benefits, and property if your parent cannot — love does not automatically give that authority, and neither does being the adult child.
- **What it does not do:** the money still belongs to the parent — the agent has a fiduciary duty to keep records, avoid mixing money, and pay for the person's care.
- **Watch for:** confirm it is durable (durable usually means it stays effective even if the person becomes incapacitated) and whether authority begins immediately or only after incapacity is documented.
- **The step:** have the agent contact major institutions early and ask: *"We have a Financial Power of Attorney. What is your process for reviewing and accepting this document?"*
- **Pause before moving money:** if Medicaid may be needed later, do not move money, retitle assets, give gifts, or add names to accounts without advice.

WHAT FAMILIES USUALLY THINK

Families often think, "I'm her son, so the bank will talk to me," "I pay Dad's bills already," "My sister is on the account," or "The Healthcare Power of Attorney covers money too." Families can sometimes help informally for a while, but when things get serious, that may not be enough. The bank may not speak with you. The facility may need an authorized signer. A Medicaid application may require financial records. The house may need to be sold. The family may agree emotionally, but the paperwork may still say no. That is why the Financial Power of Attorney matters.

Healthcare POA vs. Financial POA

These are different documents. The Healthcare Power of Attorney answers who can speak to doctors and help make healthcare decisions. The Financial Power of Attorney answers who can manage money, bills, accounts, benefits, and property. A family may need both. The same person may be named for both roles, or different people may be better suited for each — someone may be calm with doctors but terrible with paperwork, or good with money but overwhelmed by medical decisions. Choose wisely.

What It May Allow — and What It Does Not Mean

Depending on the document and state law, it may allow the agent to:

- ☐ pay bills
- ☐ manage bank and retirement accounts
- ☐ handle insurance and Social Security or pension paperwork
- ☐ manage taxes
- ☐ pay caregivers
- ☐ sign care agreements
- ☐ apply for benefits
- ☐ manage or sell property
- ☐ review mail
- ☐ work with accountants and attorneys
- ☐ manage long-term care insurance claims
- ☐ help with Medicaid paperwork

Do not assume the document allows everything — some powers may need to be specifically written. Ask an attorney if the authority is unclear.

It does not mean the agent owns the parent's money, can use it for themselves, or can ignore the person's wishes — and it does not strip siblings of the right to ask reasonable questions. The agent has a fiduciary duty: keep records, avoid mixing money, pay for the person's care, follow the document, avoid self-dealing, and ask for professional guidance when needed. Put plainly: **the money still belongs to the parent. The agent is just trusted to manage it properly.**

Durable, Immediate, or Springing

Durable usually means the document stays effective even if the person becomes incapacitated — which is exactly when families need it. A non-durable power of attorney may stop working at incapacity, depending on state law and wording. Ask whether the document is durable, when it becomes effective, who decides incapacity, what proof is required, and whether banks will accept it.

Immediate vs. springing. Some documents work immediately after signing, letting the agent help with bills and paperwork even while the person still has capacity. Others "spring" into effect only after incapacity is documented — which protects independence but can cause delays if institutions require specific proof. Know which type you have, and make sure it matches the family's reality.

Banks Can Be Difficult

Even with a valid Financial Power of Attorney, banks and financial institutions may have their own review process — they may want an original or certified copy, internal forms, attorney review, an updated document, or identification for the agent. This frustrates families. Do not wait until a bill is overdue or a facility move is pending. If possible, have the agent contact major institutions early and ask what they require. A document in a drawer is good. A document the bank has accepted is better.

When Memory Is Changing, and the Warning Signs

If memory is changing, this document becomes urgent — someone may need to help with bills, accounts, insurance, taxes, care costs, facility contracts, scam protection, and Medicaid or long-term care insurance claims. Financial mistakes happen quietly, and families often notice only after damage is done. Warning signs that help is needed include:

- ☐ unpaid bills and late notices
- ☐ mail piling up
- ☐ confusion about accounts
- ☐ large unexplained withdrawals
- ☐ new "friends" asking for money
- ☐ scam calls
- ☐ missed insurance premiums
- ☐ unfiled taxes
- ☐ a parent who becomes defensive or embarrassed about money

These signs do not mean the parent is bad with money — they may mean the system is now too heavy for one person. If memory is changing, check documents now. Do not wait until capacity is gone.

Choosing the Agent, Keeping Records, and Family Trust

The best financial agent is trustworthy, organized, calm, good with paperwork, available, able to keep records and communicate, not financially dependent on the parent, and able to avoid conflicts of interest. Do not choose someone just because they are the oldest or the loudest, or because they "deserve it." Choose the person who can carry the responsibility cleanly. The agent should keep clear records — bills paid, care expenses, reimbursements, transfers, insurance claims, benefit applications, facility and caregiver payments, and major decisions — in whatever format they will actually maintain. Clear records keep the family's trust intact.

Money can cause conflict: one sibling worries it is spent too fast, another that it buys too little care, a third about inheritance or misuse. The agent should avoid unnecessary secrecy while respecting privacy: "The money belongs to Mom; my job is to use it for her care and keep clear records." "We can talk about the care budget, but the priority is Dad's safety and needs." "If we disagree, let's involve the attorney or advisor instead of guessing." Transparency does not mean everyone controls the checkbook — but secrecy breeds suspicion. Keep things in the open.

Protecting Against Scams and Financial Exploitation

As thinking changes, the risk of being scammed goes up — often before anyone realizes memory is slipping. The same difficulty that makes bills confusing makes a smooth caller sound trustworthy, and scammers specifically target older adults with money, home equity, and good manners. This is not about your parent being foolish. Fraud is a crime aimed at them, and lost dollars are frequently one of the first places cognitive decline shows up in plain sight.

The scams that hurt families most tend to follow a few patterns:

- ☐ **Impersonation** — a caller claims to be the IRS, Medicare, Social Security, a bank "fraud department," or a utility, demanding payment or account details to fix an urgent problem.
- ☐ **The grandchild in trouble** — "Grandma, it's me, I'm in jail, please don't tell Mom, I need money now." Voice-cloning has made these calls frighteningly convincing.
- ☐ **Fake charities** — especially phony veterans, police, or disaster-relief funds that lean on duty and patriotism to rush a donation.
- ☐ **Tech support** — a pop-up or call warns the computer is infected, then asks for remote access that opens the accounts.
- ☐ **Romance and "sweetheart" scams** — a new online friend who, over weeks, becomes someone who needs money.
- ☐ **Prizes and sweepstakes** — you've won, but first you must pay a fee, almost always in gift cards or a wire, which is itself the tell.

The consumer advocate Clark Howard has hammered one rule for years that cuts through most of these at once: **no legitimate business or government agency will ever ask to be paid in gift cards.** The moment a caller says "buy gift cards and read me the numbers," the conversation is over — it is a scam, every time, with no exception worth debating.

GUARDRAILS THAT ACTUALLY HELP

You cannot listen to every call, but you can make fraud slower and harder:

- **The "no decisions by phone, in the moment" rule.** Agree as a family that no money moves, no gift cards, and no account numbers go out on a call — anything real can wait

until a trusted person is looped in. Manufactured urgency is the scammer's main tool.

- **Add a trusted contact to the accounts.** Banks and brokerages let you name someone they can call about unusual activity without giving that person control of the money.
- **Consider a credit freeze** at all three bureaus (Equifax, Experian, TransUnion) — free, reversible, and it blocks new accounts opened in your parent's name.
- **Turn on account alerts** for withdrawals and large transactions so a second set of eyes sees the money move.
- **Register at donotcall.gov** and use call-blocking — fewer calls means fewer chances.
- **Set a family code word** so a panicked "it's your grandson" call can be checked with one question.

If money has already been lost, move fast: call the bank, report to the FTC at reportfraud.ftc.gov and to Adult Protective Services (your state's APS number is in the State Guide near the back of this book), and keep every record. Recovery is not always possible, but reporting can stop the next withdrawal — and being scammed is never something to be ashamed of.

Do Not Move Money Casually

PAUSE BEFORE MOVING MONEY

If Medicaid may be needed later, do not move money, retitle assets, give gifts, or add names to accounts without advice. A well-meaning transfer can create Medicaid problems; a casual gift can become a penalty issue; adding someone to an account can create tax, ownership, creditor, or family problems. Before financial moves, ask an elder law attorney, Medicaid planner, financial advisor, accountant, or appropriate state resource. Caregiving panic is a bad financial advisor.

WHAT TO DO THIS WEEK

1. **Find out whether one exists** — check the documents folder, safe, attorney files, and bank records.
2. **Identify who is named** — the financial agent and a backup.
3. **Confirm whether it is durable** — does it remain valid if the person becomes incapacitated?
4. **Find out when authority begins** — immediately, or only after incapacity is documented?
5. **Check whether banks will accept it** — the agent should contact major institutions and ask what they require.

6. Update if needed — if it is missing, outdated, or unclear, ask an attorney or legal resource.

■ **Worksheet — Financial Power of Attorney** — fill it in from the printable companion workbook · TheCardinalsPromise.com/resources

■ CONVERSATION SCRIPTS

With the person: "We want to make sure bills, care costs, insurance, and benefits can be handled if there is ever a time when you cannot manage them yourself. Who do you trust to help with financial matters?"

With family: "This is not about taking control. It is about making sure someone has legal authority to pay bills, manage care costs, and keep records if Mom or Dad cannot."

With the bank: "We have a Financial Power of Attorney. What is your process for reviewing and accepting this document?"

ROB'S NOTE

A Financial Power of Attorney is not about greed. It is about groceries, bills, care, insurance, and keeping the lights on.

It makes sure someone can pay the care home, call the insurance company, and take care of the bank account. It also protects the person from money mix-ups or harm.

Money can stir up strong feelings in families. That's why the paperwork matters. The person in charge should be someone you trust. The records should be clean. The reason for it should stay clear.

The money belongs to the person getting care. The job is to use it for their care, safety, dignity, and needs.

CHAPTER 22 TAKEAWAY

A Financial Power of Attorney names the person who can handle financial matters if your parent cannot. The next right question is: **who is legally authorized to manage bills, accounts, benefits, and care costs if this person cannot?**

CARDINAL MOMENT



A daughter couldn't pay her father's house payment. All the accounts were in his name. After his stroke, he could no longer sign anything. The teller was kind, but the bank couldn't just take her word for it.

To fix it, she had to go to guardianship court. That meant months of waiting and thousands of dollars. One signed form, done while he was still well, would have taken care of it in one quiet afternoon.

A financial power of attorney is cheap and easy to forget today. But it is worth everything the day you have to keep the lights on for a parent who can't sign anymore.

A composite drawn from years at the bedside. Names and details changed.

Medical Orders — DNR, MOST, and POLST

THIS CHAPTER IN 30 SECONDS

- **What it does:** a living will states wishes; a signed medical order — an out-of-hospital DNR, or a portable order form such as a MOST or POLST — gives instructions that care teams, including EMS, can act on; paramedics generally cannot follow a living will sitting in a folder.
- **What it is not:** a DNR is not "do not treat" — it addresses one thing, CPR; comfort, pain relief, dignity, and medical care all continue, and the order can be changed or voided at any time.
- **Watch for:** this chapter's worksheet is not a DNR, MOST, or POLST and has no legal or medical effect — only an order signed by a healthcare provider on your state's official form directs medical care.
- **How to ask:** *"Which portable medical order form does our state use, and is it appropriate for my parent now?"* — these forms are for people who are seriously ill or frail, not every healthy 70-year-old.
- **Where it lives:** keep the original visible — on the refrigerator, by the bed, or at the front of the chart — let it travel with the person, and review it at every care transition.

WHAT FAMILIES USUALLY THINK

Families often think a DNR means "do not treat," that signing one is giving up, that these forms are for every older adult, or that the living will already covers it. None of that is right. A DNR addresses one thing — CPR — and comfort, pain relief, dignity, and medical care all continue. Portable order forms are intended for people who are seriously ill or frail — someone whose death within the coming year would not surprise their clinician — not for every healthy 70-year-old, who should usually stop at an advance directive. And the order can be changed or voided at any time by the patient (or their surrogate, within state rules). It is not a one-way door. It is a standing instruction that should be reviewed at every care transition.

What Is a DNR?

A Do Not Resuscitate (DNR) order tells healthcare providers not to perform CPR (cardiopulmonary resuscitation) if a person's heart stops or they stop breathing. CPR can

include chest compressions, electric shocks, and breathing support. A DNR does not affect other treatments unless specified — a person with a DNR still receives medical care, comfort care, and treatment for everything else.

What Is a MOST? What Is a POLST?

A **MOST** (Medical Orders for Scope of Treatment) form turns a person's preferences into signed medical orders across a range of treatments — CPR, hospitalization, antibiotics, feeding tubes, and comfort care. Some states use the MOST; many others use a **POLST** — historically "Physician Orders for Life-Sustaining Treatment," now moving to "Portable Orders for Life-Sustaining Treatment" nationally — which does the same job. Names, colors, and signature rules vary by state (in many states a nurse practitioner or physician assistant may sign). Ask the provider: "Which portable medical order form does our state use, and is it appropriate for my parent now?"

Where the Form Lives Matters as Much as What It Says

These forms only work if they can be found in the moment. At home, the original — often a brightly colored form — is usually kept where responders look: on the refrigerator, by the bed, or at the front of the care binder. In a facility, it belongs at the front of the chart. And it should **travel with the person** — to the hospital, to rehab, to assisted living — and be reviewed at every transition and after any major change in health or goals. A signed order locked in a drawer protects no one.

Why These Orders Matter

These medical orders help ensure that a person's wishes are followed during emergencies, that families and caregivers understand what care is wanted, and that healthcare providers have clear instructions when there is no time for a conversation. Have these conversations early — with the provider and with each other. They reduce confusion and panic in the hardest moment a family will face.

WHAT TO DO THIS WEEK

1. **Ask whether a medical order is appropriate now** — these forms are for serious illness and frailty; a healthy parent usually needs only the advance directive.
2. **Find out which form your state uses** — ask the provider, or check Your State Guide at the back of this book.
3. **Have the code-status conversation with the doctor** — ask what CPR would realistically look like for this person (see the questions below).

4. **Get it signed and get copies** — the original stays visible with the person; copies go to the documents folder, the facility, and the decision maker.
5. **Tell the people who would call 911** — family, caregivers, and facility staff should know the order exists and where it is.
6. **Review it at every care transition** — and remember it can be changed or voided at any time.

■ **Worksheet — Thinking About Your Preferences** — fill it in from the printable companion workbook · TheCardinalsPromise.com/resources

■ QUESTIONS TO ASK THE DOCTOR ABOUT CODE STATUS

1. Given my parent's health, what would CPR realistically look like, and what is the chance it would work?
2. Is my parent someone who should have a portable medical order (MOST/POLST) now, or is the advance directive enough?
3. Which form does our state use, who signs it, and how do we complete it?
4. Where should the original live, and who needs copies?
5. What happens if my parent changes their mind — how is the order changed or voided?
6. Will this order follow my parent to the hospital, rehab, or a facility — and who checks it at each transition?

■ CONVERSATION SCRIPT

With the doctor: "We want my parent's wishes to be followed in an emergency, not just written in a folder. Is it time for a portable medical order — in our state, I understand that may be a MOST or POLST — and can we complete it together?"

With family: "A DNR is not 'do not treat.' It is one instruction about CPR. Everything else — comfort, medicine, dignity, care — continues. We are making sure the paramedics know what Mom wants, so nobody has to guess."

ROB'S NOTE

The word DNR scares people. It can sound like a door slamming shut.

But a DNR does not mean "do not treat." It is one order about CPR. Comfort, pain relief, dignity, and care all keep going.

These orders exist so no one has to guess in the hardest moment. The paramedic, the nurse, and the family all know what the person wanted. Talk to the doctor early. Ask what CPR would really mean for this person. Then make the order match their wishes.

That is not giving up. It makes sure love has the last word, not panic.

CHAPTER 23 TAKEAWAY

A DNR, MOST, or POLST turns a person's healthcare wishes into a medical order that care teams — including EMS — can act on, but only if it is signed, current, and findable. The next right question is: **if the heart stops tonight, is there a signed order that says what to do, and can the responders find it?**

CARDINAL MOMENT



A woman told everyone who would listen that she never wanted to be brought back with CPR. But one day she collapsed at home. The paramedics had to try everything, because there was no signed order anywhere in the house.

Her family knew exactly what she wanted. But the people who came through the door had no way to know. What she wanted lived in talks with her family. What the crew could follow had to be on paper.

Wishes live in conversations. Medical orders live on the refrigerator, signed by a doctor. A paramedic is only allowed to follow one of those two.

A composite drawn from years at the bedside. Names and details changed.

Advance Care Planning Conversations

THIS CHAPTER IN 30 SECONDS

- **What it is:** the conversation that ties the documents together — what matters most, who should speak, what care would be wanted and not wanted — before a crisis forces it.
- **What it is not:** not one conversation forever — what someone wanted at 65 may not be what they want at 85; ask again after a serious diagnosis, a hospitalization, or a change in health.
- **How to start:** do not start with machines — start with values: *"I want to understand what matters most to you if your health changes, so we are not guessing later."*
- **Watch for:** a conversation is guidance, a legal document records wishes, a medical order gives instructions — depending on the state and setting, a DNR, MOST, or POLST may still be needed; do not assume one form does every job.
- **If family disagrees:** shift from "What do I want?" to "What would our loved one choose if they could speak clearly today?" — that single question lowers the temperature in many family conflicts.

Why These Conversations Matter

Families often delay because it feels uncomfortable — they do not want to scare Mom, or make Dad think they are giving up, or talk about decline, hospitals, feeding tubes, CPR, hospice, or death. So they wait. Then the crisis comes: a fall, a stroke, a hospitalization, a dementia decline, a doctor asking about code status, a family meeting no one was ready for. That is a hard time to start from zero. Advance care planning reduces guessing. It helps the family understand the person's wishes before emotions are loud and decisions are urgent. It gives the caregiver something solid to hold.

WHAT FAMILIES USUALLY THINK

Families often think, "We know what she wants," "He told me once," "The forms are enough," "Talking about this means we are giving up," or "Everyone will agree." Maybe. But in a crisis, memory gets shaky. One person remembers one conversation, another remembers something different. Someone feels guilty. Someone wants everything done, someone wants comfort,

someone says, "She would never want this," and someone else says, "You are giving up on her." Advance care planning does not remove grief. But it can reduce confusion.

What to Talk About

The conversation should cover:

- ☐ what matters most
- ☐ who should make medical decisions and who should be included in updates
- ☐ what care feels acceptable and what feels too burdensome
- ☐ where the person would prefer to receive care
- ☐ whether they would want hospitalization, ICU care, CPR, a ventilator, or artificial nutrition and hydration
- ☐ whether they would want hospice or comfort-focused care
- ☐ what spiritual or faith needs matter
- ☐ what the family should remember if decisions get hard

It does not need to happen all at once. It can start small. One honest question is enough to open the door.

When to Have the Conversation

The best time is before the emergency:

- ☐ while the person can still speak for themselves
- ☐ before major surgery
- ☐ after a serious diagnosis or a hospitalization
- ☐ when dementia or memory changes begin
- ☐ when falls or weakness increase
- ☐ when chronic illness worsens
- ☐ before an assisted living or memory care move
- ☐ before hospice or palliative care is needed

Advance care planning is not one conversation forever. What someone wanted at 65 may not be what they want at 85, and what they wanted before living with dementia, cancer, heart failure, COPD, kidney disease, or frailty may change after. Ask again.

Who Should Be Involved

Include the people who need to understand the wishes: the person receiving care, the healthcare agent and backup, the spouse, adult children, the main caregiver, a trusted family member or friend, the doctor, and — if involved or desired — the palliative or

hospice team and a clergy or spiritual advisor. Not everyone needs to be in the first conversation. Sometimes the first conversation should be quiet and private. But the decision maker should not be left in the dark. The person named to speak should know what they are being asked to carry.

How to Start

Start gently. You do not need a conference room. You need honesty. Try: "I want to make sure we understand what matters most to you if your health changes." Or: "If there ever came a time when you could not speak for yourself, what would you want us to know?" Or: "I know this is not easy to talk about, but I do not want us guessing later." Do not start with machines. Start with values — with what matters. The medical details can come later.

Wishes vs. Orders, and When It Is Hard

The difference between wishes and orders. A conversation is guidance. A legal document records wishes. A medical order gives instructions to medical professionals. They are connected but not the same — a person may say they do not want CPR and write it in an advance directive, but depending on the state and setting, a DNR, MOST, or POLST may still be needed. Ask the doctor: "Do the documents we have match the medical orders needed for this situation?" Do not assume one form does every job.

If the person refuses to talk, that does not mean the family failed. Try again later with smaller questions. Instead of "Do you want CPR?" ask, "What matters most to you if you get sicker?" Instead of "Do you want hospice?" ask, "Would you want care focused on comfort if treatment was no longer helping?" Instead of "Where do you want to die?" ask, "Where do you feel safest?" Some doors open slowly. Do not kick them down.

If family members disagree, the most important shift is from "What do I want?" to "What would our loved one choose if they could speak clearly today?" That single question lowers the temperature in many family conflicts. It moves the discussion away from guilt and toward honor. Stay anchored to the person's wishes: "Let's come back to what Mom said mattered most." "Dad told us he wanted comfort if recovery was unlikely." "This is not about what we want. It is about what she told us." The families with the least regret are not the ones who made perfect decisions. They are the ones who knew what their loved one wanted. Advance care planning gives the family a compass. It does not calm the storm, but it keeps people from walking in circles.

WHAT TO DO THIS WEEK

1. **Choose one calm time** — not during a crisis or argument if you can avoid it.

2. **Choose the first person to talk with** — start with the person receiving care, if they can participate.
3. **Ask one values question** — "What matters most to you if your health changes?" — and write the answer down.
4. **Identify the decision maker** — who should speak if the person cannot.
5. **Connect the conversation to documents** — which POA, directive, or medical orders exist?
6. **Share the key wishes** so the healthcare agent and main caregiver understand what was said.

■ **Worksheet — Advance Care Planning Conversation** — fill it in from the printable companion workbook · TheCardinalsPromise.com/resources

■ QUESTIONS FOR THE DOCTOR

1. Do the current documents match the person's medical situation, and are any medical orders needed?
2. Would a DNR, MOST, or POLST be appropriate?
3. What would CPR likely look like for this person, and what would hospitalization likely accomplish?
4. Would palliative care be helpful, and would hospice ever be appropriate?
5. What should the family understand now?

■ CONVERSATION SCRIPTS

To begin: "I want to understand what matters most to you if your health changes, so we are not guessing later."

If the person is nervous: "This does not mean anything is happening today. It just means your wishes matter enough to write them down."

With family: "We need to listen to what Mom or Dad wants, not just what each of us fears."

With the doctor: "Can you help us understand what medical decisions we should talk about now, before there is a crisis?"

ROB'S NOTE

Advance care planning is not a talk about death. It is a talk about love.

It says, "Your voice matters, even if one day you can't use it." It tells the family what to do when things get hard. It gives the healthcare agent something to hold onto besides guilt.

The goal is not to guess every medical problem ahead of time. You can't. The goal is to understand the person. What matters to them? What scares them? Who do they trust? What do they hope for? What would they want the family to remember?

Start there. The forms matter. But the talk is the heart of it.

CHAPTER 24 TAKEAWAY

Advance care planning helps the family understand a person's wishes before a crisis. The next right question is: **have we talked honestly about what matters most, who should speak, and what care would match this person's values?**

CARDINAL MOMENT



A father kept putting off "the talk." Not yet, not yet, he said, every time his children asked what he wanted at the end. There was always a better day for it. Then one morning, a stroke took that chance away.

They spent his last months guessing at everything he never got to tell them. Again and again, they wished they had just gotten through one hard dinner while there was still time.

The talk is never easy, and the timing is never right. Have it anyway, and have it early. Do it while the person can still tell you, in their own words, what matters to them.

A composite drawn from years at the bedside. Names and details changed.

Medication and Doctor Lists

THIS CHAPTER IN 30 SECONDS

- **What it is:** one current list of everything actually being taken — prescriptions, over-the-counter drugs, vitamins, supplements, drops, patches, insulin — plus allergies and recently stopped medications.
- **Why it matters:** it helps prevent confusion, delays, duplicate prescriptions, missed doses, and unsafe combinations. The test: can we hand the care team one accurate list today?
- **Bring the list, not just the bottles:** a bottle shows what was prescribed, not what is actually being taken.
- **One owner:** choose one medication-list person to keep it updated — if three people have three different lists, the family does not have a list.
- **How to ask:** *"Can we review the medication list to make sure everything is still needed, safe, and appropriate?"* Do not stop medications on your own.

Why the Medication List Matters

A medication list helps the care team understand what is actually being taken — not what was prescribed three years ago, not what is sitting in the cabinet, not what one sibling remembers. What is being taken now. It helps with doctor appointments, ER visits, hospital admissions, rehab, home health, hospice, assisted living or memory care move-ins, refills, side-effect concerns, fall-risk reviews, and end-of-life comfort planning. Include prescriptions, over-the-counter medications, vitamins, supplements, creams, inhalers, eye drops, insulin, patches, and as-needed medications. If it goes in, on, under, or near the body for medical reasons, write it down.

WHAT FAMILIES USUALLY THINK

Families often think, "Mom knows what she takes," "The pharmacy has the list," "The doctor has it in the chart," or "We will bring the bag of bottles." But medication lists are often wrong. Doctors change medications, specialists add them, hospitals stop them, rehab restarts them, pharmacies refill old prescriptions, and patients keep taking discontinued pills. Families forget over-the-counter drugs; supplements, eye drops, and inhalers drop out of the conversation. The medication list needs one owner. Someone has to keep it clean.

What Belongs on the List

For each medication, write:

- ☐ the name
- ☐ dose
- ☐ how often and what time
- ☐ the reason (if known)
- ☐ the prescribing doctor
- ☐ the pharmacy
- ☐ the start date (if known)
- ☐ special instructions
- ☐ whether it is daily or as-needed
- ☐ any side effects or concerns

Also include:

- ☐ allergies and medication reactions
- ☐ vitamins, supplements, and over-the-counter drugs
- ☐ pain, sleep, and anxiety medications
- ☐ blood thinners
- ☐ diabetes medications
- ☐ inhalers, eye drops, creams, patches, insulin, and oxygen
- ☐ medical marijuana or CBD if used and legal in the state
- ☐ any medication recently stopped

The stopped medications matter too — a discontinued medication sitting in the cabinet has a way of coming back.

Bring the List, Not Just the Bottles

Bottles can help, but bottles alone are not enough. A bottle shows what was prescribed, not what is actually being taken. The dose may have changed, the medication may have been stopped, old bottles may be mixed with new ones, and duplicates may come from different pharmacies. Bring the bottles when useful, but bring the list every time. The list should travel.

When to Update, and the Medication Review

Update the list:

- ☐ after every doctor appointment, hospital visit, or rehab discharge
- ☐ when home health or hospice starts

- ☐ when a medication is added, stopped, or a dose changes
- ☐ when the pharmacy changes
- ☐ after a fall or worsening confusion, dizziness, weakness, sleepiness, or appetite change
- ☐ after a new diagnosis
- ☐ before an assisted living or memory care move

Do not wait for the annual checkup. Medication changes are little hinges that swing big doors.

Ask for a medication review if the person takes many medications, has new confusion, is falling, feels dizzy, sleeps more, has poor appetite, or has multiple prescribers. A review can find duplicates, unnecessary medications, side effects, interactions, wrong doses, medications that raise fall risk or worsen confusion, medications that no longer match the person's goals, and expensive drugs with alternatives. Ask: "Can we review the medication list to make sure everything is still needed, safe, and appropriate?" Do not stop medications on your own. Ask.

The Doctor List, and One Owner

The doctor list is the medication list's twin — one shows what is being taken, the other who is involved. Include the primary care provider, specialists, pharmacy, hospital preference, home health, hospice, palliative team, facility nurse, private duty agency, equipment company, dentist, eye and hearing providers, therapist, and care manager, with each name, role, phone, portal info, last visit, next appointment, and main reason involved. In a crisis, "the heart doctor" is not enough — write the name down.

The family should choose one medication-list person, responsible for keeping the list updated and sharing the newest version. This does not mean one person does all the caregiving — it means one person guards the list. Choose someone organized, reachable, and willing to update details, and share the list with the main caregiver, healthcare agent, doctor, facility, and anyone taking the parent to appointments. If three people have three different lists, the family does not have a list.

WHAT TO DO THIS WEEK

1. **Gather all medications** — bottles, pill packs, inhalers, drops, creams, vitamins, supplements, patches, and OTC drugs.
2. **Write down what is actually being taken** — ask how each medication is really used, not just what the bottle says.
3. **Add the pharmacy name and phone number.**

4. **List every doctor and care contact** — specialists, facility, home health, hospice, and pharmacy.
5. **Ask for a medication review** for safety, duplicates, side effects, and cost.
6. **Put the list in the documents folder** — paper and digital — and update it whenever something changes.

■ **Worksheet — Medication and Doctor List** — fill it in from the printable companion workbook · TheCardinalsPromise.com/resources

■ APPOINTMENT QUESTIONS

1. Is this medication list accurate, and are any medications duplicated or no longer needed?
2. Could any medication increase fall risk, worsen confusion, or cause dizziness or sleepiness?
3. Are there lower-cost alternatives?
4. Are there medications that should not be taken together?
5. Should any medications be taken at a different time?
6. What should we do if a dose is missed?

HOPE'S NOTE — FROM THE PHARMACIST

When a family hands me a current, complete medication list, the whole visit changes — we catch duplicates, interactions, and doses that no longer fit the person. When they hand me a bag of bottles, I am guessing about what is actually being taken.

Bring the list. Update it every time something changes. And use one pharmacy if you can — it is the only way any pharmacist ever sees the whole picture.

ROB'S NOTE

A good list of medications can save a life.

It keeps your parent from having to guess. It keeps the doctor from missing something. It keeps the caregiver from panicking. And it keeps the family from standing in the emergency room, trying to remember if the little white pill was for blood pressure, sleep, worry, or pain.

Write it down. Keep it up to date. Bring it with you.

The list doesn't have to look nice. It just has to be right.

CHAPTER 25 TAKEAWAY

A current medication and doctor list helps the family and care team make safer, faster, clearer decisions. The next right question is: **can we hand the care team one accurate list today?**

CARDINAL MOMENT



An ER doctor asked a daughter what medicines her mother took. All she could say was "a white one, and a little blue one." Not knowing cost hours. And it forced a risky guess at the worst possible time.

After that night, she kept an up-to-date list in her wallet and on her phone. It had every medicine, every dose, and every doctor. The next emergency visit took minutes instead of hours, because the answer was already in her hand.

An up-to-date list of medicines and doctors is the most useful page you can carry. Update it the same day anything changes, not the day you need it.

A composite drawn from years at the bedside. Names and details changed.

Emergency Contacts and Crisis Plan

THIS CHAPTER IN 30 SECONDS

- **What it is:** one page that answers the crisis question — who do we call first? — so the caregiver can find the right number in thirty seconds.
- **Call 911:** for immediate danger or a life-threatening emergency. If you are unsure whether something is life-threatening, call 911 — do not let a checklist delay emergency help.
- **Otherwise, sometimes the first call is** the doctor or nurse triage line, hospice, home health, or the facility — say *"Something has changed, and I need guidance on what to do next,"* then describe the change clearly.
- **Build the call chain:** one family point person (with a backup) updates everyone else — in a crisis, roles beat opinions.
- **A caregiver crisis is still a crisis:** *"The caregiver cannot safely continue at this level. We need help making a safer plan today."* That is not weakness. That is honesty.

Why This Matters

Most families have contact information scattered everywhere — a doctor's number in one phone, the pharmacy on a pill bottle, the hospice number in a folder, the assisted living nurse's name in a text thread, the power of attorney in a drawer, and an out-of-town sibling who has no idea who to call. That works until something happens. Then the family loses time, and time matters. An emergency contact plan does not solve every problem, but it keeps the family from spinning. It creates a first step.

WHAT FAMILIES USUALLY THINK

Families often think, "We'll just call 911," "The facility will handle it," "My sister knows who to call," or "We can figure it out." Sometimes 911 is the right call. Sometimes the doctor, hospice, facility nurse, home health agency, or on-call provider should be called first — and calling 911 before calling hospice or the facility team can create confusion the family did not want. Sometimes waiting too long creates danger. The family needs a plan. Not a perfect plan. A usable one.

When to Call 911

CALL 911 IMMEDIATELY

For immediate danger or a life-threatening emergency: chest pain, severe trouble breathing, signs of stroke, uncontrolled bleeding, major injury, unconsciousness, a severe allergic reaction, a suspected overdose, a serious fall with injury, a person who cannot be safely moved, immediate danger to self or others, or fire, violence, or an unsafe environment. **If you are unsure whether something is life-threatening, call 911. Do not let a checklist delay emergency help.**

When to Call the Doctor, Hospice, Home Health, or Facility

The doctor or nurse triage line for concerns that are serious but not clearly life-threatening — fever, new weakness, worsening confusion, medication side effects, new swelling or pain, dehydration, a recent fall without obvious major injury, or a question about whether the ER is needed. Use this script: "Something has changed, and I need guidance on what to do next," then describe the change clearly.

Hospice, if the person is on it, for changes related to comfort, symptoms, decline, or end-of-life concerns — pain, shortness of breath, agitation, nausea, breathing changes, not eating or drinking, or a family sense that death may be near — unless there is immediate danger requiring 911. Say: "We are on hospice and something has changed. I need the nurse or on-call team to guide us." Ask hospice who to call after hours and put that number where everyone can find it.

Home health for questions about the plan of care, wounds, therapy, nursing visits, medication teaching, equipment, or safety concerns being monitored by the team. Home health is not an emergency service — if there is immediate danger, call 911.

The facility, if the person lives in assisted living, memory care, rehab, or a nursing home. Ask in advance who to call during the day and after hours, who the nurse and administrator are, who calls the doctor, what changes require family notification, and what changes require 911. Do not wait until the facility calls at midnight — get the contact plan in writing.

The Family Call Chain

A call chain prevents ten people from calling ten people with half-information. Choose one family point person who updates the rest of the family; if that person is unavailable, a backup takes over. The plan should answer: Who gets called first? Who calls the doctor or

agency? Who updates family? Who goes to the hospital? Who stays with the spouse or other parent? Who brings the document folder? Who handles medications, pets, home, or transportation? In a crisis, roles beat opinions.

What to Keep Ready, and What to Say

The emergency page should include:

- ☐ the person's full name, date of birth, and address
- ☐ main diagnosis and allergies
- ☐ where the medication list is
- ☐ doctor names and numbers
- ☐ pharmacy
- ☐ insurance information
- ☐ the healthcare agent
- ☐ family, facility, home health, and hospice contacts
- ☐ preferred hospital
- ☐ DNR/MOST/POLST status if applicable
- ☐ funeral home if chosen
- ☐ the document folder location

Print it, store it digitally if possible, and put it where the caregiver can grab it.

A clear call gets better help. When you call, give the name of the person receiving care, their age, location, main diagnosis, what changed and when, current symptoms, any recent medication change, and what you need. For example: "My mother, Mary Smith, is 84 and lives at home. She has dementia and heart failure. Since this morning she is much weaker, more confused, and not eating. She fell yesterday but did not go to the hospital. I need guidance on whether she should be seen today." Simple. Clear. Useful.

If the Caregiver Breaks Down

A caregiver crisis is still a crisis. Call for help if the caregiver:

- ☐ has not slept
- ☐ feels unsafe
- ☐ cannot lift or transfer the person
- ☐ cannot manage medications
- ☐ feels emotionally overwhelmed
- ☐ is afraid they may yell or collapse
- ☐ cannot leave the person alone

- ☐ has their own medical emergency
- ☐ is no longer able to provide safe care

Families sometimes wait until the caregiver is completely broken. Do not wait. Use this script: **"The caregiver cannot safely continue at this level. We need help making a safer plan today."** That is not weakness. That is honesty.

WHAT TO DO THIS WEEK

1. **Choose the main family point person** and a backup.
2. **Write down the most important medical contacts** — doctor, specialists, pharmacy, facility, home health, hospice, and preferred hospital.
3. **Write down when to call 911** so the caregiver knows what counts as immediate danger.
4. **Put the contact page somewhere visible** — refrigerator, medication area, caregiver binder, or hospital bag.
5. **Build the family call chain** — who calls whom, and who does what.
6. **Share the plan** with the caregiver, point person, healthcare agent, backup caregiver, and facility.

■ **Worksheet — Emergency Contacts & Crisis Plan** — fill it in from the printable companion workbook · TheCardinalsPromise.com/resources

■ CRISIS CALL SCRIPT

"My name is _____. I am calling about _____, who is _____ years old and is located at _____. The main diagnosis is _____. Something changed: _____. It started around _____. Current symptoms are _____. The medication list is at _____. I need guidance on what to do next."

ROB'S NOTE

A crisis plan won't feel like a big deal until the crisis comes. Then it's worth its weight in gold.

The number on the fridge. The hospice line by the phone. The doctor's name written clearly. The one family member already picked to lead. The medication list in the folder.

That's what helps a family move. Not because everything is easy, but because the first step is clear.

When people are scared, they don't need more noise. They need a number, a name, and the next right call.

CHAPTER 26 TAKEAWAY

An emergency contact and crisis plan gives the family one clear place to find who to call and what to say when something changes. The next right question is: **can the caregiver find the right number in thirty seconds?**

CARDINAL MOMENT



When their father fell at two in the morning, the brothers and sisters found out that none of them knew who to call first. They didn't know which hospital he should go to. And they didn't know where he had written down that he never wanted to be put on machines. So they figured it out as they went, badly, in the dark.

Later, when things were calm, they wrote one page. It had the numbers, the hospital, the medicines, and his wishes. They taped it inside the kitchen cabinet where anyone could find it. The next scare, they weren't guessing.

A crisis is the worst time to plan for a crisis. Write the one page while it's quiet. The scared version of you at 2 a.m. will thank you.

A composite drawn from years at the bedside. Names and details changed.

STAY CONNECTED

For caregiver tools, updates, and Rob's writing, visit
thecardinalspromise.com.

Why I Wrote This Guide

My father, Lou Brizzi, died on Christmas Day, 2025. Like most sons, I figured there'd be more time. I figured I'd be at his bedside the way I had been so many times before. I wasn't.

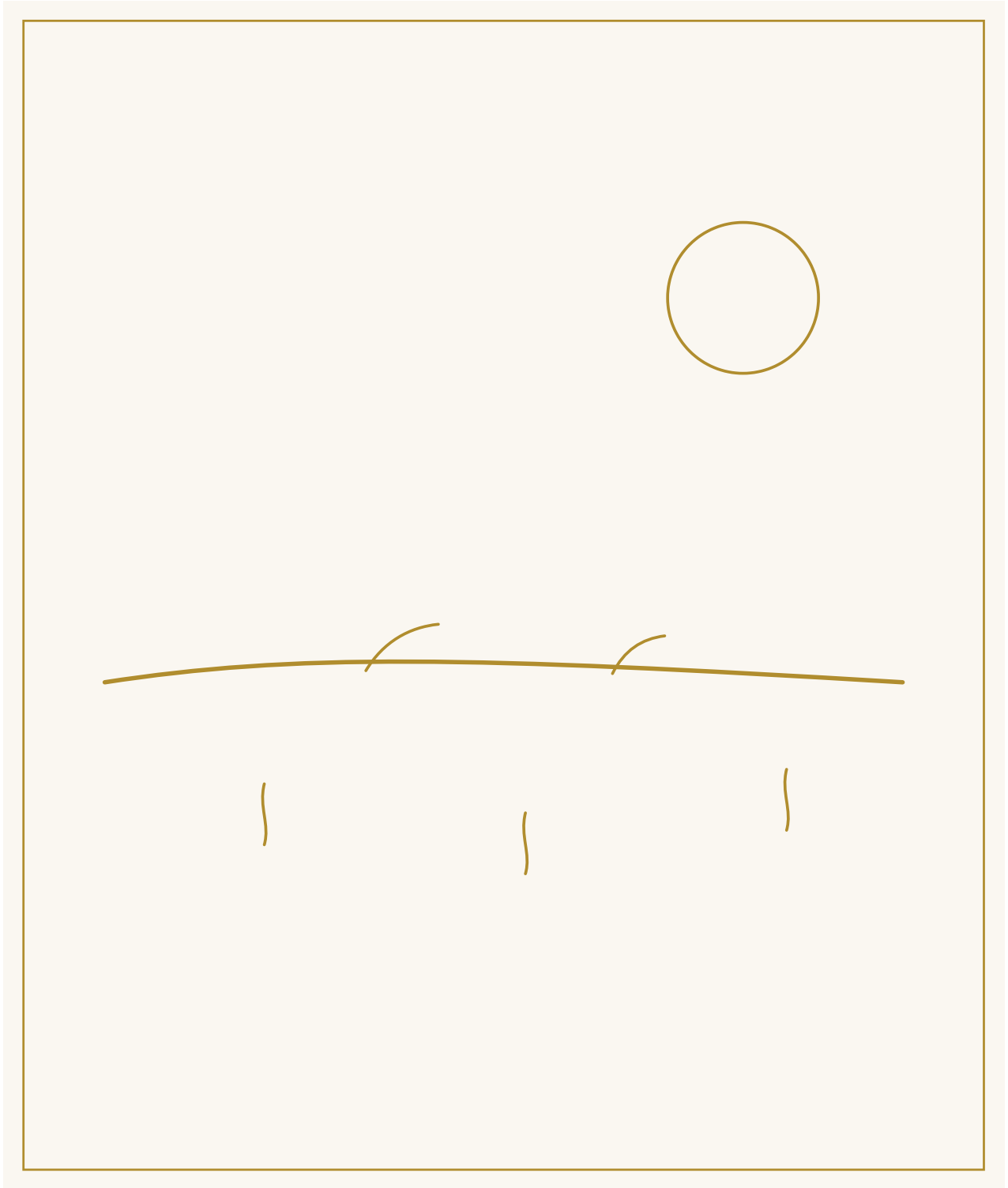
Two weeks later, on January 9, 2026, I was working — because that's what Lou taught me to do. Suit and tie, the same way he wore one for more than forty years. I was sitting in a facility parking lot when a cardinal landed on my driver's-side mirror. It was my seventeenth anniversary of being sober. Lou had pulled me out of a life that nearly ended in a West Palm Beach apartment, and there he was again. Still showing up.

I started journaling to get through the grief, and through the guilt of not being there at the very end. I wrote so much that my wife, Hope, finally told me to turn it into a book.

That book became *The Cardinal's Promise*. It's with an editor now. While I was writing it, I kept circling back to all the decisions that get dropped on families when someone they love starts to decline — and how almost nobody hands you a map. So I wrote one. *The Cardinal's Promise* is the raw version of my story, of being adopted by Lou and finding my way back to a life worth keeping. This is the guide that goes with it. For a lot of families, just knowing where to look is the whole difference between getting the help your parent needs and missing it completely.

This isn't a feel-good memoir. It's a working guide — built to knock down the myths and help you ask sharper questions when it counts.

The Cardinal's Promise is my story. This guide is yours.



Even here, there is comfort to give.

The Last Days Plan

Part 5 — Hospice and the Last Days. This is the most tender part of the guide. These chapters walk with a family through the final season — what the last days may look like, what comfort means, what to expect and what to do, and what happens after death. The goal is not to control death or predict its timing. It is to help the family show up — calmer, clearer, and more present — for the person they love. If someone is in immediate danger and emergency treatment is wanted, call 911.

“You matter because you are you, and you matter to the end of your life.”

— DAME CICELY SAUNDERS, FOUNDER OF THE MODERN HOSPICE MOVEMENT

THIS CHAPTER IN 30 SECONDS

- **What it is:** a plan made before the final days — what to expect, who to call first, what comfort means, where the documents are, who should be present, and what to do when death occurs.
- **What it is not:** predicting the exact moment — no one can do that. It is preparing the family so the room can become calmer, softer, and more focused on the person receiving care.
- **Comfort becomes the priority:** comfort care is not doing nothing — it is doing the right things when the body is changing. Never guess with comfort medications; let the care team teach the family, and write down the instructions.
- **Food and fluids:** do not force them — ask the care team about comfort feeding; mouth care often becomes more helpful than pushing intake.
- **If family disagrees:** return to the person's wishes — *“This is hard for all of us. Let's come back to what Mom wanted and what keeps her most comfortable now.”*

Why This Plan Matters

Families often wait until the final days to ask the hardest questions: Who should be called? Should we go to the hospital? Who has the hospice number? What medications are for comfort? Where is the DNR? Should we keep offering food? What if breathing changes? What if they stop waking up? Who needs to say goodbye? What happens when

death occurs? Without a plan, fear fills the room. With a plan, the family still grieves, but they are not guessing every minute. A last days plan gives everyone something steady to hold.

WHAT FAMILIES USUALLY THINK

Families often think, "We will know what to do," "The nurse will be there the whole time," "We should call 911 if breathing changes," "If they stop eating, we are starving them," or "If we give comfort medicine, we are causing death." These thoughts come from fear and love. But fear and love need guidance. The last days plan helps turn panic into care.

What the Last Days May Look Like

Every person is different, but families may notice:

- ☐ more sleeping
- ☐ less interest in food and fluids
- ☐ more weakness
- ☐ less talking
- ☐ more confusion
- ☐ changes in breathing
- ☐ cool hands or feet
- ☐ changes in skin color
- ☐ restlessness
- ☐ less urine
- ☐ difficulty swallowing
- ☐ pulling away from activity
- ☐ moments of alertness followed by sleep
- ☐ seeing or talking to people who are not present
- ☐ less ability to take pills
- ☐ more need for mouth care and repositioning

These changes can be frightening if the family does not expect them. Ask the hospice nurse, palliative care team, doctor, or facility nurse what signs to watch for and what to do. Do not rely on internet panic. Use the care team.

Comfort Becomes the Priority

In the last days, comfort may become the main goal:

- ☐ pain relief

- ☐ breathing comfort
- ☐ anxiety and restlessness support
- ☐ mouth care
- ☐ gentle repositioning
- ☐ skin care
- ☐ a quiet room
- ☐ dim lights
- ☐ familiar music
- ☐ prayer
- ☐ family presence
- ☐ welcome touch
- ☐ fewer interruptions
- ☐ avoiding unwanted transfers
- ☐ honoring the person's wishes

Comfort care is not doing nothing. It is doing the right things when the body is changing.

Food, Fluids, and Breathing Changes

Food and fluids. Families often struggle when a loved one stops eating or drinking, because food and water are love. But near the end of life the body changes — appetite decreases, swallowing may become harder, and food and fluids may no longer be helpful to the person. Ask the care team whether it is safe to offer food or fluids, what swallowing trouble looks like, whether to offer small sips or mouth swabs, and what comfort feeding means. **Do not force food or fluids.** Offer comfort. Mouth care often becomes more helpful than pushing intake.

Breathing changes. Breathing may become slower, faster, irregular, noisy, shallow, paused, or labored-looking. Call the care team when breathing changes and ask what is expected, what comfort medications may help, and when the nurse should come. If the person is on hospice, call hospice; if they are not on hospice and there is immediate danger or the family wants emergency treatment, call 911. Know ahead of time who to call first, whether you are trying to avoid hospital transfer, whether there is a DNR/MOST/POLST, and what comfort looks like. A breathing change should not be the first time the family asks these questions.

Comfort Medications

Families may be afraid of comfort medications, worrying that giving them means causing death. Discuss this fear directly with the nurse or doctor. Ask what each medication is for,

when and how much to give, how often, what to watch for, what to do if the person cannot swallow, who to call if it does not work, and how to know the person is comfortable. **Never guess with comfort medications.** Let the care team teach the family, and write down the instructions.

The Room, the People, and What to Say

Who should be there. The plan should include who to call if time becomes short — spouse, children, siblings, close friends, clergy or a spiritual advisor, the hospice chaplain, and anyone who needs a chance to say goodbye. Not everyone needs to be in the room at once. Sometimes the most loving room is quiet; sometimes it is full. Ask what the person would want.

What the room should feel like. A room can be cared for too. Consider music or quiet, prayer, dim lights, the TV off, familiar photos or blankets, limited or open visiting, and soft voices. Remember the person may still hear or sense presence. Speak with respect. Tell stories. Say what needs to be said. Do not argue in the room — the room is not the place for family politics.

What to say. There is no perfect script. Simple words are enough: "I love you." "Thank you." "I forgive you." "Please forgive me." "We are here." "You are not alone." "We will take care of each other." "You can rest." Not every family can say every sentence. That is okay. Say what is true. Say what brings peace.

If Family Disagrees, and When Death Occurs

If family members disagree in the last days — one wants the hospital, another comfort at home; one fears medication, another fears suffering — return to the person's wishes. Ask what they said they wanted, what documents exist, who the healthcare decision maker is, and what the hospice nurse recommends. Use this script: "This is hard for all of us. Let's come back to what Mom wanted and what keeps her most comfortable now." The goal is not to win the argument. The goal is to protect the person.

When death occurs, the family should already know what to do. Ask ahead: Who do we call first? If on hospice, do we call hospice? If in a facility, who does the facility call? Who pronounces death? Do we call 911? Is a funeral home selected, and who calls them? Who notifies family? What happens to medications and equipment? Who stays with the body until the funeral home arrives? What spiritual or cultural wishes should be honored? If the person is on hospice, the hospice team should explain the process clearly. Write it down before the moment comes.

WHAT TO DO THIS WEEK

1. **Ask what signs to expect** from the hospice nurse, palliative team, doctor, or facility nurse.
2. **Confirm who to call first** — hospice, facility nurse, doctor, or 911, depending on the situation.
3. **Review documents** — POA, advance directive, DNR/MOST/POLST.
4. **Choose the family point person** to update family and coordinate calls.
5. **Decide who should be notified** before the final hours.
6. **Ask about comfort medications and the death process** — have the nurse or doctor explain both.

■ **Worksheet — Last Days Plan** — fill it in from the printable companion workbook · TheCardinalsPromise.com/resources

ROB'S NOTE

The last days are sacred. They are also practical.

Someone needs the number. Someone needs the medication instructions. Someone needs to know who to call. Someone needs to know where the papers are. And someone needs to tell the family.

That doesn't make the moment any less holy. It makes it more protected.

The goal isn't to control death. The goal is to care well until it comes. Quietly. Clearly. Together.

CHAPTER 27 TAKEAWAY

A last days plan helps the family prepare for comfort, communication, calls, documents, visitors, and what to do when death occurs. The next right question is: **what can we put in place now so the final days are less chaotic and more peaceful?**

CARDINAL MOMENT



A family knew the end was close. But no one had ever said out loud where they wanted it to happen. So when the moment came, they panicked and called 911. Their mother spent her last two days in an ambulance and in the emergency room she had begged never to go back to.

What hurt them afterward wasn't that she died. It was where she died. It was a place she had asked not to be, all because there was no plan that could have fit on an index card.

The last days will come, plan or no plan. A plan is simply how you make them hers, instead of the system's.

A composite drawn from years at the bedside. Names and details changed.

What to Expect in the Last Days

THIS CHAPTER IN 30 SECONDS

- **What families may see:** more sleeping, less eating and drinking, breathing changes, cool hands or feet, restlessness, visions, sometimes a rally — every person is different. Even then, hearing may remain: speak as if they can hear.
- **Food is love, but:** offer, do not force — the family is not starving the person; the illness is changing the body. Often mouth care becomes more helpful than forcing fluids.
- **Breathing changes:** often the most frightening part — noisy breathing does not always mean the person is suffering, but still call the care team and ask for guidance. If on hospice, call hospice.
- **Comfort medications:** use them only as directed — families should never be left guessing. Ask what each is for, when and how much to give, and when to call.
- **If unsure, ask:** *"We are seeing changes and we are not sure what they mean. Can you help us understand what is expected, what may be discomfort, and what we should do next?"*

Why Families Need to Know

The body often changes in the last days, and these changes can look scary if no one explains them. A family may panic when food intake drops, fear breathing changes, worry that comfort medication is too strong, or think the person is suffering when the body is naturally slowing. Knowing what to expect does not remove grief, but it can reduce panic. It helps the family stay present, call the right person, and focus on comfort instead of fear.

WHAT FAMILIES USUALLY THINK

Families often think, "If they stop eating, they will starve," "If breathing changes, we should call 911," "If they cannot talk, they cannot hear us," "If we give comfort medicine, we are speeding things up," or "If they rally, they are getting better." Some of these thoughts come from love, some from fear, most from not knowing. The care team should explain what changes are expected and what needs immediate attention. Families should never be left guessing.

Sleeping, Eating, and Drinking Less

More sleeping. The person may sleep much more, be awake only briefly, and drift in and out. This is hard for family members who want one more long talk, but sleepiness is often part of the body slowing down. Sit nearby, speak softly, avoid forcing alertness, keep the room calm, tell them who is present, and say what needs to be said. Do not assume they cannot hear.

Eating less. The person may refuse meals, take only bites, or cough. Food is love, but near the end of life the body may not need or tolerate food the same way, and forcing it can cause distress, choking, or aspiration. Ask the care team whether it is safe to offer food, which foods are safest, and what comfort feeding means. Offer, do not force; use small amounts; stop if coughing or distress occurs; keep the mouth clean and moist. The family is not starving the person. The illness is changing the body.

Drinking less. The person may stop asking for fluids, have trouble swallowing, or cough with liquids. Often mouth care becomes more helpful than forcing fluids. Ask whether small sips or thickened liquids are safe and how often to do mouth care. Offer small sips only if safe, use mouth swabs if recommended, apply lip balm, keep the mouth moist, and do not force fluids. Comfort is the guide.

Breathing Changes

Breathing changes are often the most frightening part for families. Breathing may become slower, faster, uneven, shallow, noisy, paused, wet-sounding, or labored-looking. (Families sometimes hear noisy, wet breathing called "the death rattle" — unsettling to hear, but usually not a sign of suffering.) Call the care team when breathing changes — if the person is on hospice, call hospice. Ask whether it is expected, whether the person appears uncomfortable, whether to reposition, whether comfort medicine or oxygen should be used, and what signs mean urgent help is needed. Stay calm, keep the room calm, reposition gently if instructed, use comfort medications only as directed, avoid crowding, and speak softly. Noisy breathing does not always mean the person is suffering — but still call and ask for guidance.

Skin, Circulation, and Urine

Cool hands and feet. Hands, feet, arms, or legs may become cool, and skin may look pale, gray, bluish, or mottled as circulation changes. Use light blankets if comfortable, do not use heating pads unless instructed, avoid heavy blankets that cause discomfort, and tell the nurse what you see. The goal is comfort, not warming the body back to normal — the body may be doing what it does near the end.

Less urine. Urine may decrease and darken as intake drops and the body slows. Call the nurse if there is no urine for a long time, signs of discomfort, bladder fullness, fever, pain, agitation, or catheter concerns. Track changes and keep the person dry and clean. Do not guess. Ask.

Restlessness, Visions, and Pain

Restlessness or agitation. Some people become restless, pull at blankets, try to get up, call out, or seem anxious. Ask the care team whether it could be pain, anxiety, urinary retention, constipation, or medication, and what comfort step to use. Stay calm, reduce noise, keep the person safe, do not argue with what they are seeing, offer reassurance, and use medications only as directed. Restlessness is not a family failure. It is a symptom to report and manage.

Confusion or visions. Some people talk to loved ones who have died, reach toward something, or speak about going home. Often correction is not needed. Listen, reassure, avoid arguing, do not shame, and let the moment be peaceful. Sometimes the kindest response is: "You are safe. We are here with you."

Pain. Do not assume pain is unavoidable. Signs may include grimacing, moaning, restlessness, guarding, a tense body, faster breathing, or a furrowed brow. Ask how to know if they are in pain, what medication is ordered, when to give it, what to do if they cannot swallow, how quickly it should help, and when to call. Pain management is a central goal of comfort care. The family should not have to improvise.

The Rally, and Hearing

The rally. Sometimes a person has a period of unexpected alertness — they wake, talk, ask for food, or recognize people, and the family feels hopeful. The rally can be meaningful and beautiful, but it does not always mean recovery. Receive the moment, say what matters, do not overload the person, let them rest afterward, and avoid making sudden major decisions based only on one good hour. A rally can be a gift. Let it be one.

Hearing may remain. Even when a person cannot respond, they may still hear or sense presence. Speak as if they can hear: "We are here." "You are safe." "We love you." "Thank you." "You can rest." Avoid arguing in the room, and avoid talking over the person as if they are already gone. The room should protect their dignity.

When to Call the Care Team

Call when:

- ☐ breathing changes
- ☐ pain seems uncontrolled
- ☐ restlessness increases
- ☐ the person cannot swallow medications
- ☐ the family is unsure what to do
- ☐ there is a fall or bleeding
- ☐ there is fever or distress
- ☐ skin changes are concerning
- ☐ the caregiver is overwhelmed
- ☐ death seems near
- ☐ death has occurred

If on hospice, call hospice. If not on hospice and emergency treatment is wanted or needed, call 911. If unsure, call for guidance. Families should not sit alone with unanswered fear.

WHAT TO DO THIS WEEK

1. **Ask what changes to expect** from the nurse, doctor, hospice, or palliative team.
2. **Ask what symptoms require a call** — write down when and who to call.
3. **Review comfort medications** — know what each is for and how to use it safely.
4. **Prepare the room** with comfort items, music, blankets, photos, or prayer items.
5. **Prepare the family** — tell key members what changes may happen so they are not shocked.
6. **Write down the death process** — who to call, who pronounces, who calls the funeral home.

■ FIELD GUIDE — LAST DAYS SIGNS & COMFORT

A companion to the **Chapter 27 Last Days Plan** — keep the two together. Care team numbers, comfort medications, who to call first, what to say, and the after-death steps all live on that plan. This page is for what you are seeing and what helps. It does not replace medical advice — call the care team with concerns, and 911 for immediate danger if emergency treatment is wanted or needed.

Common changes to watch (check what is happening)

- | | |
|---|--|
| ☐ Sleeping more / less talking | ☐ Breathing changes / noisy breathing |
| ☐ Less eating / less drinking | ☐ Cool hands or feet / skin color changes |
| ☐ Trouble swallowing | ☐ Less or darker urine |
| ☐ More weakness or confusion | ☐ Pain signs |
| ☐ Restlessness or agitation | ☐ Seeing or talking to people not present |

- ☐ Short rally or burst of alertness

Eating, drinking & mouth care

Food instructions from the care team _____

Fluid instructions _____

Swallowing concerns? (yes / no / unsure) _____

Mouth care instructions _____

Supplies needed: Mouth swabs ☐ Lip balm ☐ Small spoon ☐ Thickened liquids (if ordered) ☐
Towels ☐

Breathing, pain & restlessness plan

QUESTION	WHAT THE CARE TEAM TOLD US
What breathing changes may happen, and what means discomfort?	
What position or medication may help breathing?	
How do we know if the person is in pain, and what is ordered?	
What might cause restlessness, and what should we try first?	
When should we call?	

Room comfort (check what would help)

- | | |
|---|---|
| ☐ Quiet | ☐ Familiar scent |
| ☐ Soft music | ☐ TV off |
| ☐ Prayer / scripture / readings | ☐ Limited visitors |
| ☐ Dim lights | ☐ Open visiting |
| ☐ Favorite blanket / family photos | |

If death seems near

What signs suggest death may be close? (ask the care team) _____

Who to call, who pronounces, the funeral home, and what happens with medications and equipment: see your Chapter 27 plan.

■ IF THE FAMILY IS UNSURE — SCRIPT

"We are seeing changes and we are not sure what they mean. Can you help us understand what is expected, what may be discomfort, and what we should do next?"

ROB'S NOTE

The last days can look different than families expect. That doesn't mean the family is doing something wrong. The body is changing, and the work changes too.

Less fixing. More comforting. Less pushing. More listening. Less panic. More being there.

Call the care team. Ask about what you're seeing. Write down what they tell you. Then come back to the person in the bed. Hold a hand. Speak softly. Protect the room.

The family doesn't have to understand every sign perfectly. They just need to know who to call, what comfort looks like, and that being there still matters.

CHAPTER 28 TAKEAWAY

The last days may bring changes in sleep, eating, drinking, breathing, skin, awareness, and comfort needs. The next right question is: **what are we seeing, who should we call, and what comfort support is needed now?**

CARDINAL MOMENT



A daughter was scared by the change in her father's breathing. The long pauses. The cooling hands. The deep sleep. She was sure he was suffering, and that she was failing him by not doing more.

The hospice nurse sat with her and gently explained what was normal and what it meant. Once she understood what she was seeing, her panic turned into presence. She stopped fighting it and simply stayed by his side.

Fear usually comes from not knowing what is normal. When you understand what the last days really look like, you can stop bracing and start being there.

A composite drawn from years at the bedside. Names and details changed.

After Death

THIS CHAPTER IN 30 SECONDS

- **The most important thing:** in most situations, there is time. The family can pause, sit, and say goodbye — and the right first call is rarely 911.
- **If on hospice:** call hospice first, not 911 — say "*We believe our loved one has died,*" and the team takes it from there. In many states the hospice nurse can pronounce the death, or will arrange for it.
- **If not on hospice:** the facility staff, or the doctor for an expected home death; if the death was unexpected or there is no plan, calling 911 may be necessary — keep the DNR or POLST visible so the person's wishes are honored.
- **Take your time in the room:** there is usually no rush for the funeral home to arrive — ask how much time you have; it is often more than families expect.
- **The paperwork after:** name one point person and let the rest wait — it can almost always wait a little longer than it feels like it can.

Why This Matters

The moments after a death are a strange mix of the holy and the practical. The family is grieving, and yet phone calls have to be made, a funeral home has to come, and decisions arrive that no one wants to make. Families who have a plan move through these hours more gently. Families without one often panic, call 911 by reflex, and find themselves swept into a chaotic scene the person never wanted. Knowing the steps ahead of time is a gift — it lets the family stay in the room a little longer, and it keeps the first hours from becoming a second trauma.

WHAT FAMILIES USUALLY THINK

Families often think they must call 911 immediately, that the body must be moved right away, that they cannot touch or sit with their loved one, or that everything must be handled in the first hour. In an expected death with hospice or a plan in place, none of that is usually true. Calling 911 for an expected death can bring paramedics and sometimes police, and can turn a peaceful bedside into an emergency scene. If there is any doubt about what to do, the

family should know in advance who to call — and it is usually the hospice or the doctor, not 911.

If the Person Was on Hospice

If the person was on hospice, **call hospice first — not 911**. The hospice team will guide the family through the next steps, send a nurse to confirm the death — in many states the hospice nurse can pronounce it, or will arrange for it — and help notify the physician and the funeral home. The nurse can also dispose of medications properly and arrange for equipment. This is one of the quiet gifts of hospice: the family does not have to manage the after-death process alone. Keep the hospice after-hours number where it can be found, and when you call, simply say, "We believe our loved one has died." The team takes it from there.

If the Person Was Not on Hospice

If the person was not on hospice, what to do depends on the setting and whether the death was expected. In a facility, the staff will usually know the process — they will notify the nurse and physician and help contact the funeral home. At home with an expected death and a doctor who will sign the death certificate, the family may be able to call that doctor or an on-call line. If the death was unexpected, or there is no plan and no clear person to call, calling 911 may be necessary. Understand that this may bring emergency responders — and, in some areas, law enforcement — and may lead to resuscitation attempts unless a valid DNR or POLST is present and visible. This is exactly why the documents in Part 4 matter, and why they should be easy to find.

Taking Your Time in the Room

Once the immediate calls are made, or while you are deciding, it is okay to stay. Many families find comfort in sitting with their loved one, holding a hand, praying, playing music, reading scripture, or simply being present. Some families like to bathe or dress the person, or to gather others so everyone can say goodbye. There is usually no rush for the funeral home to arrive. Ask the hospice nurse or funeral home how much time you have — it is often more than families expect. This is a moment you do not get back. If it helps you to linger, linger.

When the Funeral Home Comes

When the family is ready, the funeral home is called to take the person into their care. If a funeral home was chosen ahead of time, this call is simple. If not, the hospice or facility

can help, or the family can choose one now. When the funeral home arrives, family members may stay or step away — there is no right choice, only what each person can bear. Some want to accompany their loved one to the door; some cannot watch. Both are acts of love. The funeral home will explain the next steps, including when to come in to make arrangements and what documents they will need.

The First Days of Practical Tasks

In the days after, practical tasks arrive — often more than the family expects. These may include:

- ☐ obtaining copies of the death certificate (get several; many institutions require an original)
- ☐ meeting with the funeral home
- ☐ notifying Social Security
- ☐ contacting pension and insurance companies
- ☐ notifying banks and closing or changing accounts
- ☐ canceling appointments and services
- ☐ beginning to sort belongings

None of this has to happen at once. Choose one point person to lead, and let the rest wait. Grief and paperwork do not keep the same schedule, and the paperwork can almost always wait a little longer than it feels like it can.

WHAT TO DO — IN THE FIRST HOURS

1. **Pause.** Unless the situation requires otherwise, there is time. Breathe before you dial.
2. **Call the right first number** — hospice if on hospice, the facility staff in a facility, the doctor for an expected home death, or 911 only if there is no plan or the death was unexpected.
3. **Have the DNR or POLST visible** if responders may come, so the person's wishes are honored.
4. **Stay as long as you need.** Sit, hold a hand, pray, gather family, say goodbye.
5. **Call the funeral home when ready** — or let hospice or the facility help you choose one.
6. **Name one point person** for the calls and paperwork that follow, and let everything else wait.

■ **Worksheet — After-Death Plan** — fill it in from the printable companion workbook ·
TheCardinalsPromise.com/resources

ROB'S NOTE

The first thing I tell families after a death is simple: you don't have to hurry.

The world will tell you to move fast. Make the calls. Sign the forms. Clear the room. But most of the time, the person you love isn't going anywhere in the next hour. And neither are you. You're allowed to stay. You're allowed to sit in the quiet. You're allowed to say the last things.

I've watched families panic and call 911 without thinking. Then they stand in a hallway while strangers work over a body that was ready to rest. It doesn't have to be that way. That's what the plan is for.

I wasn't with my father when he died. You may have read that at the front of this book. It's the reason this book exists. So hear me: if you're given the hour I missed, you won't need a checklist to tell you to hold a hand. The paperwork can wait. Make the one call that matters. Then go back and be a son, a daughter, or a spouse. Not a coordinator. The rest will keep.

The Weeks After

If hospice was involved, their bereavement team stays with your family — typically for about thirteen months — with calls, mailings, and grief groups at no charge. You can say yes to it months later, even if you said no at first. Grief has a long tail: the fog, the sleeplessness, the anger, the moment you forget and reach for the phone — none of it means you are going crazy. It means you loved someone. If the weight does not begin to lift, or you simply want company inside it, a grief group or counselor is not a luxury. Ask the hospice, your doctor, or your faith community — any month, in any year.

CHAPTER 29 TAKEAWAY

After a death, there is usually more time than families think, and the right first call is rarely 911. The next right question is: **who do we call first, and what can wait until we are ready?**

CARDINAL MOMENT



In the first hour after her mother died, a daughter stood in the quiet. She had no idea what she was supposed to do. Call 911? Call the funeral home? Call no one? She felt like she was already failing at something. She didn't even know what.

The hospice nurse had left one sheet of paper. It said who to call, in what order, and what could wait until morning. That sheet let her stop managing the moment. At last, she could just grieve her mother.

In the first hours, almost everything can wait longer than you think. The few things that can't should already be written down. That way, you're free to just be their child.

A composite drawn from years at the bedside. Names and details changed.

KEEP GOING

You don't have to do this alone. Find more support — and Rob's memoir *The Cardinal's Promise* — at thecardinalspromise.com.

About the Author



Rob Brizzi is a Certified Dementia Practitioner who has spent his career in home health, hospice, senior living, and other healthcare settings — years of sitting with families in hospital hallways, facility lobbies, and living rooms as they faced the decisions in this book. He is the founder of The Cardinal's Promise, an education effort helping families navigate aging, dementia, caregiving, serious illness, and loss, and the author of the forthcoming memoir *The Cardinal's Promise*. He wrote this guide after the death of his father, Lou Brizzi — the man who taught him that showing up is most of the job.

A Note of Thanks

The guidance in this book on protecting older adults from scams and financial exploitation was motivated in part by the consumer-protection work of **Clark Howard**, whose plain rule — that no legitimate business or agency will ever ask to be paid in gift cards — has helped countless families recognize a scam before the money is gone.

Learn more, find caregiver tools, and reach the author at thecardinalspromise.com.

ALSO BY ROB BRIZZI

The Cardinal's Promise

Rob's memoir — the story of being adopted, of loss, and of learning to show up for the end of life. It's the story behind this guide. Coming soon; follow along and find free caregiver tools at thecardinalspromise.com.

Your State Guide

Almost every office in this book has a version in your state — same job, different name. This page shows you how to find each one. Programs and websites change; when in doubt, start with the Eldercare Locator (1-800-677-1116), whose entire job is connecting you to your local agencies.

WHAT THIS BOOK CALLS IT	HOW TO FIND YOUR STATE'S VERSION
State Medicaid office	Medicaid.gov → "Contact your state," or search "[your state] Medicaid long-term care." Applications run through your county or state social services office — names vary ("Social Services," "Human Services," "Job and Family Services") — or your state's online portal.
SHIP — free, unbiased Medicare counseling	shiphelp.org or 1-877-839-2675. Every state has one; many use their own name (SHIP, SHIBA, HICAP, and others).
Area Agency on Aging	Eldercare Locator, 1-800-677-1116 or eldercare.acl.gov — it routes you by ZIP code.
Adult Protective Services	Find yours through the National Adult Protective Services Association directory (napsa-now.org) or the Eldercare Locator — every state has an APS program, and many run a statewide hotline.
Portable medical order form (Chapter 23)	Ask the doctor which form your state uses — POLST in many states, MOST in others (polst.org lists every state's program and form).
Advance directive forms (Chapters 20–21)	In most states the attorney general, health department, or bar association publishes free statutory forms; an elder law attorney can confirm what your state requires (witnesses, notarization).
Veterans services (Chapter 18)	Your county Veterans Service Officer (search "[your county] veterans service officer") or VA.gov; every state also has a Department of Veterans Affairs.
Assisted living / facility inspection records	Each state's licensing agency publishes inspection reports — search "[your state] assisted living inspection reports" — and Medicare's Care Compare (medicare.gov/care-compare) rates nursing homes and home health agencies nationally.

Quick-start guides — Rob's home states. Because this work is rooted in the Southeast, this edition names the actual offices for four states. Numbers were current at press time;

confirm before relying on them, and note the state your parent lives in is what governs — not where you live.

North Carolina

WHAT YOU NEED	WHO TO REACH IN NORTH CAROLINA
Medicaid & long-term care	Apply at epass.nc.gov or your county Department of Social Services. NC Medicaid Contact Center 1-888-245-0179 .
Free Medicare counseling (SHIP)	SHIIP — Seniors' Health Insurance Information Program (NC Dept. of Insurance), 1-855-408-1212 .
Report abuse, neglect, exploitation	Your county Department of Social Services ; the NC DHHS CARE-LINE, 1-800-662-7030 , will route you.
Aging services / Area Agency on Aging	NC Division of Aging & Adult Services, ncdhhs.gov/aging , or the Eldercare Locator, 1-800-677-1116 .
Portable medical order (Ch. 23)	North Carolina uses the MOST form (Medical Orders for Scope of Treatment) — ask the doctor.
Advance directive (Ch. 20–21)	NC Health Care Power of Attorney + Advance Directive for a Natural Death; free statutory forms and the state registry at the NC Secretary of State (sosnc.gov).
Veterans (Ch. 18)	NC Dept. of Military & Veterans Affairs (milvets.nc.gov) or your county Veterans Service Officer.

Florida

WHAT YOU NEED	WHO TO REACH IN FLORIDA
Medicaid & long-term care	Apply through ACCESS Florida — myflorida.com/accessflorida or 1-866-762-2237 . Long-term care runs through the Statewide Medicaid Managed Care program.
Free Medicare counseling (SHIP)	SHINE — Serving Health Insurance Needs of Elders (Dept. of Elder Affairs), Elder Helpline 1-800-963-5337 .
Report abuse, neglect, exploitation	Florida Abuse Hotline, 1-800-962-2873 (1-800-96-ABUSE), any hour.
Aging services / Area Agency on Aging	Florida Dept. of Elder Affairs, Elder Helpline 1-800-963-5337 (elderaffairs.org).

Portable medical order (Ch. 23)	Florida uses a state DNRO — Do Not Resuscitate Order, the yellow DH Form 1896 — not POLST. Ask the doctor.
Advance directive (Ch. 20–21)	Florida Living Will + Designation of Health Care Surrogate (Fla. Stat. Ch. 765); free forms from Florida Health and the Florida Bar.
Veterans (Ch. 18)	Florida Dept. of Veterans' Affairs (floridavets.org), 1-844-693-5838.

Virginia

WHAT YOU NEED	WHO TO REACH IN VIRGINIA
Medicaid & long-term care	Virginia Medicaid — Cardinal Care — apply through Cover Virginia, 1-855-242-8282 or commonhelp.virginia.gov , or your local Dept. of Social Services.
Free Medicare counseling (SHIP)	VICAP — Virginia Insurance Counseling & Assistance Program (DARS), 1-800-552-3402 .
Report abuse, neglect, exploitation	Virginia Adult Protective Services, 1-888-832-3858 , 24 hours.
Aging services / Area Agency on Aging	Dept. for Aging & Rehabilitative Services (DARS), 1-800-552-3402 (vda.virginia.gov).
Portable medical order (Ch. 23)	Virginia uses the POST form (Physician Orders for Scope of Treatment); the National POLST form is also accepted (virginiapost.org).
Advance directive (Ch. 20–21)	Virginia Advance Directive (Code § 54.1-2984); free forms from the Virginia State Bar and virginiaadvancedirectives.org .
Veterans (Ch. 18)	Virginia Dept. of Veterans Services (dvs.virginia.gov).

South Carolina

WHAT YOU NEED	WHO TO REACH IN SOUTH CAROLINA
Medicaid & long-term care	Healthy Connections (SC Dept. of Health & Human Services) — apply at apply.scdhhs.gov ; Resource Center 1-888-549-0820 .
Free Medicare counseling (SHIP)	I-CARE — Insurance Counseling, Assistance & Referrals for Elders (SC Dept. on Aging), 1-800-868-9095 .
Report abuse, neglect, exploitation	1-888-227-3487 (1-888-CARE4US), SC Dept. of Social Services — confidential, any hour.

Aging services / Area Agency on Aging	SC Dept. on Aging, 1-800-868-9095 (aging.sc.gov) or GetCareSC.com .
Portable medical order (Ch. 23)	South Carolina uses the POST form (Physician Orders for Scope of Treatment) — ask the doctor.
Advance directive (Ch. 20–21)	South Carolina uses two documents — a Declaration of a Desire for a Natural Death (living will) and a Health Care Power of Attorney .
Veterans (Ch. 18)	SC Dept. of Veterans' Affairs (scdva.sc.gov) or your county Veterans Affairs Office.

More states are added as the Cardinal's Promise Series grows — with every program, office, and form for your state named and explained — at thecardinalspromise.com. (Virginia, it turns out, calls its Medicaid program Cardinal Care. We take that as a good sign.)

Find Your Worry

Not sure where to start? Find the worry that sounds like yours and turn to the page. The dot shows how urgent it usually is — ■ act today ■ watch closely ■ plan ahead.

When something feels off

■ Something feels off, but I can't name it.	p. 33
■ Mom keeps falling.	p. 39
■ Dad isn't steady on his feet.	p. 39
■ She's repeating herself and forgetting names.	p. 47
■ Is this normal aging, or dementia?	p. 47
■ I'm exhausted, angry, and I feel guilty.	p. 55
■ I can't do this anymore.	p. 55
■ My siblings won't help — or we can't agree.	p. 64

When you need care

■ Dad needs help at home but won't move.	p. 75
■ I need someone to help with bathing and meals.	p. 83
■ Mom can't live alone anymore.	p. 91
■ Dad wanders and isn't safe with the stove.	p. 95
■ The hospital is discharging her and says she needs rehab.	p. 101
■ Is Mom "admitted," or under "observation"?	p. 101
■ They say she's "plateaued" and therapy is ending.	p. 101
■ She's in pain but not dying — what helps?	p. 107
■ The doctor said "six months."	p. 113

When you're paying for it

■ How will we pay for any of this?	p. 121
■ What does Medicare actually cover?	p. 121
■ We're running out of money for the nursing home.	p. 127
■ Do we have to spend everything first?	p. 127
■ We'll be paying out of our own pocket.	p. 133
■ Mom has a long-term care policy — how do we use it?	p. 139
■ Her medications cost too much.	p. 144

■ Dad's a veteran — is there help?	p. 150
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When you're getting the papers in order

■ I can't find any of the paperwork.	p. 157
■ Who can make medical decisions if she can't?	p. 161
■ What treatment does she want at the end?	p. 167
■ Who handles the money and the bills?	p. 174
■ I think someone is scamming my parent.	p. 174
■ Do we need a DNR? What's a POLST?	p. 181
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■ I can't keep track of her medicines and doctors.	p. 192
■ What do I do in an emergency — who do I call?	p. 198

When the end is near

■ The end is close — what's the plan?	p. 206
■ What will the last days actually look like?	p. 212
■ She's gone. What do I do now?	p. 219

Glossary

Plain-language meanings for the terms families run into most — with the page where each is used.

Activities of Daily Living (ADLs) p. 89

The basic things a person does each day to care for themselves — eating, bathing, dressing, using the toilet, and getting around. How much help someone needs with these usually decides what kind of care they need.

Adult Protective Services (APS) p. 207

A state agency that looks into reports that an older or disabled adult is being hurt, neglected, or taken advantage of. Call them if you think someone is being harmed.

Advance Directive p. 53

A paper that says, ahead of time, what medical care a person does or does not want if they cannot speak for themselves. It usually includes a living will and a health care power of attorney.

Area Agency on Aging p. 149

A local office that helps older adults and families find services like meals, rides, in-home help, and caregiver support. Free to call.

Assisted Living p. 45

A place where seniors live in their own room or apartment and get help with daily tasks like meals, medicine, and bathing — but not full nursing care.

Do Not Resuscitate (DNR) p. 212

A doctor's order not to do CPR (restart the heart) if the person's heart or breathing stops. It only covers CPR, not other care.

Financial Power of Attorney p. 51

A legal paper that lets a trusted person handle money, bills, and accounts for someone who cannot. "Durable" means it still works after the person becomes too sick to manage.

Extra Help p. 92

A Medicare program that helps people with low income pay for their prescription drug costs.

Guardianship p. 211

When a court names someone to make decisions for an adult who no longer can — and had no power of attorney in place. It takes months and money; good paperwork ahead of time usually avoids it.

Home Health p. 39

Skilled care (nursing or therapy) given at home for a limited time, usually paid by Medicare after a hospital stay. Not the same as everyday help.

Hospice p. 58

Comfort care for someone expected to live about six months or less. It eases pain and supports the family instead of trying to cure the illness. Usually fully covered by Medicare.

Living Will	p. 72
A paper that spells out what treatment a person wants, or does not want, near the end of life — like breathing machines or feeding tubes.	
Long-Term Care	p. 69
Ongoing help with daily living for months or years — at home, in assisted living, or in a nursing home. Regular insurance and Medicare usually do not pay for it.	
Medicaid	p. 72
A government program that pays for health care and long-term care for people with low income and few savings. Run by each state, with different rules.	
Medicaid Look-Back	p. 146
When you apply for Medicaid to cover a nursing home, the state checks the last five years for money or property given away. Gifts in that window can delay coverage.	
Medicare	p. 44
The federal health insurance for people 65 and older (and some younger people with disabilities). It covers doctors and hospitals but not most long-term care.	
Medicare Advantage	p. 139
A private plan that replaces Original Medicare. Often lower cost, but a smaller network of doctors and its own rules.	
Memory Care	p. 45
A kind of assisted living made for people with dementia — secured doors and staff trained for memory loss.	
Observation Status	p. 139
When a hospital keeps a patient but calls it “observation” instead of “admitted.” This can quietly leave the family paying for the rehab that follows. Always ask which status your parent is under.	
Palliative Care	p. 60
Care that eases pain and stress from a serious illness. Unlike hospice, you can have it at any stage — even while still seeking a cure.	
POLST / MOLST	p. 72
A doctor’s order on a bright form that says what emergency treatment a seriously ill person wants. It travels with them, and paramedics must follow it.	
Health Care Power of Attorney	p. 72
A legal paper that names the person who can make medical decisions for someone who cannot speak for themselves.	
Private Duty Care	p. 44
Help you pay for yourself (or with long-term care insurance) — an aide for bathing, meals, or overnight care. Medicare usually does not cover it.	
QIO (Quality Improvement Organization)	p. 114
The group you can call for a fast, free appeal if a hospital or rehab tries to send your parent home too soon.	

Skilled Nursing Facility (SNF)

p. 44

A place that gives short-term nursing and rehab, often right after a hospital stay. Medicare may pay for a limited time — but only if the hospital “admitted” the patient.

SHIP (State Health Insurance Assistance Program)

p. 142

Free, unbiased help in every state for understanding Medicare choices. Not selling anything.

Trust

p. 148

A legal setup that holds money or property for someone. An “irrevocable” trust cannot be easily changed and is sometimes used, well ahead of time, to protect assets — always with an attorney.

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Numbers That Matter

Tear this page out, photograph it, or tape a copy to the refrigerator. Phone numbers change — confirm before relying on them, and write your local numbers in the blanks. A printable copy of this page lives at **thecardinalspromise.com**.

WHEN	WHO	NUMBER
Immediate danger or life-threatening emergency	Emergency services	911
Mental health crisis — yours or theirs, any hour	Suicide & Crisis Lifeline	988 (call or text)
Finding local aging services anywhere in the U.S.	Eldercare Locator	1-800-677-1116
Dementia questions or a dementia crisis, 24/7	Alzheimer's Association Helpline	1-800-272-3900
Medicare questions	Medicare	1-800-MEDICARE (1-800-633-4227)
Free, unbiased Medicare counseling	State Health Insurance Assistance Program (SHIP)	1-877-839-2675 · shiphelp.org
Fast appeal when a hospital, SNF, or home health stay is being cut short	Your Medicare QIO (Quality Improvement Organization)	qioprogram.org ("Locate Your QIO") or 1-800-MEDICARE
Caring for a Veteran	VA Caregiver Support Line	1-855-260-3274
Suspected abuse, neglect, or financial exploitation	Adult Protective Services (statewide hotline in most states)	napsa-now.org or Eldercare Locator 1-800-677-1116

YOUR LOCAL NUMBERS	WRITE THEM HERE
Primary doctor	
Pharmacy	
County / state social services office (Medicaid, APS)	
Area Agency on Aging	

Hospice / home health agency	
Facility nurse / after-hours line	
Family point person	

IT'S NOT YOUR FAULT

"Take the next right step."

RLB, Certified Dementia Practitioner
Cardinal's Promise Series · thecardinalspromise.com